

1. This case presents signs of magnesium sulfate toxicity, as indicated by absent patellar reflexes and reduced urine output (which can lead to accumulation of magnesium).

Option A: Change the drug to diazepam – Incorrect

- Diazepam is not the first-line treatment for severe pre-eclampsia or eclampsia.

Magnesium sulfate is the preferred drug because it is

more effective in preventing seizures.

Option B: Providing calcium gluconate – Correct

- Calcium gluconate is the antidote for magnesium toxicity. Since the patient shows signs of toxicity (absent reflexes, oliguria), administering calcium gluconate IV and stopping magnesium sulfate is the most appropriate immediate step.

Option C: Continue administering magnesium sulfate – Incorrect

- Continuing magnesium sulfate in a patient showing toxicity signs can worsen the condition, leading to respiratory depression and cardiac arrest.

Option D: Resuscitating the mother – Not immediately necessary

- The patient is not yet in cardiac or respiratory arrest, so immediate resuscitation (CPR) is not required. However, close monitoring and supportive care are necessary.

Final Answer: B. Providing calcium gluconate

2. The worded administrator involves staff in decisions, shares responsibilities, and encourages innovation, which

indicates a participatory leadership style.

Option A: Authoritarian (Autocratic) – Incorrect

- An authoritarian leader makes decisions alone, without staff input. They give strict orders and expect obedience, which contradicts the administrator's approach of involving staff in decision-making.

Option B: Democratic – Correct

- A democratic leader encourages participation, values staff input, delegates responsibilities, and fosters innovation—exactly what the administrator is doing. This is the best fit for the described leadership style.

**Option C: Laissez-faire –
Incorrect**

- A laissez-faire leader takes a hands-off approach, allowing staff complete freedom with little direction. However, the

administrator in the scenario is actively engaging with staff, not just letting them work independently.

Option D: Transformational – Partially correct but not the best

- A transformational leader focuses on inspiring and motivating staff toward a visionary goal, often driving major change. While the administrator does encourage innovation, the emphasis here

is more on shared decision-making rather than radical transformation.

Final Answer: B. Democratic

3. The research found 1% prevalence of Vitamin A deficiency among under-five children, requiring appropriate supplementation. Let's analyze each option:

Option A: Supplementation of women at 6-month intervals postpartum – Incorrect

- Postpartum women can receive high-dose Vitamin A supplementation, but this does not directly address deficiency in children. The focus should be on children under five.

Option B: Supplementation of women during the first 1 month of pregnancy – Incorrect

- **Vitamin A should not be given in high doses during early pregnancy due to the risk of teratogenic effects. The goal is to target children directly.**

Option C: Supplementation of children at 6-month intervals after 6 months of infancy – Correct

- **WHO and UNICEF recommend high-dose Vitamin A supplementation every 6 months for children 6–59**

months to prevent and treat Vitamin A deficiency. This aligns with the best practice for addressing the deficiency in the community.

Option D: Supplementation of children from birth to 6 months of infancy – Incorrect

- Newborns and infants under 6 months should receive Vitamin A through breast milk. Direct supplementation is not recommended in this age

group unless prescribed in specific medical conditions.

Final Answer: C.

Supplementation of children at 6-month intervals after 6 months of infancy

4. A 30-month-old child with a 3-week cough, low-grade fever, and a father on TB treatment suggests TB exposure or active TB infection.

Option A: Refer urgently to the hospital

- Not necessarily the first step**

Referral is needed if the child has severe TB symptoms (e.g., difficulty breathing, severe malnutrition). However, before referral, an assessment for active TB or latent TB infection is required.

Option B: Counsel on the diagnosis of TB exposure – Insufficient action

- Counseling is important but not enough as the first step. The child needs medical evaluation and possible treatment.

Option C: Initiate INH as TB prophylaxis – Incorrect (requires ruling out active TB first)

- Isoniazid (INH) prophylaxis is only for children exposed to TB but without active TB symptoms. Since this child has a persistent cough and fever, active TB must be ruled out first before considering prophylaxis.

Option D: Put the child on anti-TB treatment – Correct

- The child has TB symptoms (cough >2 weeks, fever) and direct exposure to a TB patient. This strongly suggests

active TB, and WHO guidelines recommend starting anti-TB treatment after confirming the diagnosis.

Final Answer: D. Put the child on anti-TB treatment

5 . Priority Nursing Intervention for a Woman with Inevitable Abortion

Option A: Prepare for blood transfusion – Not the first priority

- The woman has a hemoglobin level of 9g/dL, which is low but not critical for immediate transfusion. Blood loss must be monitored, but transfusion is not the first step.

Option B: Resuscitate with normal saline – Correct

- The woman has signs of hypovolemic shock (BP 90/50 mmHg, pulse 110/min, vaginal bleeding). The first priority is fluid resuscitation with normal

saline to stabilize circulation and prevent shock.

Option C: Provide analgesia

- Not the first priority**

Pain relief is necessary, but fluid resuscitation takes priority in an unstable patient.

Option D: Reassure the woman

- Not the first priority**

- Emotional support is important, but stabilizing the**

patient's condition is the top priority.

Final Answer: B. Resuscitate with normal saline.

7. IMNCI Classification of a 2-month-old Infant with Cough

Option A: Cough/Cold – Incorrect

- The infant has fast breathing (RR = 50/min for 2 months old), which is beyond the normal range (<50/min for this age group). This suggests a

more serious illness than a simple cough/cold.

Option B: Pneumonia – Correct

- According to IMNCI (Integrated Management of Newborn and Childhood Illness), pneumonia is classified when:

- Fast breathing: ≥ 50 /min in infants aged 2–11 months

No chest in-drawing, no danger signs

Option C: Very Severe

- Pneumonia – Incorrect

A child is classified as very severe pneumonia if they have danger signs (e.g., unable to feed, convulsions, lethargy, or severe respiratory distress).

This infant does not have these signs.

Option D: Severe Pneumonia – Incorrect

Severe pneumonia includes chest in-drawing or danger signs, which are not present.

Final Answer: B. Pneumonia

8 . Study Design Used in the Stomach Cancer Research

Option A: Ecological – Incorrect

- Ecological studies analyze data at the population level, not individual participants.

Option B: Case-Control – Correct

- The study involves:

Cases (198 people with stomach cancer)

Controls (396 people without stomach cancer)

Looking back at past fruit/vegetable consumption (retrospective)

This is the classic case-control study design, which examines past exposure in two groups.

Option C: Cohort – Incorrect

- Cohort studies follow people over time to see who develops a disease. This study is retrospective, not prospective.

Option D: Cross-Sectional – Incorrect

- Cross-sectional studies assess exposure and disease at the same time, whereas this study looks at past exposure (fruit/vegetable intake).

Final Answer: B. Case-Control

10 . Nursing Diagnosis for a Breastfeeding Mother with Breast Swelling

Option A: Risk for injury – Incorrect

- There is no direct physical trauma involved.**

Option B: Risk for impaired skin integrity – Incorrect

- This applies to skin breakdown (e.g., ulcers, burns, wounds), which is not the primary concern here.**

Option C: Risk for infection – Correct

- The mother has breast swelling, tenderness, and an irregular mass, which are signs of mastitis (a bacterial infection). This makes infection the primary concern.

Option D: Risk for imbalanced body temperature – Incorrect

- Fever can be a symptom, but the primary nursing diagnosis is infection risk due to mastitis.

Final Answer: C. Risk for infection

11 . IMNCI Classification of a Neonate with Jaundice

Option A: No Jaundice – Incorrect

- The neonate has yellow discoloration in the face and eyes, indicating jaundice.

Option B: Severe Jaundice – Incorrect

- Severe jaundice includes palms/soles turning yellow or other danger signs (e.g., lethargy, poor feeding), which are not present here.

Option C: Uncomplicated Jaundice – Correct

- The neonate has jaundice limited to the face and eyes, which is classified as uncomplicated jaundice per IMNCI guidelines.

Option D: Jaundice – Too vague

12 . Management of a Pregnant Woman with Hyperemesis Gravidarum

Option A: Advise the mother to take oral food – Incorrect

- She has severe nausea, vomiting, and dehydration, so oral food is not appropriate initially.

Option B: Administer anti-emetics immediately – Partially correct

- Anti-emetics help, but fluid replacement is the priority.

Option C: Administering fluid for the mother – Correct

- She has dehydration (dry mucosa, ketonuria +4, low urine output), so IV fluids are the immediate priority.

Option D: Providing total parenteral nutrition (TPN) – Not necessary at this stage

- TPN is used in extreme cases where oral/IV fluids fail.

Final Answer: C. Administering fluid for the mother

13. Management of an Infant with Severe Acute Malnutrition and Severe Anemia

Option A: Give 1st dose of antibiotic and refer – Correct

- The infant has MUAC < 11.5cm, WFL < -3Z, edema, and low RBS, indicating severe acute malnutrition (SAM).

WHO guidelines recommend:

Urgent referral

Antibiotic administration (due to high infection risk in SAM)

Option B: Give oral amoxicillin for seven days – Incorrect

- Oral treatment is not enough for a severely malnourished infant.

Option C: Admit the infant on OTP program – Incorrect

- **Outpatient programs are for moderate malnutrition. This infant needs inpatient care.**

Option D: Give folic acid supplement for anemia – Not the priority

- **Severe anemia requires urgent hospital referral.**

8. Conflict Resolution Strategy in a Health Center

Option A: Collaboration – Correct

- **Collaboration involves both assertiveness and cooperation, which means both parties work together to find a mutually beneficial solution.**

Option B: Accommodation – Incorrect

- **Accommodation means one party gives in, which does not involve assertiveness.**

Option C: Competition – Incorrect

- **Competition is a win-lose approach, lacking cooperation.**

**Option D: Avoidance –
Incorrect**

- **Avoidance means ignoring the conflict, which is not the case here**

16. First-Line Drug for Cerebral Malaria (*P. falciparum*) in a 10-Year-Old Child

Option A: Quinidine – Incorrect

- **Quinidine is an anti-malarial but is mainly used in severe**

cases when other options are unavailable. It is also associated with cardiac toxicity, making it less preferred.

Option B: Artesunate – Correct

- IV Artesunate is the first-line treatment for severe malaria, including cerebral malaria caused by *P. falciparum*. It has better efficacy and lower toxicity than quinine.

Option C: Artemether – Incorrect

- Artemether is used but is less preferred than IV artesunate in severe cases.**

Option D: Quinine

Dihydrochloride – Incorrect

- Quinine was previously first-line but has more side effects and slower action compared to artesunate.**

18. Nursing Intervention for a Child with Bedwetting (Nocturnal Enuresis)

Option A: Scolding the Child – Incorrect

- Scolding can cause emotional distress and is not an effective solution.**

Option B: Decreasing Daytime Fluids – Incorrect

- Reducing daytime fluid intake does not prevent nighttime bedwetting and can lead to dehydration.**

Option C: Using an Alarm Wake-Up System – Correct

- **Waking the child one hour before bedwetting helps train the bladder and is an effective behavioral therapy.**

Option D: Using Diapers – Incorrect

- **Diapers do not solve the problem; they only manage the symptoms.**

Final Answer: C. Counsel the family to use an alarm wake-up system

19 . Preoperative Nursing Care for a Woman with Uterine Prolapse

Option A: Prescribe Sedative Drugs – Incorrect

- Nurses cannot prescribe drugs.**

Option B: Advise About Exercise – Incorrect

- Pelvic exercises help in early-stage prolapse but are not the priority for a patient scheduled for surgery.**

Option C: Teach About Self-Care – Incorrect

- Self-care is important, but preparing for surgery takes priority.**

Option D: Physical and Psychological Preparation – Correct

- Before surgery, physical (e.g., NPO status, bowel preparation) and psychological (counseling, reassurance) preparation is essential.**

Final Answer: D. Physical and psychological preparation

5. Nursing Action for a Post-Surgical Patient with Alcohol Withdrawal Symptoms

Option A: Record Findings – Not the priority

- Documentation is important, but immediate medical intervention is needed.

Option B: Follow the Patient Closely – Not enough

- **Close monitoring is required, but urgent medical action is needed.**

Option C: Handover to the Next Nurse – Not appropriate

- **The situation needs immediate intervention, not just reporting.**

Option D: Consult the Physician Immediately – Correct

- **The patient is experiencing alcohol withdrawal seizures,**

which require urgent medical attention.

Final Answer: D. Consult the physician immediately

**6. Management Function
Leading to Low Achievement
of a Plan**

Option A: Organizing – Correct

- Organizing involves mobilizing human and material resources. Failure to do so leads to low achievement

Option B: Planning – Incorrect

- A plan was already made; the issue was executing it.

Option C: Directing – Incorrect

- Directing is about guiding staff, but the issue was resource allocation.

Option D: Controlling – Incorrect

- Controlling ensures plans are followed, but here the problem was resource obilization.

7. Nursing Diagnosis for a Woman with IUCD and Pelvic Pain

Option A: Risk for Injury – Incorrect

Injury is not the main concern.

Option B: Risk for Bleeding – Incorrect

No active bleeding is reported.

**Option C: Risk for Trauma –
Incorrect**

**Trauma is not the correct
classification.**

**Option D: Risk for Infection –
Correct**

The patient has lower abdominal pain, dyspareunia, and uterine/cervical tenderness, suggesting Pelvic Inflammatory Disease (PID), which is an infection.

Final Answer: D. Risk for Infection

8. Priority Nursing Intervention for a Newborn with Severe Breathing Difficulty

**Option A: Give Oxygen –
Correct**

**The baby has severe chest
indrawing, difficulty breathing,
and cyanosis, indicating
respiratory distress. Oxygen is
the immediate priority.**

Option B: Put in Radiant Warmer – Not the first priority

Thermal care is important but not urgent compared to oxygen support.

Option C: Encourage Breastfeeding – Not possible

The baby cannot breastfeed due to breathing difficulty.

Option D: Clear Airways – Incorrect

The baby has no airway obstruction, so oxygen therapy is the priority.

Final Answer: A. Give oxygen

9. Patient Rights Violation in a Hospital (DKA Case)

**Option A: Non-Consented Care
– Incorrect**

**The patient was informed and
agreed to treatment.**

**Option B: No Dignified Care –
Incorrect**

**The issue is financial
detainment, not dignity.**

**Option C: Detention in Facility
– Correct**

**Holding a patient in a hospital
due to inability to pay is a form
of detention and a violation of
patient rights.**

**Option D: Abandonment of
Care – Incorrect**

The hospital did not abandon the patient; they refused discharge.

Final Answer: C. Detention in Facility

10. Key Feature of COVID-19 Surveillance

Option A: Timely Reporting – Correct

Immediate reporting to health authorities is a key feature of good surveillance.

Option B: Combined Passive and Active Methods – Not specified

The question does not indicate both passive and active surveillance.

Option C: Strong Laboratory Services – Not the main feature

While important, the primary feature here is timely reporting.

**Option D: Comprehensive
Action – Not the main focus**

**Surveillance primarily focuses
on data collection and
reporting.**

**Final Answer: A. The team had
timely reporting of cases**

11. Drug of Choice for MRSA Osteomyelitis

Option A: Vancomycin and Cefotaxime – Correct

MRSA (Methicillin-Resistant Staphylococcus Aureus) is treated with Vancomycin, a first-line antibiotic for MRSA infections.

**Option B: Nafcillin and
Cefotaxime – Incorrect**

**Nafcillin is not effective against
MRSA.**

**Option C: Vancomycin and
Oxacillin – Incorrect**

**Oxacillin is not effective
against MRSA.**

**Option D: Oxacillin and
Cefotaxime – Incorrect**

Oxacillin does not cover MRSA.

**Final Answer: A. Vancomycin
and Cefotaxime**

12. Nutritional Status of a Newborn Weighing 2.0 kg

Option A: Obese – Incorrect

2.0 kg is not obese.

**Option B: Overweight –
Incorrect**

Overweight babies weigh >4.0 kg.

**Option C: Low Birth Weight –
Correct**

Low birth weight is <2.5 kg.

**Option D: Normal Birth Weight
– Incorrect**

**Normal birth weight is 2.5–4.0
kg.**

Final Answer: C. Low Birth Weight

1. Priority Nursing Intervention for Unexplained Infertility

Option A: Advise to Reduce Alcohol Consumption – Incorrect

There is no mention of alcohol use, so this is not the most relevant intervention.

Option B: Provide Medication to Resume Fertility – Incorrect

**There is no specific medication
for unexplained infertility
unless further tests indicate a
treatable cause.**

Option C: Reassure the Couple for Psychological Upset – Correct

Unexplained infertility can cause emotional distress. Reassurance and psychological support are crucial before considering advanced interventions.

Option D: Suggest Alternative Sources of Childbearing – Premature

Suggesting adoption or assisted reproduction is

appropriate later but not the first intervention.

Final Answer: C. Reassure the couple for psychological upset

2. Sign of Improvement in Hyperemesis Gravidarum

Option A: Increased Blood Glucose – Incorrect

Blood glucose improvement alone does not indicate full recovery.

Option B: Increased Hematocrit Level – Incorrect

High hematocrit suggests dehydration, so an increase is not a sign of improvement.

**Option C: Protein-Free Urine –
Incorrect**

**Proteinuria is not a key feature
of hyperemesis gravidarum.**

**Option D: Ketone-Free Urine –
Correct**

**Ketones indicate severe
starvation and dehydration.
The disappearance of ketones**

**suggests hydration and
adequate nutrition.**

**Final Answer: D. Ketone-Free
Urine**

**3. Likely Complication of
Measles in a Child**

**Option A: Seizure Shock –
Incorrect**

Not a primary complication.

**Option B: Cerebral Palsy –
Incorrect**

**Measles does not directly
cause cerebral palsy.**

Option C: Croup – Incorrect

Croup is a viral infection but not a common measles complication.

Option D: Pneumonia – Correct

Pneumonia is the most serious complication of measles and can lead to death.

Final Answer: D. Pneumonia

4. Most Suitable Health Insurance Scheme for Farmers

Option A: Individual Health Insurance – Incorrect

Farmers need community-based coverage, not individual plans.

Option B: Mandatory Health Insurance – Incorrect

Mandatory insurance is government-enforced and may not fit rural farmers.

Option C: Social Health Insurance – Incorrect

Social health insurance is for employed workers in formal sectors.

Option D: Community-Based Health Insurance – Correct

This scheme is designed for informal sectors like farmers, making healthcare affordable.

Final Answer: D. Community-Based Health Insurance

5. Mother Who Needs Nutritional Counseling the Most (MUAC 21 cm)

Option A: RH Negative Mother – Incorrect

RH status is not related to nutrition.

**Option B: HIV Positive Mother
– Correct**

**HIV increases the risk of malnutrition and infections.
She needs urgent nutritional counseling.**

Option C: Normotensive Mother – Incorrect

**Blood pressure alone is not a
nutritional risk factor.**

Option D: Primigravida Mother – Incorrect

**Being a first-time mother does
not automatically indicate
nutritional risk.**

Final Answer: B. HIV Positive Mother

6. Ethical Principle in Ovarian Cancer Treatment

Option A: Nonmaleficence – Incorrect

Nonmaleficence means "do no harm," but forcing treatment violates autonomy.

Option B: Justice – Incorrect

Justice deals with fairness, not patient choice.

**Option C: Beneficence –
Correct**

The healthcare team acted in the best interest of the patient by saving her life, even though it led to infertility.

**Option D: Autonomy –
Incorrect**

Autonomy was overridden in favor of saving her life.

Final Answer: C. Beneficence

7. Malaria Type Causing Severe Symptoms in Pregnant Woman

**Option A: Plasmodium Ovale –
Incorrect**

Rare and mild.

**Option B: Plasmodium Vivax –
Incorrect**

Less severe than P. falciparum.

**Option C: Plasmodium
Malariae – Incorrect**

Not linked to severe malaria.

**Option D: Plasmodium
Falciparum – Correct**

P. falciparum causes severe malaria in pregnancy, including hyperparasitemia and anemia.

Final Answer: D. Plasmodium Falciparum

8. Reason for Low Latrine Coverage

Option A: Interest and Support of Authorities – Not the main issue

The project was initiated, so authorities likely supported it.

Option B: Availability of Resources – Incorrect

The project provided materials, so resource shortage is unlikely.

Option C: Low Community Acceptability – Correct

If people do not accept or prioritize latrines, they won't use them even when available.

Option D: Complexity of the Problem – Incorrect

Building latrines is not very complex, but acceptance is key.

**Final Answer: C. Low
Community Acceptability**

9. Recommended Treatment for Oral Candidiasis in a 5- Month-Old

Option A: Acyclovir – Incorrect

**Acyclovir treats viral
infections, not fungal.**

**Option B: Amoxicillin –
Incorrect**

**Amoxicillin is an antibiotic,
ineffective for fungal
infections.**

**Option C: Prednisolone –
Incorrect**

**Steroids worsen fungal
infections.**

Option D: Nystatin – Correct

Nystatin is the first-line antifungal for oral candidiasis (thrush).

Final Answer: D. Nystatin

10. Reason to Accuse a Nurse for Giving Medication Early

Option A: Omission of Care – Incorrect

**The medication was given, not
omitted.**

Option B: Negligence – Incorrect

**Negligence refers to not
performing duty, which is not
the case here.**

Option C: Commission of Care – Correct

**Administering medication
earlier than prescribed can
lead to dosing errors.**

Option D: Breach of Duty – Incorrect

**Breach of duty applies if the
nurse completely ignored care.**

**Final Answer: C. Commission
of Care**

**11. Health Education Approach
for Family Planning**

**Option A: Informed Decision-
Making Approach – Correct**

The nurse provided full information, allowing the woman to choose independently.

**Option B: Persuasion Approach
– Incorrect**

Persuasion means convincing a patient to choose a method.

Option C: Deliberate Attempt Approach – Incorrect

This approach involves influencing choices, which was not done here.

Option D: Directive Approach – Incorrect

Directive means the nurse directly suggests a method, which was not the case.

Final Answer: A. Informed Decision-Making Approach

12. Priority Nursing Intervention for a Woman with VVF and Body Image Issues

**Option A: Provide Information
on Self-Care – Not the first
priority**

Self-care is important, but emotional support is more urgent.

**Option B: Empower
Economically – Not directly relevant**

Financial empowerment is beneficial but not the priority.

**Option C: Encourage a
Balanced Diet – Not the
priority**

**Diet does not address body
image concerns.**

**Option D: Teach Positive Self-
Talk – Correct**

**Positive self-talk helps restore
confidence and improve body
image.**

Final Answer: D. Teach the Woman About Positive Self-Talk

1. Legal Requirement for a Nurse to Practice

Option A: Certification Evidence – Incorrect

Certification verifies completion of education but

**does not grant legal
permission to practice.**

**Option B: Registration
Evidence – Incorrect**

**Registration means being
listed in a professional body,
but it is not a legal permit to
practice.**

**Option C: License Evidence –
Correct**

A license is the legal document issued by a government agency that authorizes a nurse to practice.

**Option D: Accreditation
Evidence – Incorrect**

Accreditation applies to educational institutions, not individuals.

Final Answer: C. License Evidence

2. Diagnosis of a Baby with Lethargy and Weak Reflexes After Preterm Birth

Option A: Asphyxia – Correct

Birth asphyxia occurs when the baby does not get enough oxygen at birth, leading to lethargy, weak Moro reflex, and no crying.

**Option B: Hypothermia –
Incorrect**

Hypothermia causes shivering and cold skin, not weak reflexes.

Option C: Hypoxia – Incorrect

Hypoxia is low oxygen in tissues, but birth asphyxia is the specific cause in newborns.

Option D: Hypoxemia – Incorrect

Hypoxemia is low blood oxygen, but asphyxia involves both oxygen and blood flow issues.

Final Answer: A. Asphyxia

3. Core Symptoms of Depression

Option A: Depressed Mood and/or Loss of Interest – Correct

Depression is diagnosed based on persistent sadness (depressed mood) and loss of interest in activities.

Option B: Hopelessness and/or Distractibility – Incorrect

Hopelessness is common but not a primary diagnostic criterion.

Option C: Distractibility and/or Sleep Disturbance – Incorrect

These are symptoms but not core diagnostic criteria.

Option D: Loss of Interest and/or Hopelessness – Incorrect

Hopelessness is not a core symptom, but loss of interest is.

**Final Answer: A. Depressed
Mood and/or Loss of Interest**

**4. Best Intervention for an
Elderly Patient with Memory
Loss and Weight Loss**

Option A: Provide Psychological and Food Services – Correct

**The patient has weight loss
and memory issues, so
nutritional support and mental
health care are priorities.**

**Option B: Participate in
Exercise and Fitness Programs
– Incorrect**

Exercise is beneficial but not the priority.

Option C: Engage in Recreational and Educational Programs – Incorrect

These are good for cognitive function but won't address weight loss.

Option D: Facilitate Hearing Aid and Hearing-Aid Usage – Incorrect

No mention of hearing problems.

Final Answer: A. Provide Psychological and Food Services

5. Most Likely Fetal Outcome for a Malnourished Pregnant Woman

**Option A: Infant Who Initiates
Breastfeeding Successfully –
Incorrect**

**Malnutrition can affect feeding
ability, so this is unlikely.**

Option B: Infant with Low Weight for Gestational Age – Correct

Poor maternal nutrition leads to intrauterine growth restriction (IUGR), causing low birth weight.

Option C: Infant with Large Weight for Gestational Age – Incorrect

Poor maternal nutrition does not lead to large babies.

Option D: Infant with Normal Weight for Gestational Age – Incorrect

Malnourished mothers are unlikely to have normal-weight babies.

Final Answer: B. Infant with Low Weight for Gestational Age

6. Priority Nursing Outcome for Anemia in Pregnancy

Option A: Having Hgb Greater than 13 g/dL – Incorrect

The goal is to reach normal levels (above 11 g/dL), not excessively high.

Option B: Free from Anxiety – Incorrect

Addressing physical health (anemia) is the priority.

Option C: Subside from Pale Discoloration – Incorrect

**Paleness improves gradually,
but hemoglobin levels matter
more.**

**Option D: Free from Headache
– Correct**

**Headache in anemia is due to
low oxygen delivery. Resolving
it indicates improvement.**

Final Answer: D. Free from Headache

7. Key Sign of Foreign Body Aspiration in a Child

Option A: Gagging – Incorrect

**Gagging occurs in initial stages,
not for confirming aspiration.**

**Option B: Signs of Infection –
Incorrect**

**Infection occurs later, not as
an immediate sign.**

**Option C: Hemoptysis
(Coughing Blood) – Incorrect**

Rare in foreign body aspiration.

Option D: Atelectasis (Lung Collapse) – Correct

Atelectasis occurs when a foreign object blocks the airway, leading to lung collapse.

Final Answer: D. Atelectasis

8. First-Line Drug for Severe Malaria with Coma in a Child

Option A: Quinidine – Incorrect

Quinidine is used in adults, not children.

Option B: Artesunate – Correct

IV artesunate is the first-line treatment for severe P. falciparum malaria.

Option C: Artemether – Incorrect

Artemether is an option but not the first-line drug for severe cases.

**Option D: Quinine
Dihydrochloride – Incorrect**

**Quinine is an alternative but
not preferred over artesunate.**

Final Answer: B. Artesunate

9. Likely Complication of Measles in a Child

(Duplicate Question, Answer Already Given Above)

Final Answer: D. Pneumonia

10. Most Likely Cause of Sustained Fever with Rose Spots in a Child

Option A: Giardiasis – Incorrect

Giardiasis causes diarrhea, not sustained fever.

Option B: Typhoid Fever – Correct

Typhoid fever presents with rose spots, fever, and relative bradycardia.

Option C: Amoebiasis – Incorrect

Amoebiasis causes bloody diarrhea, not rose spots.

Option D: Bacillary Dysentery – Incorrect

Dysentery presents with severe diarrhea, not rose spots.

Final Answer: B. Typhoid Fever

11. Priority Nursing Intervention for a Woman with VVF

**(Duplicate Question, Answer
Already Given Above)**

**Final Answer: D. Teach the
Woman About Positive Self-
Talk**

**12. Reason for Failure of
Latrine Coverage Project**

Option A: Interest and Support of Authorities – Incorrect

Authorities initiated the project, so lack of interest is unlikely.

Option B: Availability of Resources – Incorrect

Resources were provided, so this is not the main issue.

Option C: Low Community Acceptability – Correct

If the community does not see latrines as necessary, they won't use or build them, leading to project failure.

Option D: Complexity of the Problem – Incorrect

Building latrines is not a complex problem, but acceptance is crucial.

Final Answer: C. Low Community Acceptability

1. Management of Magnesium Sulfate Toxicity in Severe Pre-eclampsia

Option A: Change the Drug to Diazepam – Incorrect

Diazepam is an alternative anticonvulsant for eclampsia but does not reverse magnesium toxicity.

Option B: Provide Calcium Gluconate – Correct

The patient has absent reflexes, low urine output, and respiratory depression, indicating magnesium toxicity.

**Calcium gluconate is the
antidote to reverse toxicity.**

**Option C: Continue
Administering Magnesium
Sulfate – Incorrect**

**Magnesium sulfate should be
stopped immediately due to
toxicity signs.**

**Option D: Resuscitate the
Mother – Incorrect**

**Resuscitation is for
cardiopulmonary arrest, which
is not the case here.**

**Final Answer: B. Provide
Calcium Gluconate**

2. Prioritized Nursing Diagnosis for Anemia in Pregnancy

Option A: Having Hgb Greater than 13 g/dL – Incorrect

The target is above 11 g/dL, but focusing on excessive levels is unnecessary.

Option B: Free from Anxiety – Incorrect

**Anxiety is a concern, but
physical health is the priority.**

**Option C: Subside from Pale
Discoloration – Incorrect**

**Paleness improves gradually,
but it's not the most critical
outcome.**

**Option D: Free from Headache
– Correct**

Headache in anemia is due to low oxygen supply, so relief suggests improvement.

Final Answer: D. Free from Headache

3. Missing Criterion for PTSD Diagnosis

**Option A: Recurrent
Experiences of Feeling
Detached from Self – Incorrect**

**Dissociation can occur in PTSD,
but it's not mandatory for
diagnosis.**

**Option B: Efforts to Avoid
Trauma-Related Cues –
Incorrect**

Avoidance is a key symptom in PTSD.

Option C: Direct Exposure to Trauma or Witnessing It – Correct

To diagnose PTSD, the patient must have directly experienced or witnessed trauma. This case lacks clear evidence of direct exposure.

**Option D: Recurrent
Experiences of Unreality of
Surroundings – Incorrect**

**Derealization can happen in
PTSD but is not required for
diagnosis.**

**Final Answer: C. Direct
Exposure to Traumatic Event**

4. Most Likely Sign of Prolonged Labor

**Option A: Uterine Contraction
– Incorrect**

Contractions can be weak or strong, but do not define prolonged labor.

**Option B: Cervical Dilation –
Correct**

The cervix has only dilated 4 cm in 14 hours, which is too slow, indicating prolonged labor.

**Option C: Status of Membrane
– Incorrect**

Membrane rupture alone does not indicate labor progress.

Option D: Station – Incorrect

Station describes fetal descent but does not define labor duration.

Final Answer: B. Cervical Dilation

5. Key Criterion for Alcohol-Induced Depressive Disorder

Option A: Clinically Significant Distress – Incorrect

Distress is common but not unique to alcohol-induced depression.

Option B: Symptoms Developed During Substance Withdrawal – Correct

Depressive symptoms appeared after stopping alcohol, indicating alcohol-induced depression.

Option C: Persistent Depressed Mood – Incorrect

Depression can exist without alcohol withdrawal, so this is not diagnostic.

Option D: Drinking Alcohol for Five Years – Incorrect

Chronic alcohol use is relevant, but the timing of symptoms is crucial.

Final Answer: B. Symptoms Developed During Substance Withdrawal

6. Key Teaching Point for Substance Use Disorder Recovery

**Option A: Substance Use is Not
Related to Individual
Weakness – Incorrect**

**While true, it does not directly
aid recovery motivation.**

Option B: Focusing on Motivation for Change is Best – Correct

**Patients often deny problems,
so enhancing motivation for
change is the best approach.**

**Option C: Communicating That
Substances Can Relieve Stress
– Incorrect**

This encourages continued use.

Option D: Stating That Recovery is Possible Without Treatment – Incorrect

Most people need structured treatment.

Final Answer: B. Focus on Motivation for Change

7. Most Likely Nursing Diagnosis for Neonatal Sepsis

**Option A: Ineffective Airway
Clearance – Incorrect**

**No evidence of airway
obstruction.**

Option B: High Risk for Hypoventilation – Incorrect

The baby has tachypnea, not hypoventilation.

Option C: Ineffective Tissue Perfusion – Correct

Sepsis causes poor oxygen delivery to tissues, leading to cyanosis, tachycardia, and tachypnea.

Option D: Ineffective Breathing Pattern – Incorrect

The baby's breathing is rapid, but perfusion is the critical issue.

Final Answer: C. Ineffective Tissue Perfusion

8. Diagnosis of a Man with Chronic Khat Use and Social Dysfunction

Option A: Stimulant Use Disorder – Correct

The patient has compulsive use despite social problems, meeting criteria for substance use disorder.

**Option B: Stimulant
Withdrawal – Incorrect**

No withdrawal symptoms are reported.

**Option C: Unspecified
Stimulant-Related Disorder –
Incorrect**

**The case meets full criteria for
substance use disorder.**

Option D: Stimulant Intoxication – Incorrect

Intoxication refers to acute effects, but this case describes chronic use.

Final Answer: A. Stimulant Use Disorder

9. Site for Implanon Administration

**Option A: Deltoid Muscle –
Incorrect**

**Implanon is not given
intramuscularly.**

Option B: Inner Side of Upper Arm – Correct

Implanon is inserted subdermally on the inner upper arm for long-term contraception.

Option C: Outer Side of the Thigh – Incorrect

Not the recommended site.

**Option D: Gluteal Muscle –
Incorrect**

**Implanon is not an
intramuscular injection.**

**Final Answer: B. Inner Side of
Upper Arm**

10. Management of Ear Infection in a Child per IMNCI Guidelines

Option A: Give Ampicillin and Refer – Incorrect

Ampicillin is for severe pneumonia, not first-line for ear infections.

Option B: Treat with Amoxicillin for 5 Days – Correct

IMNCI recommends oral amoxicillin for mild ear infections.

Option C: Treat with Quinolone Ear Drops for 2 Weeks – Incorrect

Ear drops are not first-line treatment.

**Option D: Give
Chloramphenicol and Refer –
Incorrect**

**Chloramphenicol is for severe
infections, not ear infections.**

**Final Answer: B. Treat with
Amoxicillin for 5 Days**

11. Managerial Role in a Measles Outbreak

Option A: Spokesperson – Correct

A spokesperson reports information to the public.

Option B: Disseminator – Incorrect

**Disseminators share
information within the
organization, not externally.**

Option C: Liaison – Incorrect

**Liaisons coordinate with
external stakeholders.**

Option D: Monitor – Incorrect

Monitors analyze internal information, not publicize it.

Final Answer: A. Spokesperson

I'll continue with the remaining questions if you'd like!

1. Management of Ear Infection in an 18-Month-Old Child According to IMNCI

Option A: Give First Dose of Ampicillin and Refer – Incorrect

Ampicillin is used for severe pneumonia, not first-line for ear infections.

**Option B: Treat with
Amoxicillin for 5 Days – Correct**

**According to IMNCI guidelines,
acute ear infections with pus
drainage for less than 14 days
should be treated with oral
amoxicillin for 5 days.**

**Option C: Treat with
Quinolone Ear Drop for 2
Weeks – Incorrect**

Quinolone ear drops are used for chronic or recurrent infections but are not the first-line treatment for this case.

Option D: Give First Dose of Chloramphenicol and Refer – Incorrect

Chloramphenicol is used for severe sepsis, not ear infections.

Final Answer: B. Treat with Amoxicillin for 5 Days

2. Mean CD4 T Cell Count of HIV Patients

We calculate the mean CD4 count:

$$\frac{230 + 205 + 313 + 207 + 227 + 245 + 173 + 58 + 103 + 181 + 105 + 301 + 169}{13} = 193.6$$

Final Answer: B. 193.6

**3. Best Graph for Presenting
Malaria Trends Over Five Years**

Option A: Line Graph – Correct

A line graph is best for showing trends over time, making it the most appropriate choice for malaria trends.

Option B: Histogram – Incorrect

Histograms are used for frequency distributions, not trends.

Option C: Scatter Plot – Incorrect

Scatter plots are for relationships between two variables, not trends.

Option D: Frequency Polygon – Incorrect

Frequency polygons show distribution patterns, not trends over time.

Final Answer: A. Line Graph

4. Management of a Four-Year-Old with Diarrhea per IMNCI

**Option A: Give ORS Solution
for Each Loss – Incorrect**

**While ORS is essential, the
initial management requires
structured rehydration over 4
hours.**

**Option B: Give RL Solution
Intravenously Over 3 Hours –
Incorrect**

Rapid IV fluids are for severe dehydration, but this child shows moderate dehydration.

Option C: Give ORS Over a 4-Hour Period – Correct

This is the recommended IMNCI approach for moderate dehydration.

**Option D: Give ReSoMAL
Solution for Each Loss –
Incorrect**

**ReSoMAL is used for severe
malnutrition with dehydration,
not for this case.**

**Final Answer: C. Give ORS Over
a 4-Hour Period**

5. Nurse's Action for a Patient Threatening to Spread HIV

Option A: Tell His HIV Status to the Emergency Nurse – Incorrect

This violates patient confidentiality.

Option B: Tell His Status to the Management Team – Incorrect

Confidentiality should be maintained unless there is immediate public harm.

Option C: Accuse Him of Endangering Public Health – Incorrect

While the statement is concerning, direct accusations are not the best first step.

**Option D: Further
Communication and
Counseling with the Patient –
Correct**

**The nurse should engage in
further counseling to help the
patient understand the risks of
spreading HIV and encourage
safer behavior.**

**Final Answer: D. Further
Communication and
Counseling with the Patient**

**6. Conflict Resolution Strategy
Used by the Managers**

**Option A: Resolution
Aftermath – Incorrect**

This applies to managing conflicts after they are settled, but the managers were actively involved in negotiation.

Option B: Lose-Lose – Incorrect

In this case, both sides gained benefits, so it was not a lose-lose situation.

Option C: Win-Lose – Incorrect

Both parties reached an agreement, meaning one did not lose entirely.

Option D: Win-Win – Correct

The health staff continued working, and they gained extra income from private clinics, making this a win-win outcome.

Final Answer: D. Win-Win

7. Diagnosis of a Child with Social and Communication Deficits

Option A: Down Syndrome – Incorrect

Down syndrome is associated with physical and intellectual disabilities, not repetitive behaviors and social withdrawal.

Option B: Autism Spectrum Disorder – Correct

The child exhibits social withdrawal, repetitive behavior, and communication

deficits, which are key features of autism spectrum disorder (ASD).

Option C: Childhood Depression – Incorrect

Depression does not typically involve repetitive behaviors or communication issues.

Option D: Attention Deficit Disorder – Incorrect

**ADHD is characterized by
hyperactivity and impulsivity,
not social withdrawal.**

**Final Answer: B. Autism
Spectrum Disorder**

8. Best Question to Encourage Open Discussion in Counseling

Option A: "What Do You Want to Talk About?" – Incorrect

Too broad and might not encourage deeper discussion.

Option B: "How Have Things Been Going?" – Correct

This is an open-ended question, encouraging the patient to share more details about their experience.

Option C: "Can We Discuss Your Partners and Friends?" – Incorrect

This is too specific and might make the patient uncomfortable.

Option D: "How Many Partners Do You Have?" – Incorrect

This is a closed question that does not encourage discussion.

Final Answer: B. How Have Things Been Going?

9. Most Likely Diagnosis for a Hyperactive Child with Attention Issues

Option A: Down Syndrome – Incorrect

Does not fit the behavioral profile described.

Option B: Childhood Schizophrenia – Incorrect

Schizophrenia in children is rare and involves hallucinations, which are not mentioned.

**Option C: Attention Deficit Hyperactivity Disorder (ADHD)
– Correct**

The child shows hyperactivity, inattentiveness, impulsivity, and difficulty following

instructions, all consistent with ADHD.

Option D: Mental Retardation Disorder – Incorrect

Intellectual disability does not explain hyperactivity and impulsivity.

Final Answer: C. Attention Deficit Hyperactivity Disorder (ADHD)

10. Most Likely Danger Sign in a Two-Year-Old Child

Option A: Cough – Incorrect

Cough is common but not a critical danger sign.

Option B: Vomiting Everything – Correct

Persistent vomiting prevents fluid intake, leading to dehydration and metabolic disturbances.

**Option C: Convulsion –
Incorrect**

The child had a past convulsion but is not actively convulsing now.

Option D: Fever – Incorrect

Temperature 36.5°C is normal, so fever is not a present danger sign.

**Final Answer: B. Vomiting
Everything**

11. Patient Right Breached by the Nurse

Option A: Right to Get Information – Correct

The nurse should have explained treatment options instead of assuming the patient knew.

**Option B: Right to Respect –
Incorrect**

**Respect was not explicitly
violated.**

**Option C: Right to Control
One's Life – Incorrect**

**The patient was not forced
into treatment.**

Option D: Right to Choose Treatment – Incorrect

Choosing treatment requires informed consent, which comes after information.

Final Answer: A. Right to Get Information

1. Reason for ICU Nurse Consulting the Doctor About a Medication Order

Option A: Since He is Newly Assigned, He is Not Clear with ICU Procedures – Incorrect

While a newly assigned doctor may be unfamiliar with procedures, the nurse's primary concern is patient safety, not the doctor's experience level.

**Option B: Fears Not to Take
Accountability for Any Bad
Consequence – Incorrect**

**Nurses are accountable for
their actions, but fear of blame
is not the main reason for
questioning a wrong order.**

**Option C: Lacks Confidence to
Work Independently as a
Professional Nurse – Incorrect**

The nurse is demonstrating professionalism and responsibility, not lack of confidence.

Option D: To Prevent Malpractices That Risk Patients and to Safeguard Them – Correct

The nurse's primary role is patient safety. Consulting the doctor before administering a

**doubtful medication helps
prevent potential medication
errors.**

**Final Answer: D. To Prevent
Malpractices That Risk the
Patients and to Safeguard
Them**

2. Most Likely Outcome for a 68-Year-Old Woman with Progressive Cognitive Decline

Option A: Eventual Partial Return of Premorbid Level of Functioning – Incorrect

Cognitive disorders like dementia are progressive and irreversible, meaning partial recovery is unlikely.

Option B: Eventual Full Return to Premorbid Level of Functioning – Incorrect

**Full recovery is not possible in
degenerative conditions like
Alzheimer's disease or vascular
dementia.**

Option C: Peaceful Death Within a Short Period of Time – Incorrect

Dementia is progressive but not immediately fatal. Life expectancy varies based on the type and stage of dementia.

Option D: Behavioral Control with Delaying of Cognitive Deterioration – Correct

Treatment focuses on managing symptoms and slowing cognitive decline

**through medications and
supportive care.**

**Final Answer: D. Behavioral
Control with Delaying of
Cognitive Deterioration**

**3. Most Appropriate
Prevention Strategy for Scabies
in the Community**

**Option A: Prevent Swimming
and Crossing in Infected Water
– Incorrect**

**Scabies is not waterborne; it
spreads through direct skin
contact and contaminated
fabrics.**

**Option B: Discharging the
Accumulated Water from the
Area – Incorrect**

Stagnant water is relevant for mosquito-borne diseases like malaria, but not scabies.

Option C: Avoid Drinking Contaminated Water – Incorrect

Scabies does not spread through contaminated water.

Option D: Provision of Adequate Water for Personal Hygiene – Correct

Scabies spreads due to poor hygiene and overcrowding. Regular washing of clothes, bedding, and skin helps prevent transmission.

Final Answer: D. Provision of Adequate Water for Personal Hygiene

4. Most Likely Indicative Characteristic of a Pregnant Woman with Fatigue and Dyspnea

Option A: Dyspnea – Incorrect

Dyspnea is a symptom but not specific to anemia. It can occur due to pregnancy-related changes.

Option B: Pale Conjunctiva – Correct

Pale conjunctiva is a key sign of anemia, which is common in pregnancy due to increased iron demands.

**Option C: Hgb 10 g/dl –
Incorrect**

**Although Hgb 10 g/dl is low, it
is not severe enough to be the
main indicative characteristic.**

**Option D: Headache –
Incorrect**

**Headaches can have many
causes, such as preeclampsia**

or stress, and are not specific to anemia.

**Final Answer: B. Pale
Conjunctiva**

**5. Most Likely Strategy to
Prevent Malaria in a
Community**

Option A: Treatment of an Individual Who Develops the Disease – Incorrect

Treatment is important but does not prevent new cases.

Option B: Avoid Drinking Contaminated Water – Incorrect

Malaria is not waterborne; it is transmitted by mosquito bites.

Option C: Isolation of an Individual Who Develops the Disease – Incorrect

Malaria is not contagious from person to person, so isolation is not necessary.

**Option D: Discharging the
Accumulated Water from the
Area – Correct**

**Malaria prevention focuses on
eliminating mosquito breeding
sites by removing stagnant
water.**

**Final Answer: D. Discharging
the Accumulated Water from
the Area**

6. IMNCI Classification of a Two-Day-Old Neonate with Yellow Discoloration

Option A: Jaundice – Correct

Jaundice in newborns is common and classified based on severity.

Option B: Physiological Jaundice – Incorrect

Physiological jaundice typically starts after 24 hours and peaks by days 3–5. The case does not confirm if it is physiological.

Option C: Severe Jaundice – Incorrect

Severe jaundice extends beyond palms/soles or occurs within 24 hours, which is not indicated here.

Option D: Uncomplicated Jaundice – Incorrect

"Uncomplicated jaundice" is not a standard IMNCI classification.

Final Answer: A. Jaundice

7. Appropriate Action for an HIV-Positive Patient Threatening to Spread the Virus

Option A: Accuse Him of Endangering Public Health – Incorrect

Direct accusations could worsen the patient's reaction.

Option B: Further Communication and Counseling with the Patient – Correct

The nurse should engage in counseling to help the patient understand the risks and ethical responsibilities of transmission.

Option C: Tell His HIV Status to the Emergency Nurse – Incorrect

Disclosing his status without consent is a breach of confidentiality.

Option D: Tell His Status to the Management Team – Incorrect

Confidentiality should be maintained unless there is an immediate public health risk.

Final Answer: B. Further Communication and Counseling with the Patient

8. Problem Prioritization Criterion for Addressing Hospital Stock-Outs

**Option A: Magnitude –
Incorrect**

**Magnitude refers to how
widespread the problem is, but
the focus here is on
community concerns.**

Option B: Sustainability – Incorrect

Sustainability means long-term impact, which is not the focus of the discussion.

Option C: Community Concern – Correct

**The community leaders
expressed concern about**

stock-outs, making this the key prioritization criterion.

**Option D: Feasibility –
Incorrect**

Feasibility refers to how easily the issue can be addressed, but the decision is based on community input.

Final Answer: C. Community Concern

9. Pregnant Woman Who Requires Nutritional Counseling the Most

**Option A: Primigravida Mother
– Incorrect**

Primigravida mothers may need general counseling but not necessarily more than others.

**Option B: HIV-Positive Mother
– Correct**

HIV-positive mothers have higher nutritional needs and are at greater risk of malnutrition, making them the priority for counseling.

**Option C: RH-Negative Mother
– Incorrect**

**RH-negative status is
important for blood
compatibility, not nutrition.**

**Option D: Normotensive
Mother – Incorrect**

Normal blood pressure does not indicate higher nutritional risk.

Final Answer: B. HIV-Positive Mother

First Action When a Nurse Observes a Medication Administration Error

Option A: Confront His Friend – Incorrect

While discussing the error with the colleague may be beneficial, confrontation is not the first step. The priority is patient safety and proper reporting to ensure accountability and prevent future errors.

Option B: Follow Secretly and Supervise – Incorrect

Supervising the nurse without addressing the incident does not resolve the problem.

Errors should be reported and proper corrective actions should be taken to maintain patient safety.

Option C: Report to the Nurse Director – Correct

The first step is to report the error to the nurse director or

**appropriate supervisor,
following the hospital's
incident reporting protocol.
This ensures that:**

The error is documented

**Proper investigation and
corrective measures are taken**

**Patient safety is prioritized,
and similar errors are
prevented in the future**

Option D: Report to the Legal Judiciary Body – Incorrect

Legal action is not the first step, especially for a minor complication that was treated. The issue should be handled internally first, unless it involves gross negligence or malpractice.

Final Answer: C. Report to the Nurse Director

Principle Applied in the Case of Amputation Decision

Option A: Compassion – Incorrect

Compassion refers to showing empathy and care towards a patient, but it is not the primary ethical principle

guiding the nurse's decision in this scenario. While the nurse may act compassionately, the focus is on decision-making for the patient's survival.

**Option B: Paternalism –
Correct**

Paternalism occurs when a healthcare provider makes a decision for the patient's benefit, even if it overrides

their autonomy. In this case, the nurse persuades the patient to accept amputation, prioritizing his life over cultural beliefs. This is a classic example of paternalism, where the healthcare provider acts in the patient's best interest despite their initial refusal.

Option C: Veracity – Incorrect

Veracity refers to truthfulness and honesty in communication. While the nurse might be truthful in explaining the necessity of the procedure, the primary ethical concern here is about making a decision for the patient's benefit rather than just providing truthful information.

**Option D: Proportionality –
Incorrect**

Proportionality refers to balancing risks and benefits, often used in ethical dilemmas where interventions must be justified based on their consequences. While this concept applies generally in medical ethics, the primary principle at play here is paternalism, since the nurse actively influences the patient's decision.

Final Answer: B. Paternalism

Effect of Increasing Confidence Level on Confidence Interval Width

Option A: Unchanged – Incorrect

The width of a confidence interval depends on the confidence level. If the confidence level increases, the interval must expand to

account for more possible values. Therefore, it cannot remain unchanged.

**Option B: Decreased –
Incorrect**

A higher confidence level means we want to be more certain that the interval contains the true population mean. To achieve this, we need a wider range to capture

more possible values, so the width does not decrease.

Option C: Increased – Correct

As the confidence level increases (from 90% to 99%), the confidence interval becomes wider. This happens because a higher confidence level requires more margin for error, leading to a broader range.

Option D: Precise – Incorrect

Higher confidence means less precision because the interval widens. If we want a narrower (more precise) interval, we would need to lower the confidence level or increase the sample size.

Final Answer: C. Increased

Question 1: Step of Planning Emphasized in the Quote

"In resource-poor countries like us, one does not go in search of a cure for ringworm while leaving leprosy unattended."

**Option A: Problem
Identification – Incorrect**

Problem identification is the first step, where various health issues are recognized.

However, the quote focuses on choosing which issue to address first, not just identifying problems.

**Option B: Situational Analysis
– Incorrect**

Situational analysis involves gathering data on existing

conditions. While important, this step does not directly involve choosing which problem to prioritize.

Option C: Problem Prioritization – Correct

The manager is emphasizing that in resource-limited settings, severe health issues (like leprosy) should be prioritized over less severe

ones (like ringworm). This aligns with problem prioritization, where health concerns are ranked based on severity, prevalence, and available resources.

Option D: Objective Setting – Incorrect

Objective setting occurs after prioritization, when specific goals and plans are developed.

The quote is about choosing priorities, not setting objectives.

Final Answer: C. Problem Prioritization

Question 2: Type of Screening Bias

An investigator finds that survival among screened individuals appears longer than for those not screened.

**Option A: Lead-time Bias –
Correct**

Lead-time bias occurs when a disease is detected earlier through screening, making it

appear as if survival time is longer.

However, the patient's actual lifespan may not change—it just seems longer because diagnosis happened sooner.

**Option B: Recall Bias –
Incorrect**

Recall bias occurs when participants have differences in memory recall, often in retrospective studies.

This scenario does not involve memory issues, so it's not recall bias.

Option C: Length-biased Sampling – Incorrect

Length-biased sampling occurs when screening detects milder, slow-progressing cases more often than severe cases, leading to overestimation of survival rates.

This bias is possible, but lead-time bias is a more direct explanation here.

Option D: Volunteer Bias – Incorrect

Volunteer bias happens when people who choose to get screened are often healthier than those who don't.

While this may contribute to survival differences, the primary issue in this case is early detection creating the illusion of longer survival.

**Final Answer: A. Lead-time
Bias**

**Question 3: Grade of Open
Fracture**

A 45-year-old with an open fracture, 5 cm wound, and minor soft tissue damage.

Gustilo-Anderson Open Fracture Classification:

**Grade I: Wound < 1 cm,
minimal soft tissue damage**

**Grade II: Wound 1–10 cm,
moderate soft tissue damage**

**Grade III: Wound >10 cm,
extensive soft tissue damage**

**III A: Adequate soft tissue
coverage**

III B: Requires flap coverage

**III C: Vascular injury requiring
repair**

Correct Answer: Grade II

The 5 cm wound and minor soft tissue damage fit Grade II criteria.

More severe injuries (large wounds, extensive soft tissue loss) would be Grade III.

Final Answer: Grade II

**Question 4: Focus of
Situational Analysis in
PRECEDE-PROCEED Model**

**A health educator assesses
resources, budget,
implementation timeline,**

**coordination, and
organizational circumstances.**

Option A: Epidemiological Diagnosis – Incorrect

**This step focuses on disease
burden and health conditions
in the population, not
organizational resources.**

Option B: Social Diagnosis – Incorrect

**Social diagnosis examines
community needs,
perceptions, and quality of life
factors.**

**It's more about social and
cultural influences, not
organizational readiness.**

Option C: Administrative and Policy Diagnosis – Correct

This step in the PRECEDE-PROCEED Model evaluates organizational capacity, funding, policies, and logistics—which matches the question details.

Option D: Educational Diagnosis – Incorrect

This step looks at knowledge, attitudes, and behaviors affecting health, not administrative resources.

**Final Answer: C.
Administrative and Policy
Diagnosis**

Question 1: Nursing Intervention for Bilateral Purulent Nasal Discharge

**A 19-year-old male with
bilateral purulent nasal
discharge for one week,
halitosis, peri-orbital pain, and
no cervical adenopathy.**

**He did not improve after a full
course of Amoxicillin, which**

suggests a possible chronic or resistant bacterial sinusitis.

**Option A: Antiviral Treatment
– Incorrect**

This is not a viral infection. The persistent purulent discharge suggests a bacterial sinus infection, making antiviral treatment ineffective.

Option B: Nasal Irrigation Using Normal Saline – Correct

Nasal irrigation with normal saline helps clear thick mucus, reduce inflammation, and improve drainage.

This is a first-line supportive treatment for chronic sinusitis.

Option C: Steroid-Based Nasal Decongestants – Incorrect

Steroid nasal sprays help in allergic or inflammatory conditions but are not the first-line treatment for bacterial sinusitis.

Option D: Antihistamine Treatment – Incorrect

Antihistamines help with allergic rhinitis, but they thicken mucus and can worsen sinusitis symptoms.

Final Answer: B. Nasal Irrigation Using Normal Saline

Question 2: Most Likely Diagnosis for a Patient with Purulent Urethral Discharge and Genital Ulcer

A 27-year-old male with:

**Acute onset of profuse,
purulent urethral discharge**

**Urgency, itching, and burning
during urination**

**Painless, regular ulcer on the
genitalia**

Option A: Syphilis – Correct

**Primary syphilis presents with
a painless, regular ulcer
(chancre).**

**Secondary syphilis may cause
urethritis, but purulent
discharge is uncommon.**

Confirm with RPR/VDRL tests and Treponema pallidum serology.

Option B: Herpes Genitalis – Incorrect

Herpes causes painful, multiple, vesicular ulcers, not painless regular ulcers.

Option C: Chancroid – Incorrect

Chancroid (*Haemophilus ducreyi*) causes painful, irregular ulcers with purulent base, unlike the painless ulcer in this patient.

Option D: Gonorrhea – Incorrect

Gonorrhea can cause profuse, purulent discharge, but it does not cause painless ulcers.

Final Answer: A. Syphilis

Question 3: Most Important Nursing Action After Thyroid Surgery

**A 50-year-old male transferred
to the surgical ward after
thyroid surgery.**

**Option A: Monitoring
Symptoms of Any
Complication – Partially
Correct**

General monitoring is important, but specific complications should be targeted first.

Option B: Administering Calcium Gluconate – Correct

Thyroid surgery can accidentally damage the parathyroid glands, leading to hypocalcemia.

Signs of hypocalcemia include tetany, muscle spasms, and Chvostek/Trousseau signs.

Calcium gluconate is the immediate treatment for symptomatic hypocalcemia.

Option C: Monitoring Input and Output – Incorrect

Important for overall care, but not the priority after thyroid surgery.

Option D: Monitoring Hydration Status – Incorrect

Dehydration is not a primary concern after thyroid surgery.

**Final Answer: B. Administering
Calcium Gluconate**

**Question 4: Priority Nursing
Diagnosis for Chronic Otitis
Media**

**A 26-year-old with chronic
otitis media, experiencing:**

Purulent ear discharge

Tenderness around the mastoid

Reduced hearing

Option A: Impaired Tissue Integrity – Correct

Chronic infection damages the ear tissue, leading to persistent ear discharge and hearing loss.

Tissue damage and infection control should be the priority.

Option B: Pain – Incorrect

Pain is important but not the main concern in chronic otitis

media (more common in acute cases).

**Option C: Body Image
Disturbance – Incorrect**

Hearing loss may affect self-esteem, but tissue damage is more urgent.

Option D: Risk for Infection – Incorrect

The patient already has an infection, so controlling it is more critical than preventing a new one.

**Final Answer: A. Impaired
Tissue Integrity**

Question 5: Nursing Intervention for Acute Renal Failure with Low Phosphorus and Calcium

**A patient with acute renal
failure and low phosphorus
and calcium levels.**

**Option A: Assisting with an
Exercise Program – Incorrect**

Not a priority during acute renal failure.

Option B: Encourage Compliance with Fluid Restriction Therapy – Incorrect

Fluid balance is important, but low phosphorus and calcium require direct intervention.

Option C: Administer Blood Component Therapy – Correct

Hypocalcemia and low phosphorus in renal failure often need IV calcium and phosphate binders.

Severe electrolyte imbalances can cause neuromuscular dysfunction, cardiac

**arrhythmias, and seizures,
making correction essential.**

**Option D: Teaching to Report
Fluid Overload – Incorrect**

**Important for long-term care,
but electrolyte correction is
more urgent.**

Final Answer: C. Administer Blood Component Therapy

**I'll go through each question
one by one and provide the
correct answer with an
explanation.**

**1. Managerial Level of Nurse
Expressed in Training**

Scenario: A nurse attending a hospital reform training introduces herself as serving with skill and knowledge expected for her position.

Analysis:

First-level managers are direct-care providers (e.g., charge nurses, head nurses) who manage daily nursing tasks.

**Middle-level managers
oversee multiple units and
ensure policies are
implemented.**

**Top-level managers (e.g.,
nurse directors) handle
strategic planning.**

**Second-level is not a standard
managerial tier in nursing.**

Correct Answer: B. First Level

2. IMNCI Classification for Two-Month-Old Infant with Cough

Findings:

Cough for two days

**Respiration: 50/min
(tachypnea for <2 months:
>60/min is abnormal)**

**Normal pulse rate (110/min),
fever (38°C)**

**No danger signs (feeding well,
alert, no severe distress)**

Diagnosis Based on IMNCI:

Severe pneumonia → Danger signs (not alert, unable to feed) (Not present)

Very severe pneumonia → Severe distress (Not present)

**Pneumonia → Fast breathing
but no severe signs (Correct
choice)**

**Cough/cold → No fast
breathing (Incorrect)**

Correct Answer: C. Pneumonia

3. Likely Behavioral Disorder in 11-Year-Old Child

Symptoms:

Restlessness

Cannot follow instructions

Loses learning materials

Runs/climbs frequently

Diagnosis:

**ADHD (Attention Deficit
Hyperactivity Disorder)
involves hyperactivity,
impulsivity, and inattention,
which matches the case.**

Down syndrome involves cognitive delay and distinct facial features (not in the case).

Mental retardation (intellectual disability) involves low IQ and developmental delays (not purely behavioral).

Childhood schizophrenia presents with

hallucinations/delusions (not present).

Correct Answer: B. Attention Deficit Hyperactive Disorder

4. Consequence of Untreated Hernia with Abdominal Distention

Findings:

Abdominal pain, vomiting, constipation, high-pitched bowel sounds

History of hernia → Likely intestinal obstruction

If left untreated, it can lead to:

**Increased pain, dehydration,
and shock**

**Toxicity from bowel necrosis
(sepsis)**

Correct Answer: A. More intense pain and slowly dehydrated patient

5. Priority Nursing Intervention for Trauma Patient with Open Wounds & Bleeding

Findings:

**Open wounds, tenderness,
muscle spasm**

**Potential spinal injury → Risk
of further damage if moved**

**Priority: Immobilization first,
then pain management**

**Correct Answer: C.
Immobilization and Maintain
Spinal Alignment**

**6. Priority Teaching Plan for a
Diabetic Post-Cardiac Surgery
Patient**

**Risk factors for heart disease in
diabetes:**

**Smoking worsens
atherosclerosis**

**Diet and exercise help, but
smoking cessation is the
priority**

**Correct Answer: B. Promote
Cessation of Tobacco Use**

7. Effect of Nitroglycerine for Myocardial Infarction (MI) Patient

**Nitroglycerine: Vasodilator →
Reduces cardiac workload,
improves blood supply to the
heart**

Most direct effect:

**Dilates coronary blood vessels
(relieves angina)**

**Correct Answer: C. Dilate
Coronary Blood Vessels**

8. Type of Angina with Chest Pain at Rest & ST Elevation that Reversed

Stable angina: Occurs with exertion, relieved by rest (Incorrect)

**Unstable angina:
Unpredictable, worsening (No ST elevation, Incorrect)**

**Variant angina (Prinzmetal's):
Occurs at rest, causes ST
elevation, but reversible
(Correct)**

**Correct Answer: C. Variant
Angina**

9. Initial Step for 46-Year-Old with Severe Chest Pain & High Troponin

Findings:

Severe chest pain

Elevated troponin (MI likely)

**First step → Aspirin to prevent
clot progression**

**Correct Answer: D. Administer
Large Doses of Aspirin**

**10. Immediate Management
for Burned Child with**

Shortness of Breath & Weak Pulse

Severe burns → Fluid shift → Hypovolemic shock

**Airway compromise possible
→ First step is oxygen**

Correct Answer: C. Put on 100% Oxygen

11. Confirmatory Finding for Neonatal Hyperbilirubinemia

**Best marker: Yellow palms
(Jaundice spreading to
extremities = severe)**

**Correct Answer: A. Yellow
Palms**

12. Most Likely Cause of Watery, Rice-Water Diarrhea in a 5-Year-Old

**Vibrio cholerae → Causes
profuse, non-bloody, rice-
water diarrhea**

**Correct Answer: B. Vibrio
Cholerae**

**13. Indicator of HIV-Exposed
Infant at 6 Months**

**Best Indicator: Born from an
HIV-positive mother
(confirmed exposures**

**Correct Answer: D. Born from
Positive Mother**

**14. Best Drug for HIV-Exposed
Infant at Birth**

**Standard Prophylaxis:
Nevirapine for 6 weeks**

Correct Answer: B. Provide Nevirapine Prophylaxis for Six Weeks

15. Best Time for Antibody Testing in HIV-Exposed Infant

Antibody tests in infants are accurate after maternal antibodies wane (~12 weeks)

Correct Answer: D. At 12 Weeks

16. Likely Nursing Diagnosis for Tachycardic, Tachypneic Baby

Findings suggest: Respiratory distress → Ineffective breathing pattern

Correct Answer: A. Ineffective Breathing Pattern

17. Risk Factor for Hypothermia in a Neonate

**Best option: Being preterm
(poor thermoregulation)**

Correct Answer: A. Being Preterm

18. Priority Nursing Intervention for Neonate with Hypoglycemia & Sepsis

Hypoglycemia is life-threatening → Treat first

Correct Answer: B. Treat Hypoglycemia

19. Initial Step for Neonate with Conjunctivitis & Respiratory Distress

Breathing issue = oxygen first

**Correct Answer: C. Administer
Supplementary Oxygen**

**20. Likely Complication for
Meconium-Stained Newborn
with Respiratory Distress**

**Complication: Meconium
aspiration → Respiratory
distress → Shock**

Correct Answer: C. Shock

**Here is a detailed explanation
of each choice one by one:**

1. A woman came to a family planning unit with complaints of vomiting and severe dehydration for five days. She was using combined oral contraceptive for the past two months. Her blood pressure is 110/70 mmHg, pulse rate is 82/min, and temperature is 36.4°C. What is the likely nursing diagnosis for this woman?

(A) Risk for shock: Incorrect.
Shock is characterized by hypotension, tachycardia, and decreased perfusion. This woman has normal BP and pulse, so shock is unlikely.

(B) Risk for infection: Incorrect.
There is no indication of fever or an infection source.

(C) Risk for acute confusion: Incorrect. There is no sign of altered mental status.

(D) Risk for electrolyte imbalance: Correct. Vomiting for five days can lead to dehydration and electrolyte imbalances such as hypokalemia or hyponatremia, which could cause weakness and dizziness.

2. A 24-year-old woman came to a hospital to utilize a contraceptive method. The nurse provided counseling regarding the different contraceptive options available and their side effects. The nurse identified that the woman had understood their discussion. The woman showed a desire to use COC. What most likely response of

the woman that shows her understanding?

(A) She decides to change her idea: Incorrect. Changing her mind does not indicate understanding.

(B) She takes another appointment: Incorrect. Taking another appointment does not indicate comprehension.

(C) She lists contraceptive methods: Incorrect. Listing methods does not confirm understanding of side effects or usage.

(D) She asks a question about pregnancy: Correct. Asking a relevant question shows engagement and understanding.

3. A 30-year-old woman came to a family planning room to use contraceptive. The health care provider in the unit counseled her about methods, side effects, effectiveness, and time return. Finally, she chose to take Depo-Provera. What is the most appropriate site to administer this contraceptive?

(A) IM on the thigh: Correct. Depo-Provera is given intramuscularly in a large

muscle, typically the thigh or gluteal muscle.

**(B) IM on the abdomen:
Incorrect. Abdominal
injections are used for
subcutaneous medications, not
IM.**

**(C) IM on the right-side deltoid
muscle: Incorrect. The deltoid
is not a preferred site for deep
IM injections.**

(D) IM on the left-side deltoid muscle: Incorrect. Similar to the right deltoid, it is not ideal for deep IM injections.

4. An adolescent came to a pediatric OPD with complaints of fever, erythema, and worsening pain in the tibial bone. The culture results revealed methicillin-resistant

Staphylococcus aureus (MRSA). What is the most appropriate drug of choice for this patient?

(A) Nafcillin and cefotaxime: Incorrect. Nafcillin is ineffective against MRSA.

(B) Oxacillin and cefotaxime: Incorrect. Oxacillin does not work against MRSA.

(C) Vancomycin and oxacillin: Incorrect. Oxacillin is not used for MRSA.

(D) Vancomycin and cefotaxime: Correct. Vancomycin is the first-line treatment for MRSA infections.

5. A seven-year-old child was brought to a clinic with a complaint of vesicular lesions.

On assessment, the vesicles were tiny initially and rapidly changed to a honey-colored crusted plaque around the mouth. What is the most likely diagnosis?

(A) Impetigo: Correct. Impetigo is characterized by honey-colored crusts and is caused by Staphylococcus or Streptococcus bacteria.

(B) Cellulitis: Incorrect.

Cellulitis presents as deep tissue infection with swelling and redness, not vesicles.

(C) Tinea corporis: Incorrect.

This is a fungal infection that presents as ringworm.

(D) Varicella-zoster: Incorrect.

Chickenpox presents with widespread vesicular rash, not localized crusting.

6. A two-year-old male baby was brought to a health center with a complaint of barking cough, which worsens at night. He has no drooling or difficulty swallowing. What is the most likely diagnosis?

**(A) Whooping cough:
Incorrect. This is caused by
Bordetella pertussis and**

presents with severe coughing fits.

(B) Epiglottitis: Incorrect. Epiglottitis presents with drooling, high fever, and respiratory distress.

(C) Croup: Correct. Croup presents with a barking cough, mild fever, and worsens at night due to inflammation of the larynx and trachea.

(D) Pertussis: Incorrect.
Pertussis is different from croup and has a characteristic "whooping" sound.

7. A 62-year-old man with HIV/AIDS came to a hospital with lesions on the shoulder. The lesions followed a dermatome pattern with tingling and severe pain. What

is the most appropriate management?

**(A) Administer Tramadol:
Incorrect. Pain management is
needed, but antiviral therapy
is the priority.**

**(B) Administer antibiotics:
Incorrect. This is a viral
infection, not bacterial.**

(C) Administer Acyclovir:
Correct. Herpes zoster (shingles) is treated with acyclovir, especially in immunocompromised patients.

(D) Administer Calamine lotion: Incorrect. This helps with itching but does not treat the underlying infection.

8. A 59-year-old woman came to a medical OPD with complaints of abdominal pain, feeling of epigastric fullness, nausea, and belching. She has been taking NSAIDs for osteoarthritis. What is the most important additional question the nurse should ask?

(A) 'Have you ever noticed a change in odor of breath?'

Incorrect. Bad breath is not a key symptom.

(B) 'Have you noticed any problems with your breathing?' Incorrect.

Respiratory symptoms are not typical of NSAID-induced ulcers.

(C) 'Have you ever vomited blood or had very dark stools?'

Correct. NSAID use can cause gastric ulcers, which can lead to GI bleeding.

(D) 'Did you experience nausea always whenever you take the drug?' Incorrect. While NSAIDs can cause nausea, GI bleeding is a more serious concern.

9. A 22-year-old male patient has severe abdominal pain worsening at the right lower quadrant after 24 hours. Physical exam shows a rigid

abdomen and normal bowel sounds. What is the most likely complication?

(A) Perforation: Correct.
Perforated appendicitis can lead to peritonitis.

(B) Abscess: Incorrect.
Abscesses form over a longer period.

(C) Peritonitis: Incorrect. This is a possible consequence of perforation but not the direct cause.

(D) Infection: Incorrect. Infection is present but not the primary complication.

10. A 45-year-old female patient underwent extensive abdominal surgery. She has

multiple IV lines and was unconscious for several days. The nurse suspects pulmonary embolism (PE). What is the most likely assessment finding?

(A) Increased BP, chills, JVD, and painful cough: Incorrect. JVD is more typical of heart failure.

(B) Lethargy, nausea, vomiting, and diminished heart sounds: Incorrect. These are signs of other conditions.

(C) Fever, right upper quadrant pain, and dyspnea: Incorrect. Fever and RUQ pain are not classic for PE.

(D) Chest pain, shortness of breath, and tachypnea:

Correct. These are hallmark signs of PE.

Let's go through the options one by one and explain the rationale for each:

1. A 45-year-old male patient has come to an outpatient department with a complaint of loss of voice. Though he had a common cold, currently the symptom has improved. He

has no history of smoking and exposure to known irritants.

What is the immediate nursing intervention to regain the voice of the patient?

(A) Give Ant-ga (Antibiotics) – This is not appropriate, as the patient's loss of voice is likely due to a viral infection (common cold) and antibiotics are not effective for viral infections unless there is a secondary bacterial infection,

which is not mentioned in the history.

(B) Take vocal rest – This is the most appropriate intervention. Vocal rest can help the larynx recover if the voice loss is due to overuse or irritation.

(C) Give Ambionics (likely referring to some medication) – This is not specified enough and could be misleading;

medications are generally not the first line for voice loss unless a specific condition like a bacterial infection or reflux is identified.

(D) Increase fluid intake – Increasing fluid intake is beneficial for hydration and can help soothe the throat, but it will not directly help the voice to return as quickly as vocal rest would.

2. A 25-year-old male patient came to a clinic with complaints of irritation and fullness in the throat for four months. His sore throat worsens during swallowing and coughing. Additional investigations revealed that he has no thyroid gland disorder. What is the most appropriate intervention for this patient?

(A) Applying cold compression to support the sore throat –

Cold compresses can provide temporary relief for throat discomfort, but it does not address the underlying cause. This may be more suitable for acute sore throat rather than long-term irritation.

(B) Advising the patient to make oral gargling – Gargling with warm salt water may be beneficial for soothing a sore throat, but it would not

address the underlying cause of irritation and fullness.

(C) Providing antihistamine medication – This could be appropriate if the throat issue is related to an allergic reaction, but it's unclear from the information whether allergies are the cause.

(D) Providing painkiller medication – Painkillers like

ibuprofen or acetaminophen can help with symptom relief, but this does not treat the cause of the throat irritation.

3. A 50-year-old male patient has been admitted to a medical ward with pneumonia. The patient has fever, shortness of breath, tachycardia, tachypnea, and altered mental status. His vital signs are in a normal range.

What will be the priority nursing diagnosis?

(A) Imbalanced nutrition less than body requirement – This could be relevant, but it is not the priority. The patient's symptoms point toward an acute infection.

(B) Risk for deficient fluid volume – Fluid volume balance is important in pneumonia

management, but this is not the priority diagnosis given the altered mental status and respiratory symptoms.

(C) Ineffective airway clearance
– Given the patient's shortness of breath, tachypnea, and pneumonia, ineffective airway clearance is the most appropriate nursing diagnosis as it directly addresses his respiratory symptoms.

(D) Activity tolerance – While pneumonia can affect activity tolerance, this is not the priority at this stage, as the patient is experiencing more immediate respiratory distress.

4. A 24-year-old male patient came to a hospital with complaints of severe headache after having subtotal gastrectomy. On examination, the patient had numbness and

tingling sensation over his fingers, toes, and he has pale conjunctiva. The patient's pulse rate was 112/min and blood pressure was 110/65 mmHg. On laboratory investigation, his hematocrit level was 35% and increased mean corpuscular volume (MCV) of red blood cells. What is the most appropriate management for this patient?

(A) Monthly IM injections of vitamin B12 – The patient is likely experiencing a B12 deficiency after subtotal gastrectomy, which affects B12 absorption. Monthly B12 injections are a common treatment for such deficiencies.

(B) Improve the nutritional status of the patient – This is a good general approach, but it does not specifically address

the patient's deficiency in vitamin B12, which needs immediate attention.

(C) Ferrous sulphate, 325 mg (65 mg elemental iron) – Iron supplementation is not appropriate in this case because the patient's problem seems to be related to vitamin B12 deficiency, not iron deficiency.

(D) Give 1mg of folic acid daily with intramuscular route – Folic acid supplementation may be appropriate in some cases of anemia, but the patient's symptoms and lab results point more toward B12 deficiency rather than folate deficiency.

5. A 34-year-old man came to a dermatology clinic with painful grouped vesicles. The patient also complained of malaise

and headache before the eruption of vesicles. The nurse diagnosed the patient with herpes zoster. What additional sign and symptoms help to reach this diagnosis?

(A) The vesicles that affect the genitalia – Herpes zoster typically presents with a unilateral rash in a dermatomal pattern. Genital involvement would be more

suggestive of herpes simplex virus, not zoster.

(B) The ruptured vesicles which left yellow crust – This could indicate secondary bacterial infection of the vesicles, which is not a hallmark of herpes zoster itself.

(C) The recurrent and persistent wound over the thorax – Herpes zoster commonly affects the thorax

and follows a dermatomal distribution, making this an appropriate sign to confirm the diagnosis.

(D) Unilateral band-like configuration over the cervical nerve – The band-like configuration following a single nerve root is a characteristic feature of herpes zoster, particularly when it affects the cervical region.

1. A 40-year-old male patient with mastoiditis had surgery. He was well informed about the procedure and coped adequately with every situation after the surgery. The nurse provided postoperative nursing care and prepared the patient's expected outcome. What is the most likely expected outcome prepared for the patient?

(A) Demonstrates reduced anxiety about surgical procedure – This is a reasonable expected outcome, especially if the patient had preoperative education and is coping well. However, it focuses more on emotional response, which, although important, might not be the most specific outcome after surgery.

(B) Attainment of a positive self-concept – This is an ideal long-term goal but is not immediately expected after surgery. Postoperative recovery is more likely to focus on immediate outcomes like pain management and physical recovery.

(C) Remains free of discomfort or pain – This is a realistic and important expected outcome after surgery. Managing

postoperative pain and discomfort is a key goal in nursing care.

(D) Remains free of injury and trauma – Preventing further injury is a critical goal post-surgery, especially if the patient is recovering from an infection like mastoiditis, which could require care to prevent complications. However, managing pain and discomfort

tends to be a more immediate and measurable goal.

Correct Answer: (C) Remains free of discomfort or pain

Explanation: The priority in postoperative care is ensuring the patient remains free of discomfort or pain, as this directly impacts recovery and quality of life. This outcome aligns with immediate postoperative goals like pain management.

2. A 22-year-old man came to an ENT clinic with an ear problem that persisted for seven weeks. A nurse performed a holistic assessment and diagnosed the patient with Chronic Otitis Media. What is the typical manifestation that helps the nurse with the diagnosis?

(A) Sensation of fullness in the ear – This can occur in chronic

otitis media, but it is more common in conditions like Eustachian tube dysfunction or acute otitis media.

(B) Foul-smelling Otorrhea – Chronic otitis media often presents with foul-smelling discharge from the ear, a hallmark symptom of ongoing infection or fluid accumulation in the middle ear.

(C) Sensorineural hearing loss
– Sensorineural hearing loss is more likely associated with damage to the inner ear or nerve (e.g., due to aging or noise exposure), not typically with chronic otitis media, which often causes conductive hearing loss.

(D) Tenderness over the ear –
Tenderness is more common in acute otitis media or other

acute conditions, not as typical in chronic otitis media.

Correct Answer: (B) Foul-smelling Otorrhea

Explanation: Foul-smelling otorrhea is a characteristic symptom of chronic otitis media, as the infection in the middle ear often results in purulent discharge with a foul odor.

3. A 26-year-old known sinusitis patient came to an ENT clinic with complaints of worsening of his symptoms. Based on the patient's complaint, a duty nurse performs a holistic assessment and develops nursing diagnoses. What is the appropriate collaborative nursing diagnosis of this patient?

(A) Fever – Fever can be a symptom of sinusitis, especially if there's an infection, but it may not be the most relevant collaborative diagnosis since it's more of a symptom.

(B) Headache – Headache is a common symptom of sinusitis and is often a focal point in the assessment. However, it is a symptom, not a collaborative diagnosis in itself.

(C) Brain abscess – A brain abscess is a rare but serious complication of sinusitis, especially if the infection spreads. This is a possible but unlikely diagnosis unless there is evidence of severe, complicated infection.

(D) Pain and tenderness – Pain and tenderness, especially over the sinuses, is a hallmark of sinusitis. This is a common

and accurate nursing diagnosis reflecting the patient's symptoms.

Correct Answer: (D) Pain and tenderness

Explanation: Pain and tenderness over the sinuses is a typical finding in sinusitis.

This nursing diagnosis is appropriate to address the patient's primary symptoms of

discomfort and pain from sinus inflammation.

4. A 36-year-old woman gave birth to twin babies by elective cesarean section. After four days of the procedure, the patient developed a Deep Venous Thrombosis (DVT). On nursing assessment, the

patient was not mobile due to fear of incision site pain. Her BMI was 30. What was the main risk factor that possibly caused the development of DVT?

(A) Endothelial blood vessel trauma due to surgery – Surgery, particularly cesarean section, can cause endothelial injury, increasing the risk of clot formation. However, this

alone is not as significant as other risk factors.

(B) Altered coagulation due to her twin pregnancy – Pregnancy can lead to hypercoagulability, increasing the risk of DVT. However, the primary risk factor for this patient is likely related to immobility post-surgery.

(C) Increased emboli due to body fat and obesity – Obesity (with a BMI of 30) is a risk factor for DVT because it can contribute to slower blood flow and increased strain on the veins, which predisposes to clot formation.

(D) Venous stasis due to bed immobilization – The primary risk factor for DVT in this case is venous stasis, as the patient's lack of mobility post-

surgery due to pain can lead to stagnant blood flow, increasing the risk of clot formation.

Correct Answer: (D) Venous stasis due to bed immobilization

Explanation: Immobility after surgery leads to venous stasis, which is a key risk factor for developing DVT. The patient's

**lack of movement due to pain
and fear of incision site
discomfort exacerbates this
risk.**

**Let's go through each question
and explain one choice at a
time with reasoning:**

23. A two-year-old child was brought to a hospital with a complaint of cough, poor feeding, inspiratory stridor, convulsions during coughing, nasal flaring, respiratory rate of 50/min, pulse rate, and temperature of 37.5°C. What is the most appropriate nursing management for this child?

(A) Administer 0.5 mg/kg Diazepam injection solution per rectum – Diazepam is

typically used for seizure management, but this child seems to have symptoms related to a respiratory issue (stridor, cough, etc.), not a seizure disorder. The symptoms seem more related to respiratory distress.

(B) Administer first dose of IV/IM Ampicillin and Gentamycin – Antibiotics like Ampicillin and Gentamycin are used to treat bacterial

infections, but the child's symptoms do not suggest an infection requiring antibiotics. The symptoms are more indicative of a respiratory issue like croup or a viral condition.

(C) Administer oral Amoxicillin for five days – Amoxicillin is used for bacterial infections but is not indicated for the child's current symptoms, which are more related to viral

infections or respiratory distress.

(D) Administer maintenance fluid IV – This is an appropriate choice. The child's respiratory rate and poor feeding suggest dehydration and the need for IV fluids to maintain hydration, especially if the child cannot take fluids orally due to respiratory distress.

Correct Answer: (D) Administer maintenance fluid IV

Explanation: The child's symptoms of poor feeding, respiratory distress, and nasal flaring indicate the need for hydration support. IV fluids are essential to prevent dehydration, especially in a child who is unable to take fluids orally.

24. A two-year-old child was brought to a hospital with diarrhea of three days duration. The diarrhea is watery, but not bloody. The child has sunken eyes, cannot drink, skin pinch goes back within four seconds, and the weight-for-length Z-score is <-3 standard deviation. What is

the most appropriate drug management for this child?

(A) Administer 75 ml/kg ORS for four hours – Oral Rehydration Solution (ORS) is recommended for dehydration in children due to diarrhea, especially in cases of moderate to severe dehydration. The child's symptoms suggest dehydration, so this is an appropriate management.

(B) Administer 5 ml/kg 5% glucose solution for two hours – Glucose solution alone is not ideal for rehydrating a child with diarrhea, as it lacks the necessary electrolytes to address dehydration effectively.

(C) Administer 5 ml/kg ReSoMal solution per 30 minutes for two hours – ReSoMal is used for malnutrition-related diarrhea

and is not necessary unless malnutrition is a factor.

(D) Administer 100 ml/kg IV Ringer's lactate solution for three hours – IV fluids might be indicated for severe dehydration or when ORS is not tolerated. However, the child's dehydration is likely not severe enough to require immediate IV therapy, as oral rehydration is the first-line treatment.

Correct Answer: (A) Administer 75 ml/kg ORS for four hours

Explanation: ORS is the recommended treatment for dehydration due to diarrhea. The child's symptoms indicate moderate dehydration, and ORS is the first line of treatment to restore lost fluids and electrolytes.

25. A known HIV-positive woman gave birth to her first baby. She was not on follow-up, and the infant was exposed to HIV, which was identified through infant HIV antibody testing. The infant is on breastfeeding, and the nurse has anticipated the case and intends to give proper care to prevent the infant from

actual infection. What is the most appropriate action that the nurse should take in this case?

(A) Provide Abacavir prophylaxis for the baby – Abacavir is not typically used for prophylaxis in infants exposed to HIV. Instead, antiretroviral drugs like Nevirapine are preferred.

(B) Provide Nevirapine prophylaxis for six weeks – Nevirapine is the standard HIV prophylaxis for infants born to HIV-positive mothers to reduce the risk of HIV transmission through breastfeeding or during labor.

(C) Recommend mother to stop breastfeeding immediately – Stopping breastfeeding is not recommended unless the

mother is not on effective antiretroviral therapy or has an open lesion. In many cases, breastfeeding can continue with proper antiretroviral prophylaxis for the infant.

(D) Provide Stavudine prophylaxis for the baby for six months – Stavudine is not typically used as a first-line option for HIV prophylaxis in infants. Nevirapine is the

preferred regimen for the first six weeks.

Correct Answer: (B) Provide Nevirapine prophylaxis for six weeks

Explanation: Nevirapine is the recommended prophylaxis for HIV-exposed infants to prevent transmission of HIV during the early period after birth,

especially when the mother is breastfeeding.

26. A nurse in the PMTCT unit has prepared a nursing care plan to provide holistic care to HIV-exposed infants. A mother planned to stop breastfeeding today following the advice from the nurse. On the follow-

up visit schedule, the nurse planned to do a rapid antibody test. When is the best time to do the antibody test in this case scenario?

(A) At birth – A rapid antibody test at birth may not accurately reflect the infant's HIV status, as maternal antibodies can be present for several months after birth.

(B) At 6 weeks – This is an appropriate time for an antibody test. By 6 weeks, the infant's own antibodies would start to be detectable, and the results would be more reliable.

(C) At 10 weeks – Waiting until 10 weeks could delay the confirmation of the infant's HIV status. The recommended time for testing is 6 weeks after birth.

(D) At 12 weeks – 12 weeks is a bit late for the rapid antibody test. Early testing (at 6 weeks) is preferred to allow for earlier intervention if necessary.

Correct Answer: (B) At 6 weeks

Explanation: The best time to test an HIV-exposed infant is at 6 weeks. This allows enough time for the infant's immune system to produce detectable

antibodies while minimizing the chance of detecting maternal antibodies.

Let's go through each question one by one, explaining each choice and selecting the correct one with the reason:

9. A 42-year-old male patient presented with weakness,

fatigue, and dizziness for 20 days. His BMI is 28 kg/m², and his hemoglobin is 10 mg/dl. What is the most likely ART drug related to this side effect?

(A) Tenofovir – Tenofovir can cause kidney toxicity and bone density issues but is less commonly associated with anemia or fatigue.

(B) Efavirenz – Efavirenz can cause central nervous system side effects, including dizziness, but it is not primarily associated with anemia.

(C) Zidovudine – Zidovudine (AZT) is well known for causing hematologic side effects like anemia, which can present with fatigue, weakness, and dizziness due to its effect on bone marrow suppression.

(D) Lamivudine – Lamivudine is usually well tolerated and does not cause significant hematologic side effects like those seen with Zidovudine.

Correct Answer: (C) Zidovudine

Explanation: Zidovudine is known to cause anemia and related symptoms, such as weakness and fatigue, due to its effect on the bone marrow.

10. A 28-year-old female came to a youth-friendly service center in the hospital. After counseling, she tested positive for HIV. On assessment, she has cough and weakness. The ART focal person decided to initiate prophylaxis for TB.

What is the most appropriate drug to prescribe?

(A) Ethambutol – Ethambutol is used for tuberculosis (TB) treatment but is typically part of a combination regimen, not used alone for prophylaxis.

(B) Isoniazid – Isoniazid is commonly used for TB prophylaxis, especially for

those with HIV, to prevent the development of active TB.

(C) Rifampicin – Rifampicin is a strong anti-tuberculosis drug but is usually part of the active treatment regimen, not used for prophylaxis alone.

(D) Pyrazinamide – Pyrazinamide is used in active TB treatment, not as a prophylactic treatment.

Correct Answer: (B) Isoniazid

Explanation: Isoniazid is the standard drug used for TB prophylaxis, especially in HIV-positive individuals, to prevent the development of active tuberculosis.

11. A 25-year-old male patient presented with complaints of tinnitus and severe earache. He was treated for a throat problem last month. He has malaise and diminished hearing. On assessment, his eardrum seems red, and there is slight tenderness behind the ear. What is the most likely diagnosis?

(A) Parotitis – Parotitis refers to inflammation of the parotid

glands (typically due to viral infection like mumps) and would not usually present with the ear symptoms described.

(B) Mastoiditis – Mastoiditis is an infection of the mastoid bone behind the ear, often associated with ear infections. The symptoms of ear pain, tenderness behind the ear, and redness of the eardrum point to this condition.

(C) Otitis media – Otitis media refers to an infection of the middle ear, but it may not specifically cause tenderness behind the ear or redness of the eardrum as described.

(D) Serous Otitis – Serous otitis is characterized by fluid in the middle ear without infection, but the presence of redness and ear pain makes it less likely.

Correct Answer: (B) Mastoiditis

Explanation: Mastoiditis is characterized by inflammation or infection of the mastoid bone and is typically associated with ear infections. The symptoms described (earache, tenderness behind the ear, and redness of the eardrum) point toward this condition.

12. A 57-year-old female came to the hospital for cataract treatment. After assessing her visual acuity, the best corrected visual acuity result is 20/400. What is the most priority nursing diagnosis in this case?

(A) Low Self-Esteem – This diagnosis may be relevant in some cases of visual impairment but is not the priority in this case based on the information provided.

(B) Body Image Disturbance – Body image disturbances can be related to physical appearance but are not the primary concern for a patient with cataracts and visual impairment.

(C) Interrupted Family Processes – While visual impairment may affect family dynamics, it is not the most pressing issue in this case.

(D) Disturbed Sensory Perception – Disturbed sensory perception is the most appropriate diagnosis as the patient's significant vision impairment (20/400) indicates a sensory perception problem.

**Correct Answer: (D) Disturbed
Sensory Perception**

**Explanation: The most
appropriate diagnosis is
"Disturbed Sensory
Perception" because the
patient's visual impairment
directly affects her sensory
function.**

13. A 37-year-old female patient came to the hospital with foreign body sensation, scratching or burning sensation of the eye for three days. She has itching, photophobia, watery discharge from her eye, and lid edema. Conjunctivitis was diagnosed. What type of conjunctivitis is this?

(A) Viral – Viral conjunctivitis is often associated with redness, watery discharge, and irritation. However, the symptoms described may not be specific enough to suggest viral.

(B) Toxic – Toxic conjunctivitis can occur due to exposure to chemicals or irritants but is usually associated with more specific external causes.

(C) Allergic – Allergic conjunctivitis is characterized by itching, burning, watery discharge, and photophobia, often triggered by allergens. The symptoms described, especially the itching and photophobia, are typical of allergic conjunctivitis.

(D) Bacterial – Bacterial conjunctivitis typically presents with purulent discharge and less itching or

**burning than allergic
conjunctivitis.**

Correct Answer: (C) Allergic

**Explanation: Allergic
conjunctivitis fits the
description of itching,
photophobia, watery
discharge, and lid edema.
These are typical signs of an
allergic reaction affecting the
eyes.**

14. A 26-year-old man came to the hospital for a medical certificate to take a driving license. The nurse in charge tries to assess his visual acuity using the Snellen test. What will be the distance of the client from the chart?

(A) 20 feet – The standard distance for performing a Snellen test is 20 feet (or 6 meters), which is the correct distance to assess visual acuity.

(B) 30 feet – 30 feet is not the standard distance used for a Snellen test.

(C) 70 feet – 70 feet is much further than the recommended distance for a Snellen test.

(D) 200 feet – 200 feet is much too far for a Snellen test.

Correct Answer: (A) 20 feet

Explanation: The standard distance for a Snellen visual acuity test is 20 feet, which is

the correct distance for testing visual acuity.

15. A 42-year-old woman came to the hospital with malaise and cough for four days. Her cough has mucopurulent sputum but started as dry and irritating. She has inspiratory wheezing, moist crackles, a

**temperature of 38.8°C,
respiratory rate of 24/min,
pulse of 96/min, and blood
pressure of 100/70 mmHg.
What is the priority nursing
diagnosis?**

**(A) Acute Pain – While the
patient may be experiencing
discomfort, her symptoms
suggest a respiratory issue
rather than acute pain being
the priority.**

(B) Hyperthermia – The patient's fever (38.8°C) suggests a possible infection, but hyperthermia is not the most urgent concern here.

(C) Impaired Breathing Pattern – The presence of wheezing, crackles, and cough points to a respiratory issue, but the patient's breathing rate is relatively normal at 24/min.

(D) Ineffective Airway Clearance – The patient's productive cough, wheezing, crackles, and temperature suggest a respiratory infection or condition leading to airway obstruction or ineffective airway clearance.

Correct Answer: (D) Ineffective Airway Clearance

Explanation: The patient's cough, sputum production, wheezing, and crackles point to a problem with airway clearance, making "Ineffective Airway Clearance" the priority diagnosis.

Let's break down the questions and provide the correct answer with explanations:

1. A 42-year-old woman came to the hospital with malaise and cough for four days. Her cough has mucopurulent sputum but was dry and irritating in the beginning. On assessment, she has inspiratory wheezing, moist inspiratory crackles, temperature 38.8°C, respiratory rate 24/min, pulse 96/min, and blood pressure

100/70 mmHg. What is the priority nursing diagnosis for this patient?

(A) Acute Pain – Acute pain may occur with respiratory infections, but the primary concern here is the difficulty with breathing and airway clearance.

(B) Hyperthermia – The patient's temperature (38.8°C)

indicates a fever, but the focus should be on respiratory symptoms.

**(C) Impaired Breathing Pattern
– The patient's wheezing and crackles may suggest an issue with breathing, but the pattern is not necessarily abnormal at this moment.**

(D) Ineffective Airway Clearance – Given the

mucopurulent sputum, wheezing, and crackles, this suggests that the patient is struggling to clear her airways effectively.

Correct Answer: (D) Ineffective Airway Clearance

Explanation: The patient is experiencing difficulty with airway clearance as evidenced by her cough, sputum

**production, and wheezing.
Ineffective airway clearance is
the priority nursing diagnosis.**

**2. A 35-year-old woman
admitted in the medical ward
with a diagnosis of heart
failure. On assessment, the
patient has fatigue,
drowsiness, and delayed**

reaction time. Her blood pressure is 100/60 mmHg, respiratory rate 32/min, and pulse rate 110/min. The nurse in charge in the ward decided to give oxygen. What is the most likely reason for the nurse's decision to give oxygen?

(A) Pulse rate – A pulse rate of 110/min is elevated but does not directly indicate the need for oxygen.

(B) Drowsiness – Drowsiness may result from inadequate oxygenation, but this symptom alone does not indicate the immediate need for oxygen.

(C) Respiratory rate – The patient's respiratory rate of 32/min is elevated and suggests respiratory distress, which may require oxygen to ensure adequate oxygenation.

(D) Delayed reaction time –
Delayed reaction time can be a
sign of insufficient oxygenation
to the brain, which can be
addressed by administering
oxygen.

Correct Answer: (C)

Respiratory rate

Explanation: The elevated
respiratory rate (32/min)

suggests the patient is working harder to breathe, potentially due to inadequate oxygen levels. Oxygen therapy is often needed to correct this.

3. A 57-year-old known HIV patient admitted in the medical ward. The patient has cough with mucopurulent

sputum, dyspnea, orthopnea, headache, and crackles. On assessment, the nurse found chest pain that aggravates with deep breathing and also has cyanotic lips. Vital signs: Temperature 39°C, respiratory rate 30/min, pulse rate 104/min, and blood pressure 110/76 mmHg. What is the priority nursing intervention in this case?

(A) Administer oxygen – The patient's cyanosis and respiratory distress are signs of hypoxia, and oxygen therapy would be the first step in managing this issue.

(B) Administer oral fluids – While hydration is important, this is not the priority in a patient who is showing signs of respiratory distress and hypoxia.

**(C) Administer acetaminophen
– This could help reduce fever,
but it does not address the
underlying respiratory issue.**

**(D) Administer expectorants as
ordered – While expectorants
can help loosen mucus, the
priority is to address
oxygenation and stabilize the
patient.**

Correct Answer: (A) Administer oxygen

Explanation: The patient's symptoms of cyanosis and difficulty breathing indicate that the priority is to improve oxygenation to prevent further complications.

4. A 57-year-old known diabetic intubated man admitted to the intensive care unit for four days. He has developed chest pain that intensified by breathing. He has cough, dyspnea, and crackles. His body temperature is 38.9°C, WBC count is 15,000 cells/ml, and there is newly developed parenchymal opacity on chest radiography. What is the most likely diagnosis for this patient?

(A) Hospital-acquired pneumonia – This is the most likely diagnosis, as the patient is in the ICU, intubated, and has developed symptoms of pneumonia after a few days in the hospital.

(B) Community-acquired pneumonia – Community-acquired pneumonia is less likely, as the patient has been in the hospital for several days.

(C) Ventilator-associated pneumonia – Ventilator-associated pneumonia (VAP) is a type of hospital-acquired pneumonia, especially in intubated patients. It could be a likely diagnosis.

(D) Health care-associated pneumonia – Health care-associated pneumonia is also a possibility, but given the

patient is intubated, VAP is more specific.

Correct Answer: (C) Ventilator-associated pneumonia

Explanation: Given the patient's intubation and the development of pneumonia-like symptoms after several days in the ICU, ventilator-associated pneumonia (VAP) is the most likely diagnosis.

5. A 32-year-old female patient presented with complaints of itching and burning sensation of the face and hands for one day. She said it happened after washing her face and hands with a new brand of soap. She has weeping and formed crusts on skin lesions on her face and

hands. She feels that her skin was peeling. What type of hypersensitivity does the patient have?

(A) Type I – Type I hypersensitivity is an allergic reaction, such as anaphylaxis, caused by IgE antibodies, typically involving a rapid onset, not matching the delayed onset of skin symptoms here.

(B) Type II – Type II hypersensitivity involves cytotoxic reactions and is associated with conditions like hemolytic anemia, not skin irritation from a soap.

(C) Type III – Type III hypersensitivity involves immune complexes, typically causing more systemic reactions like vasculitis or glomerulonephritis, not the

**localized skin response
described here.**

**(D) Type IV – Type IV
hypersensitivity is a delayed-
type hypersensitivity,
commonly seen with contact
dermatitis, such as the
reaction to the new soap the
patient used.**

Correct Answer: (D) Type IV

Explanation: The patient's symptoms of itching, burning, and skin lesions following exposure to a new soap indicate a contact dermatitis reaction, which is a classic example of Type IV hypersensitivity.

Let's go over each question and provide the correct answer with explanations:

51. A 50-year-old diabetic patient with uncontrolled glycemia was brought to the Emergency Department with sudden loss of vision after 12 years of disease onset. The nurse in charge suspected a complication. What is the most likely complication expected for the above scenario?

(A) Diabetic ketoacidosis –
Diabetic ketoacidosis is more common in uncontrolled type 1 diabetes and usually presents with metabolic acidosis, vomiting, and dehydration, not sudden vision loss.

(B) Diabetic nephropathy –
Diabetic nephropathy primarily affects the kidneys, leading to proteinuria and

kidney failure, but it does not cause sudden vision loss.

**(C) Diabetic neuropathy –
Diabetic neuropathy affects the nerves, leading to symptoms like numbness or tingling, not sudden vision loss.**

**(D) Diabetic retinopathy –
Diabetic retinopathy is a common complication of long-**

term uncontrolled diabetes that affects the blood vessels in the eyes, often leading to sudden vision loss.

Correct Answer: (D) Diabetic retinopathy

Explanation: Diabetic retinopathy is a common complication of long-term uncontrolled diabetes and is known to cause vision

problems, including sudden vision loss.

52. A client who spent one week in the ward refused to stay and wanted to be discharged by his own decision. The nurse in charge tried to explain the process of care to keep the patient on

treatment. Unfortunately, the patient escaped without fulfilling formal procedure from the ward. What nursing action takes priority in the above scenario?

(A) Record the fact on the form and have another health professional witness – Documenting the patient's escape is essential but not the first priority.

(B) Search for the client's caregiver around the hospital to sign the form – While caregivers should be involved, the priority is notifying the appropriate personnel.

(C) Notify the physician, nurse in charge, and agency administration – This is the most appropriate action to ensure the situation is handled properly and all relevant

parties are aware of the situation.

(D) Call the police officer of the hospital and city administration – This is a last resort and not the first action unless the situation escalates.

Correct Answer: (C) Notify the physician, nurse in charge, and agency administration

Explanation: The nurse should first notify the relevant healthcare team and administration to ensure proper documentation and follow-up for the patient's safety.

53. After 10 days of inpatient care, a client is going to be

discharged today. The nurse obtained authorization for discharge from the physician in charge. What should be the nurse's next plan of action for this case?

(A) Discharge the client – While discharging the patient is the goal, there are steps that must be followed first.

**(B) Write discharge summary –
The discharge summary should
be written before discharge, to
ensure continuity of care.**

**(C) Notify business personnel –
This is not a priority action at
this stage.**

**(D) Provide discharge
instructions – Providing the
patient with discharge
instructions is essential for**

ensuring safe discharge and proper follow-up care.

Correct Answer: (D) Provide discharge instructions

Explanation: Before discharge, the nurse should provide the patient with detailed instructions for aftercare, medications, follow-up appointments, and warning signs to monitor.

54. A 65-year-old client with a laboratory result of low white blood count was admitted to the medical ward. The nurse in charge wore a mask, washed hands on entry to the room, and kept the client in a private closed room with specially monitored room pressure. The

nurse advised the client to wear a mask during transportation from one room to another. What principle of standard precaution method is applied in the above scenario?

(A) Airborne precautions – Airborne precautions are used for diseases that spread via tiny droplets, such as tuberculosis, and involve specific air handling systems, which is not the case here.

(B) Contact precautions –
Contact precautions are for preventing the spread of infections through direct contact, which does not match this scenario.

(C) Droplet precautions –
Droplet precautions are used for infections spread by larger respiratory droplets, which might be applicable here if the

client has a respiratory condition.

(D) Standard precautions – Standard precautions include basic hygiene and infection control measures for all patients, but in this case, specific precautions for low white blood count point to more specific measures.

Correct Answer: (A) Airborne precautions

Explanation: Although the client's specific infection is not stated, the precautions involving a private room and mask usage point to the need for airborne precautions to protect against potential transmission.

55. A nurse was performing wound dressing of an infected wound in the emergency ward and needs to decontaminate all the medical instruments used. What is the duration of decontamination the nurse should wait?

(A) 5 minutes – This is likely too short of a time for proper decontamination.

(B) 8 minutes – This is not long enough for effective decontamination.

(C) 10 minutes – Ten minutes is a reasonable time for many decontamination procedures using disinfectants.

(D) 12 minutes – While possibly effective, this duration is usually reserved for

**more intensive
decontamination procedures.**

Correct Answer: (C) 10 minutes

**Explanation: A 10-minute
duration is often
recommended for disinfecting
medical instruments,
depending on the disinfectant
used.**

76. A 29-year-old female patient came to the emergency after a motor vehicle accident. Initial findings include bilateral findings on the lung fields, respiratory tract unobstructed, skin is cool, bloody, and clammy; heart rate is 110bpm; and peripheral pulses are intact. What nursing diagnosis is priority for this case?

(A) Ineffective breathing pattern – This would be a priority if the patient was having difficulty breathing, but there is no mention of that in the description.

(B) Impaired gas exchange – The respiratory findings may suggest an issue with gas exchange, which is a priority concern.

(C) Impaired skin integrity – This may be relevant but is not the most urgent priority given the clinical presentation.

(D) Fluid volume deficit – The patient's cool, bloody, and clammy skin suggests possible hypovolemia, making fluid volume deficit the priority.

Correct Answer: (D) Fluid volume deficit

Explanation: The patient's skin signs, heart rate, and other symptoms point to shock and fluid volume deficit, which must be addressed immediately.

77. A 39-year-old male patient was admitted after a motorcycle crash. He has facial contusions and lacerations, a fractured sternum, three fractured ribs, and hemothorax. Initial findings include sharp pain over chest, patent airway with absent breath sounds in the right basal lung field, respiratory rate 32 shallow per minute, blood pressure is 94/70 mmHg, skin is cool, bloody, and

**clammy; heart rate is 110/min.
Which intervention is life-saving and reverses most conditions of the patient?**

(A) Fluid administration – Fluid administration would help address shock, but may not directly address the hemothorax.

(B) Inserting chest tube – Inserting a chest tube to drain

the blood from the pleural cavity (hemothorax) is crucial for this patient's survival and recovery.

(C) Fixation of fracture – While fracture fixation is important, the immediate life-saving intervention is addressing the hemothorax.

(D) Stop bleeding – Stopping external bleeding is important,

but the most immediate life-saving intervention here is to address the internal bleeding (hemothorax).

Correct Answer: (B) Inserting chest tube

Explanation: The patient has a hemothorax, and the life-saving intervention is to insert a chest tube to relieve the

pressure and drain blood from the pleural cavity.

78. The nurse conducted an interview with a male patient. The patient started to explain about his health problem and said "since the past five years I am suffering from joint pain, my legs are not moving fully

**and I have severe gastritis."
The nurse in between reacted
as "you mean you were in
great suffering." Which types
of interviewing technique is
used by the nurse?**

**(A) Clarifying – Clarifying
would involve asking questions
to make the patient's
statements clearer.**

(B) Paraphrasing – Paraphrasing involves repeating what the patient said in a different way, which the nurse did not do.

(C) Reflecting – Reflecting involves echoing the patient's feelings or concerns, which is what the nurse did by emphasizing the patient's suffering.

(D) Summarizing –
Summarizing is a brief recap,
not done in this situation.

Correct Answer: (C) Reflecting

Explanation: The nurse
reflected the patient's feelings
of suffering, which is a
common technique to show
empathy and understanding.

79. A 32-year-old woman came to the hospital seeking medical care. The physician assessed her and prescribed medications. He told her "after you complete this medication I will operate on you." She asked him why she will be operated. The physician replied, "The problem is not confirmed yet, and I will tell you when confirmed." Which

bill of right of the patient is violated?

(A) Right to obtain information about each procedure to be conducted – The patient is being told that the procedure will be explained later, which violates the right to be informed about the procedure.

(B) Right to obtain information concerning diagnosis and

treatment – This is directly relevant, as the physician is withholding the diagnosis.

(C) Right to obtain information for referral for better investigation – This right is not violated in this case.

(D) Right to receive information necessary to give informed consent – The patient is not receiving all the

information needed to give informed consent.

Correct Answer: (D) Right to receive information necessary to give informed consent

Explanation: The physician's refusal to provide details about the operation violates the patient's right to make an informed decision regarding her treatment.

Let's go through each question one by one and analyze the best possible answer:

16. Discharge teaching plan for a 55-year-old man being discharged after carotid artery surgery for atherosclerosis. He has been a smoker for 15

years. What should be the priority teaching plan the nurse should consider for this patient?

(A) Eat grains and vegetables more – While diet is important, this isn't the first priority.

(B) Promote cessation of tobacco use – Smoking is a major risk factor for

cardiovascular disease and cessation is crucial for improving long-term health and recovery.

(C) Perform physical activity daily – Physical activity is important, but first, addressing smoking cessation is more urgent.

(D) Minimize vegetable oils – While dietary changes are

essential, the priority is to address smoking cessation.

Correct Answer: (B) Promote cessation of tobacco use

Explanation: Smoking cessation should be the primary focus, as it is a major risk factor for worsening atherosclerosis and cardiovascular complications.

17. A 65-year-old patient was admitted to a cardiac unit with a diagnosis of myocardial infarction secondary to coronary artery disease. The patient was instructed to take nitroglycerine. What is the most likely predicted effect of this drug on this patient?

(A) Improve pulmonary circulation – Nitroglycerine does not primarily affect pulmonary circulation.

(B) Improve systemic circulation – Nitroglycerine affects systemic blood vessels but not in a way that directly improves overall systemic circulation in a broad sense.

(C) Dilate coronary blood vessels – Nitroglycerine works by dilating the coronary arteries, improving blood flow to the heart muscle, and reducing chest pain.

(D) Dilate systemic blood vessels – While nitroglycerine dilates systemic blood vessels to reduce blood pressure, its primary action in a heart attack is to improve coronary blood flow.

Correct Answer: (C) Dilate coronary blood vessels

Explanation: Nitroglycerine is a vasodilator that primarily dilates coronary blood vessels, improving oxygen supply to the heart and relieving chest pain.

18. An adult female patient was admitted with a diagnosis of angina pectoris. On history, chest pain occurs both at rest and with extraneous activities with no radiation pattern. During examination, there was ST segment elevation over ECG that reversed in a second measurement four hours later. What type of angina manifested in this patient?

(A) Stable angina – Stable angina typically occurs with exertion and relieves with rest, which doesn't fit this case.

(B) Unstable angina – Unstable angina occurs at rest or with minimal exertion and may be associated with ECG changes. This fits the patient's description.

(C) Variant angina – Variant angina (Prinzmetal's angina) is due to coronary artery spasm, usually with a pattern of ST elevation.

(D) Intractable angina – Intractable angina is severe and refractory to treatment, which doesn't match the symptoms described.

Correct Answer: (B) Unstable angina

Explanation: Unstable angina occurs unpredictably and may happen at rest. The ST elevation that reverses suggests ischemia associated with unstable angina.

19. A 46-year-old male patient with acute chest pain. On examination, he held his left side of the chest during breathing, has a BMI of 25 kg/m², blood pressure 110/85 mmHg, respiratory rate 26/min, cholesterol of 250 mg/dl, and increased cardiac-specific Troponin. What is the most appropriate initial step in management?

(A) Administer morphine –
Morphine is used for pain relief in suspected myocardial infarction, but the first step is typically administering aspirin to prevent clot formation.

(B) Administer propranolol –
Beta-blockers are used to reduce heart rate, but aspirin is the first line of treatment.

(C) Administer nitroglycerine – Nitroglycerine is for pain relief in angina, but the priority is aspirin for its anticoagulant properties.

(D) Administer large doses of aspirin – Aspirin is the first line of treatment to inhibit platelet aggregation in the case of acute chest pain due to suspected myocardial infarction.

Correct Answer: (D) Administer large doses of aspirin

Explanation: The first step in managing acute chest pain with suspected myocardial infarction is administering aspirin to prevent further thrombus formation and reduce complications.

20. A child has wheezing, intercostal muscle retraction, and cool skin with a pulse rate of 100/min. What is the most likely nursing diagnosis that needs priority for this child?

(A) Impaired gas exchange – Given the child's respiratory symptoms, impaired gas exchange is a likely diagnosis.

(B) Ineffective airway clearance – Airway obstruction or narrowing may cause the child to be unable to clear secretions.

(C) Ineffective breathing pattern – The symptoms may also suggest an altered breathing pattern.

(D) Impaired spontaneous ventilation – This could be

applicable, but given the symptoms, the focus should be on gas exchange and airway clearance first.

Correct Answer: (A) Impaired gas exchange

Explanation: The child is exhibiting signs of respiratory distress, including wheezing and intercostal retraction,

indicating impaired gas exchange as a priority issue.

21. A seven-year-old child with multiple injuries from a vehicle accident presents with suspected neck and back injuries. What is the initial step the nurse should do for this child?

(A) Perform jaw thrust – This maneuver is used to open the airway in trauma patients but isn't the first step in this case.

(B) Clear secretions from the throat – While this is important, the first priority is to stabilize the neck and spine.

(C) Inspect mouth and remove foreign body – This is not the

immediate priority; spine stabilization is crucial.

(D) Check the airway by looking for chest movements – This could be necessary, but spinal precautions should be taken first.

Correct Answer: (A) Perform jaw thrust

Explanation: In a trauma situation with suspected neck or spine injury, the nurse should perform a jaw thrust to open the airway while maintaining spinal precautions.

22. A preschool-age child with a history of childhood asthma

presents with a peak expiratory flow of 30%, oxygen saturation of 78%, and loud wheezing. What is the actual nursing diagnosis that needs priority for this child?

(A) Impaired gas exchange – This is a priority diagnosis due to low oxygen saturation and wheezing.

(B) Ineffective airway clearance – The wheezing indicates potential airway obstruction, but the priority is gas exchange.

(C) Ineffective breathing pattern – While this might be present, impaired gas exchange is a higher priority.

(D) Impaired spontaneous ventilation – This could be a

concern but is secondary to gas exchange in this case.

Correct Answer: (A) Impaired gas exchange

Explanation: The child's oxygen saturation and wheezing indicate significant respiratory distress, leading to impaired gas exchange as the priority diagnosis.

23. A nine-year-old child with diarrhea, non-bilious projectile vomiting, and low-grade fever is seen in the pediatric outpatient department. What is the most likely diagnosis for this child?

**(A) Intussusception –
Intussusception presents with**

abdominal pain, vomiting, and sometimes bloody stools, but non-bilious vomiting and diarrhea point away from this.

(B) Intestinal volvulus – This presents with vomiting, abdominal pain, and can cause bowel obstruction, but diarrhea isn't typical.

(C) Pyloric stenosis – This causes projectile vomiting in

infants but does not present with diarrhea.

(D) Hirschsprung disease – This can cause constipation, abdominal distention, and vomiting but would not typically cause diarrhea.

**Correct Answer: (A)
Intussusception**

Explanation: Intussusception can present with abdominal pain, vomiting, and sometimes fever. It is a likely cause given the child's symptoms.

24. A nurse is planning health education for a mother of a seven-year-old child with juvenile idiopathic arthritis.

What is the most appropriate advice the nurse could give to the mother?

(A) Restrict from fatty foods during severe form of illness – While diet is important, this is not the primary concern in managing juvenile idiopathic arthritis.

(B) Restrict from exercise during moderate form of

illness – Exercise is important for managing juvenile idiopathic arthritis, and restrictions should only be made during acute flare-ups.

(C) Restrict from protein foods during severe form of illness – There is no specific restriction on protein foods unless there is kidney involvement.

(D) Avoid systemic steroids during moderate form of illness – Steroids may be necessary in flare-ups, so avoiding them is not the best advice.

Correct Answer: (B) Restrict from exercise during moderate form of illness

Explanation: During periods of moderate illness or flare-ups, it

may be beneficial to restrict certain physical activities, but exercise is typically encouraged during remission phases to maintain joint mobility and function.

Let's go through each question and analyze the most appropriate answers:

39. An elderly woman was admitted to an orthopedic ward due to a fracture. Her laboratory results show the following: calcium 5.6 mg/dl, phosphorus 5.78 mg/dl, and sodium 130 mEq/L. What is the most likely explanation for this condition?

**(A) Increased serum uric acid –
This would be relevant if the**

patient had gout, but there's no mention of that here.

**(B) Decreased serum calcium –
The calcium level is low (5.6 mg/dl), which is below the normal range (8.5-10.5 mg/dl).
This suggests hypocalcemia.**

**(C) Decreased serum sodium –
The sodium level (130 mEq/L) is lower than normal (135-145**

mEq/L), which could indicate hyponatremia.

(D) Increased serum phosphorus – The phosphorus level (5.78 mg/dl) is elevated above the normal range (2.5-4.5 mg/dl), which could be due to kidney dysfunction or bone problems.

Correct Answer: (D) Increased serum phosphorus

Explanation: The elevated phosphorus level is the most significant abnormality here and may be linked to bone issues, kidney function problems, or complications related to fracture healing.

40. A 45-year-old client was presented to an orthopedic unit after sustaining blunt trauma to the right upper limb. The patient has an open fracture with a large wound measuring 5 cm and minor soft tissue damage. What is the most likely grade of fracture in this patient?

(A) Grade I – Grade I fractures are simple fractures with small

wounds (usually less than 1 cm).

(B) Grade II – Grade II fractures have larger wounds (more than 1 cm) but without extensive soft tissue damage.

(C) Grade III – Grade III fractures have significant soft tissue damage and may have a wound larger than 5 cm.

(D) Grade IV – Grade IV is not a standard grading for fractures; the common grading system is I to III.

Correct Answer: (B) Grade II

Explanation: A Grade II fracture has a large wound (greater than 1 cm) but relatively minor soft tissue damage, which fits the description in this case.

50. A 66-year-old patient was admitted with a diagnosis of gout arthritis. The nurse advised the patient to increase water intake up to 3000 ml per day. What is the most likely anticipated complication by the nurse?

(A) Urolithiasis – Increasing fluid intake helps prevent kidney stones (urolithiasis), particularly in patients with gout, who are at higher risk of developing urate crystals in the kidneys.

(B) Hypotension – Increased fluid intake is unlikely to cause hypotension.

(C) Dehydration – Dehydration is less likely given that the patient is advised to increase fluid intake.

(D) Cholelithiasis – This is not typically related to fluid intake in the context of gout.

**Correct Answer: (A)
Urolithiasis**

Explanation: Increasing fluid intake is necessary for gout patients to prevent urate crystal formation, but excessive uric acid may still lead to kidney stones (urolithiasis).

51. A 38-year-old woman was presented with acute

tenderness, swelling, and paravertebral muscle spasm after a motor vehicle crash. She is conscious with no open wounds or bleeding. The pain is worse with movement and coughing. What is the priority nursing intervention for this patient?

(A) Immobilization and maintain spinal alignment – The patient's symptoms, including tenderness and

swelling in the back, suggest a potential spinal injury, so immobilization is necessary.

(B) Advise the patient to practice leg exercises – This is not appropriate at this point due to the possible spinal injury.

(C) Ambulate the patient and maintain mobility – This would

not be appropriate for a potential spinal injury.

(D) Give pain medication as needed – Pain relief is important, but spinal immobilization is the priority here.

**Correct Answer: (A)
Immobilization and maintain spinal alignment**

Explanation: Given the potential for spinal injury, immobilization is crucial to prevent further damage and stabilize the spine.

52. A nurse is starting to take vital signs for a patient who is stressed about re-integration activities in a clinic. What is

the most likely abnormal finding expected in this patient?

**(A) Decreased pulse rate –
Stress typically causes an increase in pulse rate, not a decrease.**

**(B) Increased blood pressure –
Stress is likely to elevate blood pressure.**

(C) Increased temperature – Stress can cause a temporary increase in body temperature, but it is not as common as an increase in pulse rate or blood pressure.

(D) Decreased respiratory rate – Stress usually causes an increase in respiratory rate, not a decrease.

Correct Answer: (B) Increased blood pressure

Explanation: Stress often leads to an increase in blood pressure, as it triggers the body's "fight or flight" response.

54. A 25-year-old patient was presented to an emergency

medical unit with a complaint of headache and blurred vision. The blood pressure is 190/126 mmHg. The physician and nurse agree on the need for immediate action. What is the most likely type of medication order in this case?

(A) Stat order – A stat order is used when immediate action is required, which fits this case of extremely high blood pressure.

(B) PRN order – A PRN (as needed) order would not be appropriate in this emergency situation.

(C) Verbal order – A verbal order could be given, but this is less common in emergency situations.

(D) Standing protocol – A standing protocol is typically used for ongoing treatments,

**not for immediate
emergencies.**

Correct Answer: (A) Stat order

**Explanation: Given the severity
of the patient's blood
pressure, a stat order for
immediate treatment is
necessary.**

55. A physician decided to discharge a 26-year-old mother after a normal birth, according to the Ethiopian National Maternal Discharge Protocol. What is the most likely initial nursing consideration to be taken?

(A) Ensuring clinician criteria for discharge – The nurse should first ensure that the

**clinician's discharge criteria
have been met.**

**(B) Communicating the
planned discharge –
Communication with the rest
of the healthcare team is
important but should come
after confirming discharge
criteria.**

**(C) Documenting discharge
checklist – Documentation is**

essential but typically follows confirming discharge criteria.

(D) Complete a discharge summary – A discharge summary is important but comes later in the discharge process.

Correct Answer: (A) Ensuring clinician criteria for discharge

Explanation: The nurse must first confirm that all the clinician's discharge criteria have been met before proceeding with the discharge process.

56. A 34-year-old female patient is receiving a blood transfusion due to severe

anemia from blood loss. The nurse is monitoring the patient for signs of transfusion reaction. What is the priority action the nurse should take in this case?

(A) Stop transfusion – The first action should be stopping the transfusion if a reaction is suspected.

(B) Check vital signs – Checking vital signs is important, but stopping the transfusion comes first.

(C) Check the skin for rashes – While checking the skin is important for transfusion reactions, stopping the transfusion is the priority.

(D) Document transfusion reaction – Documentation is

necessary but should be done after the immediate steps are taken.

Correct Answer: (A) Stop transfusion

Explanation: The priority is to stop the transfusion if any signs of a reaction are present, to prevent further complications.

Let's go through each of the scenarios and their possible answers:

1. A child is brought to the emergency department with dislodged trapping, gurgling, and choking. There is retraction of intercostal muscles and a quick

**respiratory rate of 62/min,
with a pulse rate of 100 bpm.
What is the most likely actual
nursing diagnosis that needs
priority for this child?**

**(A) Impaired gas exchange –
This could be due to the
choking and difficulty
breathing.**

**(B) Ineffective airway
clearance – Choking and**

retraction of muscles indicate difficulty in clearing the airway.

(C) Ineffective breathing pattern – This could be a result of the child's struggle to breathe.

(D) Impaired spontaneous ventilation – This could occur, but it is less likely than the

immediate need for airway clearance.

Correct Answer: (B) Ineffective airway clearance

Explanation: The child's symptoms, including gurgling, choking, and muscle retraction, suggest airway obstruction that requires immediate attention.

2. An unconscious seven-year-old child has sustained multiple body injuries secondary to a vehicle accident. The nurse suspects neck and back injury. What is the initial step the nurse should do for this child?

(A) Perform jaw thrust – This maneuver is used to open the airway if there is a risk of cervical spine injury, which is likely in this case.

(B) Clear secretions from the throat – This may be necessary but should be done after the airway is secured.

(C) Inspect mouth and remove foreign body – This should only

be done if there is a clear obstruction.

(D) Check the airway by looking for chest movements – While important, the first priority is spinal stabilization.

Correct Answer: (A) Perform jaw thrust

Explanation: The jaw thrust maneuver is key to ensuring

the airway is open without compromising the cervical spine in cases of suspected neck or back injury.

3. A preschool-age child with a history of childhood asthma is brought to the pediatric emergency unit. The child has a peak expiratory flow of 30%,

oxygen saturation of 78%, and loud wheezing. What is the actual nursing diagnosis that needs priority for this child?

(A) Impaired gas exchange – The low oxygen saturation (78%) suggests impaired gas exchange.

(B) Ineffective airway clearance – The wheezing indicates some form of

obstruction but gas exchange is the immediate concern.

(C) Ineffective breathing pattern – Breathing difficulties may be present but are secondary to impaired gas exchange.

(D) Impaired spontaneous ventilation – This diagnosis might be possible if the child were unable to breathe

independently, but gas exchange is more urgent.

Correct Answer: (A) Impaired gas exchange

Explanation: The critical low oxygen saturation and wheezing indicate impaired gas exchange, which requires immediate intervention.

4. A nine-year-old child presents with complaints of diarrhea, non-bilious projectile vomiting, and low-grade fever. What is the most likely diagnosis for this child?

(A) Intussusception – This condition can cause vomiting and abdominal pain.

(B) Intestinal volvulus – Can cause similar symptoms, but intussusception is more common in this age group.

(C) Pyloric stenosis – This usually causes projectile vomiting, but without diarrhea.

(D) Hirschsprung disease – This typically presents with constipation, not diarrhea.

Correct Answer: (A)

Intussusception

Explanation: The combination of vomiting, diarrhea, and fever is most consistent with intussusception, a condition in which part of the intestine folds into another part.

5. A nurse planned to give health education to the mother of a seven-year-old child with juvenile idiopathic arthritis. What is the most appropriate advice the nurse could give to the mother?

(A) Restrict from fatty foods during the severe form of illness – This may help with inflammation but is not the primary concern.

(B) Restrict from exercise during moderate form of illness – Moderate exercise is generally encouraged to maintain joint function.

(C) Restrict from protein foods during severe form of illness – There is no need to restrict protein.

(D) Avoid systemic steroids during the moderate form of illness – Steroids may be needed to manage inflammation, so this is not advisable unless contraindicated.

Correct Answer: (B) Restrict from exercise during moderate form of illness

Explanation: In juvenile idiopathic arthritis, gentle exercises are encouraged, but during flare-ups or more severe symptoms, exercise should be minimized to avoid joint stress.

6. A 37-year-old woman presents to the surgical

emergency unit with multiple fractures. On assessment, there is paradoxical chest-wall movement with inspiration and expiration. What is the priority nursing intervention next to immobilization?

**(A) Administer pain medication
– Pain relief is necessary, but stabilizing breathing takes priority.**

(B) Monitor for signs of respiratory infection – This is not the priority intervention right now.

(C) Instruct the patient to breathe on the affected side – This could worsen the condition and should be avoided.

(D) Instruct the patient to take deep breaths – Taking deep

breaths could be difficult or painful with paradoxical chest movement and could worsen the injury.

Correct Answer: (A) Administer pain medication

Explanation: Paradoxical chest movement is a sign of rib fractures or flail chest. Pain management is critical to allow

the patient to breathe more effectively.

7. A 65-year-old man with bronchiectasis is admitted for heart failure. He is able to breathe only in a sitting position. What is the priority nursing diagnosis for this patient?

(A) Anxiety – This could be a factor but is not the priority.

(B) Activity intolerance – The patient's inability to breathe comfortably while lying down indicates activity intolerance.

(C) Ineffective airway clearance – This may be a concern with bronchiectasis, but the

breathing difficulty is related to positioning.

**(D) Impaired physical comfort
– The patient's discomfort is likely due to difficulty breathing in a supine position.**

Correct Answer: (B) Activity intolerance

Explanation: The patient's inability to breathe

comfortably in any position other than sitting suggests activity intolerance, which is related to the progression of heart failure and bronchiectasis.

8. A 31-year-old male patient admitted to a medical ward with acute illness. Physical

examination reveals dry mucous membranes, poor skin turgor, and orthostatic hypotension. What is the primary nursing diagnosis for this patient?

(A) Risk for impaired skin integrity – Not the main concern here, although dehydration could contribute.

(B) Deficient knowledge – This doesn't fit the clinical signs presented.

(C) Fluid volume deficit – The clinical signs (dry mucous membranes, poor skin turgor, hypotension) point to dehydration.

(D) Activity intolerance – Not the primary issue at this point.

Correct Answer: (C) Fluid volume deficit

Explanation: The patient's signs of dehydration point to a fluid volume deficit, which is the priority nursing diagnosis.

9. A 36-year-old patient presents with complaints of

peripheral edema and cola-colored urine. The patient has a history of frequent tonsillitis and infections. Lab investigations show a GFR of 100 mL/min. What is the most likely fluid and electrolyte balance change expected for this patient?

(A) Hypokalemia – This is unlikely given the patient's kidney function.

(B) Hypernatremia – The kidney's inability to filter could lead to water retention, causing hyponatremia, not hypernatremia.

(C) Marked hypertension – This could be possible, especially with kidney dysfunction.

(D) Decreased urine output – This could be a result of kidney dysfunction, but the patient's

symptoms point more to fluid retention.

Correct Answer: (C) Marked hypertension

Explanation: The patient's kidney function, indicated by the GFR and symptoms of fluid retention, could cause hypertension.

10. A 63-year-old patient presents with sudden onset of disturbance, paralysis on one side of the body, and severe headache. The patient has a history of poorly controlled hypertension. What is the most likely diagnosis of this patient?

(A) Embolic stroke – This typically involves sudden onset but is usually caused by a clot traveling to the brain.

(B) Ischemic stroke – This is a possible diagnosis given the patient's history of hypertension.

(C) Thrombotic stroke – This could be a type of ischemic

stroke caused by a clot forming in the brain.

**(D) Hemorrhagic stroke –
Given the history of
hypertension, this is a strong
possibility.**

**Correct Answer: (D)
Hemorrhagic stroke**

**Explanation: The patient's
sudden symptoms with a**

history of uncontrolled hypertension are consistent with a hemorrhagic stroke, where blood vessels in the brain rupture.

11. A 36-year-old patient presents with severe headache, neck stiffness, and fever. The physician plans to

**perform a lumbar puncture.
What is the most likely
contraindication for this
procedure?**

**(A) Decreased level of
consciousness – This would
increase the risk of brain
herniation if a lumbar
puncture were performed.**

(B) Decreased intracranial pressure – This is not typically a contraindication.

(C) Infection at the proposed site – This would be a contraindication for a lumbar puncture.

(D) Increased blood pressure – Increased blood pressure could be a concern but is not an absolute contraindication.

Correct Answer: (A) Decreased level of consciousness

Explanation: A decreased level of consciousness suggests elevated intracranial pressure, which makes lumbar puncture risky due to the potential for brain herniation.

**Let's go through each scenario
and analyze the most likely
answers:**

**1. A 21-year-old patient with a
history of hepatitis presents to
the medical ward. The patient
has a history of IV drug use
and previous blood
transfusions. What is the most**

likely etiology for the patient's diagnosis?

(A) Hepatitis A Virus – Unlikely; Hepatitis A is typically transmitted through contaminated food or water.

(B) Hepatitis B Virus – This is a possibility, but Hepatitis B is generally less common in IV drug users than Hepatitis C.

(C) Hepatitis C Virus – Most likely, given the history of IV drug use and blood transfusions, both of which are risk factors for Hepatitis C.

(D) Hepatitis D Virus – Hepatitis D requires a co-infection with Hepatitis B, so this is less likely in isolation.

Correct Answer: (C) Hepatitis C Virus

Explanation: Hepatitis C is the most likely cause due to the patient's history of IV drug use and blood transfusions, both major risk factors for Hepatitis C transmission.

2. A 30-year-old male patient with confirmed appendicitis is prepared for appendectomy. What is the most appropriate preoperative care for this patient?

(A) Administer enema – This is usually done in some abdominal surgeries but not for appendicitis.

**(B) Administer antihistamine –
This is not relevant in the
preoperative care for
appendicitis.**

**(C) Encourage ambulation –
Ambulation is encouraged
post-surgery, not pre-surgery.**

**(D) Stimulate bowel activity –
This is not typically necessary
before appendectomy.**

Correct Answer: (A) Administer enema

Explanation: An enema might be administered in some cases to clear the bowel before surgery, especially if the appendix is inflamed or if there is a risk of infection spreading to the bowel.

3. A 26-year-old male patient who works in a construction firm was brought to the emergency department with suspected large bowel obstruction. What is the most likely clinical presentation that helps diagnose this condition?

(A) Vomiting – Common in bowel obstruction but not definitive.

(B) Dehydration – This can be seen but is not the primary diagnostic clue.

(C) Gross abdominal distention – This is a hallmark sign of large bowel obstruction.

(D) Bowel sounds of 45-50/min – This can indicate peristalsis but does not specifically

diagnose large bowel obstruction.

Correct Answer: (C) Gross abdominal distention

Explanation: Abdominal distention is a key sign of large bowel obstruction due to accumulation of gas and fluid in the bowel.

4. A 64-year-old female patient, unconscious for 4 days due to head trauma and excessive blood loss, requires nasogastric (NG) tube placement. What is the most appropriate technique for determining the length of the NGT?

(A) From the tip of the nose to the earlobe and from the earlobe to the umbilicus – This method is used for determining the appropriate length for NG tube placement.

(B) From the earlobe to the tip of the nose and from the tip of the nose to the xiphoid process – This is another common method but typically used in emergency situations.

(C) From the tip of the nose to the earlobe and from the earlobe to the xiphoid process – Common and appropriate measurement technique.

(D) From the tip of the nose to the earlobe and halfway between the xiphoid process and umbilicus – This might be less commonly used but still acceptable.

Correct Answer: (C) From the tip of the nose to the earlobe and from the earlobe to the xiphoid process

Explanation: This is the most common and accurate method for determining the length of the NGT.

5. A 62-year-old obese male patient diagnosed with duodenal peptic ulcer disease presents with abdominal pain. What is the most likely clinical manifestation the nurse would use to diagnose this patient?

(A) Ingestion of food relieves pain – Common symptom in duodenal ulcers, as food can neutralize stomach acid.

(B) Pain occurs one hour after meals – This is more typical of gastric ulcers, not duodenal ulcers.

(C) Being obese – Obesity can be a risk factor but not a direct clinical manifestation.

(D) Relieved by vomiting – Not typically seen in duodenal ulcers; this is more related to gastric ulcers.

Correct Answer: (A) Ingestion of food relieves pain

Explanation: Duodenal ulcers often cause pain that is relieved by food or antacids because food buffers the stomach acid, which irritates the ulcer.

6. A 40-year-old male patient was admitted with subdural hematoma. Assessment showed decreased blood pressure and oxygen saturation. The nurse administered nasal oxygen. What is the most likely indicator of a good prognosis following this intervention?

(A) SpO₂ of at least 95% – This is an ideal target for oxygen saturation.

(B) PaCO₂ of at least 20mmHg – This is low and does not correlate with a good prognosis.

(C) Respiratory rate of 28/min – This is a bit high but might be acceptable in acute situations.

**(D) PaO₂ at least 20mmHg –
This is too low and would
indicate inadequate
oxygenation.**

**Correct Answer: (A) SpO₂ of at
least 95%**

**Explanation: A SpO₂ of 95% or
above indicates that
oxygenation is sufficient,
which is an important factor in**

assessing recovery and prognosis.

7. A 72-year-old female patient presents with symptoms of dizziness, blurred vision, and increased appetite. Her random blood glucose level is 320 mg/dL. What is the priority nursing diagnosis?

(A) Activity intolerance – This is not the priority at the moment.

(B) Impaired verbal communication – This is not related to the symptoms.

(C) Impaired skin integrity – There is no indication of skin issues.

(D) Ineffective health maintenance – Given the symptoms of uncontrolled diabetes and high blood glucose, this is the most appropriate diagnosis.

Correct Answer: (D) Ineffective health maintenance

Explanation: The patient's uncontrolled blood glucose suggests ineffective health

maintenance, especially in managing diabetes.

8. A 50-year-old male patient was transferred to the surgical unit after thyroid surgery. The nurse needs to monitor for early postoperative complications due to injury to

adjacent glands. What is the most important action?

(A) Administering calcium gluconate – This is critical if there's hypocalcemia, often after thyroid surgery.

(B) Monitoring hydration status – Important but secondary to monitoring for tetany or hypocalcemia.

(C) Monitoring symptoms of tetany – Tetany can result from injury to the parathyroid glands, which control calcium levels.

(D) Monitoring input and output – Less important than monitoring for symptoms of tetany.

Correct Answer: (C) Monitoring symptoms of tetany

Explanation: Injury to the parathyroid glands during thyroid surgery can lead to hypocalcemia, which manifests as tetany.

9. A 54-year-old female patient with symptoms of shortness of

breath, constipation, loss of appetite, weight gain, and cold intolerance is suspected of having primary hypothyroidism. What is the most likely laboratory finding to confirm this diagnosis?

(A) Normal thyroid hormone – Would not indicate hypothyroidism.

(B) Decreased thyroid hormone – Common in hypothyroidism, but needs to be paired with TSH levels.

(C) Increased thyroid stimulating hormone (TSH) – In hypothyroidism, TSH is usually elevated as the pituitary tries to stimulate the thyroid.

(D) Decreased thyroid stimulating hormone (TSH) –

**This is unlikely in
hypothyroidism.**

**Correct Answer: (C) Increased
thyroid stimulating hormone
(TSH)**

**Explanation: In
hypothyroidism, the thyroid
gland fails to produce enough
hormones, so the pituitary
secretes more TSH to**

compensate, leading to high levels of TSH.

10. A 38-year-old patient experiences complications after a cesarean section, including pain, episiotomy wound complications, and delayed healing. What is the most likely nursing diagnosis?

(A) Self-care deficit – This might be relevant but is not specific to the wound issues.

(B) Delayed wound healing – Likely, given the delayed healing of the episiotomy wound.

(C) Discomfort related to sexuality – This is not the primary issue at the moment.

**(D) Impaired tissue integrity –
Given the wound issues, this is
the most appropriate
diagnosis.**

**Correct Answer: (B) Delayed
wound healing**

**Explanation: Delayed healing
of the episiotomy wound is the
most likely diagnosis, which
can be caused by various**

factors, including infection or poor nutritional status.

1. Complaints of pain, episiotomy wound complications, and delayed healing with poor continuous discharge. What is the most likely nursing diagnosis for this mother?

(A) Self-care deficit – While this may be relevant, it's not

the primary concern based on the symptoms described.

(B) Delayed wound healing – This is the most likely diagnosis, as the mother is experiencing complications with the episiotomy wound, which is healing poorly.

(C) Discomfort related to sexuality – This is less likely to be the main issue here.

**(D) Impaired tissue integrity –
This is a possibility, but
"delayed wound healing"
specifically addresses the issue
better.**

**Correct Answer: (B) Delayed
wound healing**

**Explanation: The episiotomy
wound is not healing properly,
and this diagnosis directly**

**addresses the issue of
impaired wound healing and
potential infection.**

2. A nurse is caring for a two-day postnatal mother. The nursing care plan focuses on promoting health and preventing disease for both the mother and the newborn.

What is the most appropriate intervention to include?

(A) Episiotomy care for the mother – Important, but it doesn't address the broader goal of promoting health and disease prevention for both mother and newborn.

(B) Treatment of postnatal infection – This is important but should be part of the

overall care rather than the main intervention.

(C) Inspecting the vagina for bleeding – While bleeding inspection is critical in the early postnatal period, this intervention focuses more on a specific aspect.

(D) Providing iron and folic acid supplementation – This is the best option for both

mother and newborn to promote health and prevent complications such as anemia and developmental issues in the baby.

Correct Answer: (D) Providing iron and folic acid supplementation

Explanation: Iron and folic acid are important for preventing anemia in the mother and

ensuring proper development of the newborn. It's a general, preventative measure for both the mother and newborn.

3. A pregnant mother delivered a 3.8 kg neonate with vacuum assistance. One hour after delivery, the uterus is contracted, and there is

bright red vaginal bleeding. On examination, there is a laceration of the vagina and cervix. The nurse starts an IV fluid transfusion. What is the most appropriate next management for this woman?

(A) Repairing the tears – This is necessary, as the bleeding is likely coming from the vaginal and cervical lacerations, but other steps must be taken first.

(B) Give Oxytocin 10 units IM – This can help contract the uterus, reducing further bleeding if uterine atony is suspected.

(C) Removal of the placenta manually – This is not indicated since the uterus is already well contracted, and the cause of bleeding seems to be the laceration.

(D) Bimanual compression of the uterus – This would be useful if uterine atony was present, but in this case, the uterus is contracted, so this is unlikely to be necessary.

Correct Answer: (A) Repairing the tears

Explanation: Since the bleeding is coming from

vaginal and cervical lacerations, the first priority is to repair these tears to stop the bleeding.

27. A male patient is dispensed with non-tachycardia and tachypnea. What is the most likely nursing diagnosis for this case?

(A) Ineffective breathing pattern – This is a likely

diagnosis, as tachypnea (increased respiratory rate) often indicates issues with the breathing pattern.

(B) High risk for hypoventilation – This could be relevant if the tachypnea were compensatory for hypoventilation, but the description does not suggest this.

(C) Ineffective airway clearance
– This would be relevant if the patient had signs of obstruction or difficulty clearing secretions, but there's no mention of this.

(D) Ineffective tissue perfusion
– This is a possibility if there are signs of inadequate oxygenation, but there's not enough information to indicate this.

Correct Answer: (A) Ineffective breathing pattern

Explanation: Tachypnea is a common sign of an ineffective breathing pattern, often associated with respiratory distress or compensation for hypoxia.

28. A 11-week-old neonate with neonatal jaundice has a temperature of 36.2°C, pulse rate of 120/min, and a random blood sugar of 15 mg/dL. What is the most likely risk factor for hypothermia in this neonate?

(A) Being preterm – Preterm neonates are at higher risk for hypothermia because they have less body fat and less ability to regulate body temperature.

(B) Having hypoglycemia – Hypoglycemia can increase the risk of hypothermia as the body loses energy that would otherwise be used to maintain normal temperature.

(C) Adverse effect of phototherapy – Phototherapy can lower body temperature, but this would not be the most likely risk factor here.

(D) Undressing it for phototherapy – Undressing the neonate can indeed lead to heat loss, but hypothermia is more likely associated with other factors.

Correct Answer: (A) Being preterm

Explanation: Preterm infants have a higher risk of

hypothermia because they lack the fat stores and thermoregulatory mechanisms necessary to maintain body temperature.

29. A 34-week-old neonate with neonatal jaundice and sepsis is admitted to the NICU. The neonate is having difficulty

feeding, has a temperature of 36.2°C, pulse rate of 120/min, and a random blood sugar of 34 mg/dL. What is the most priority nursing intervention for this neonate?

(A) Treat hypothermia – Although the neonate's temperature is low, the priority intervention is addressing the low blood sugar.

(B) Treat neonatal sepsis –
Sepsis is a concern but would not be the most urgent in this context.

(C) Treat hypoglycemia –
Hypoglycemia is the most immediate concern and requires prompt treatment to avoid complications.

(D) Treat neonatal jaundice –
Jaundice typically requires

treatment but is not as urgent as hypoglycemia in this case.

Correct Answer: (C) Treat hypoglycemia

Explanation: The neonate has a very low blood sugar (34 mg/dL), which requires immediate intervention to prevent further complications like brain damage.

30. A two-day-old preterm male neonate with neonatal conjunctivitis was admitted to the NICU. He shows chest in-drawing, an expiratory grunt, lips turning blue, pulse rate of 170/min, respiratory rate of 80/min, and a temperature of 36.9°C. What is the most appropriate initial step in the management of this patient?

**(A) Advise on oral feeding –
This would be inappropriate
given the respiratory distress.**

**(B) Administer maintenance
fluid – Fluids might be
necessary, but respiratory
distress requires more
immediate intervention.**

**(C) Administer supplementary
oxygen – This is the most**

appropriate intervention, as the neonate is showing signs of respiratory distress and hypoxia (cyanosis).

(D) Maintain the temperature range – This is important but secondary to addressing the neonate's respiratory distress.

Correct Answer: (C) Administer supplementary oxygen

Explanation: The neonate shows signs of hypoxia (cyanosis, chest in-drawing), so administering oxygen is the priority to stabilize the patient's oxygen levels.

31. A neonate with clinical manifestations of respiratory distress, fetal stress, and

meconium-stained amniotic fluid is admitted. Physical examination shows central cyanosis, chest in-drawing, pulse rate of 170/min, and respiratory rate of 80/min. What is the most likely complication this infant may develop?

(A) Meningitis – Meningitis is a possibility in any newborn but is less likely in this case.

(B) Jaundice – Jaundice is common in neonates but would not be the most immediate concern here.

(C) Shock – Shock is a possibility, especially with respiratory distress and meconium aspiration, but it would be secondary to the respiratory distress.

(D) Anemia – Anemia could develop, but it is not the most likely complication given the signs of respiratory distress.

Correct Answer: (C) Shock

Explanation: The neonate is showing signs of respiratory distress and central cyanosis, which could lead to shock if not addressed promptly.

Respiratory failure and

subsequent shock are common complications following meconium aspiration syndrome.

12. A baby presents with hydrops fetalis, pallor, hepatosplenomegaly, edema, petechiae, and ascites. The Coombs test is positive, and the hemoglobin value is 9 g/dl. What is the most likely clinical sign indicating hemolytic disease in this case?

(A) Hepatosplenomegaly – While this is seen in hemolytic disease, it is not the most characteristic sign of hemolytic disease.

(B) Edema and ascites – These could be a result of severe anemia and fluid overload, but they are more general signs of hydrops fetalis.

(C) Severe jaundice – Jaundice is a prominent sign in hemolytic disease due to the rapid breakdown of red blood cells.

(D) Hydrops fetalis – This is a severe form of fetal anemia, often resulting from hemolytic disease, characterized by edema, ascites, and heart failure.

Correct Answer: (D) Hydrops fetalis

Explanation: Hydrops fetalis is commonly associated with severe hemolytic disease, often caused by Rh incompatibility or other immune-mediated causes.

13. A nurse is assessing a mother on breastfeeding techniques. During observation, the infant sucks five times before pausing, and a gurgling sound is heard during swallowing. What is the most important additional finding that can show the adequacy of breastfeeding?

(A) Chin of the infant should touch the breast – This is important for latch, but not

the most critical sign for adequacy.

(B) The infant's mouth should be widely open – A wide mouth helps achieve a good latch, which is essential for effective feeding.

(C) More areola seen on the upper part of the breast – This is not an accurate indicator of proper breastfeeding.

(D) Dimpling of cheeks may be seen while suckling – Dimpling cheeks indicate that the infant is actively sucking and drawing milk effectively.

Correct Answer: (D) Dimpling of cheeks may be seen while suckling

Explanation: Dimpling of the cheeks is a sign that the infant

is properly latching and sucking effectively.

14. A 10-month-old child with clinical features of severe wasting, edema of the lower extremities, and palmar pallor is admitted to the hospital. What is the priority nursing intervention for this child?

(A) Mas folate – Folate may be necessary but is not the priority intervention here.

(B) IV fluid infusions – This is important for rehydration, especially in cases of severe malnutrition with edema.

(C) Deworming – Deworming may be considered, but addressing the child's severe

nutritional deficits is more urgent.

**(D) Folic acid supplementation
– Similar to folate, this is not the most urgent intervention.**

Correct Answer: (B) IV fluid infusions

Explanation: Severe malnutrition often leads to dehydration, and IV fluids are

essential to stabilize the child before proceeding with further interventions.

15. A child with complicated severe acute malnutrition was not responding to treatments and was classified under failure to respond to treatment. What is the most

likely indicator of failure to treatment?

(A) Failure to lose edema on day 10 – Edema resolution is a good indicator of treatment progress, but failure to resolve by day 10 suggests treatment failure.

(B) Failure to regain appetite on day 10 – Appetite is a critical indicator, but it is not

as central as weight gain in evaluating progress.

(C) If edema persists for more than 10 days – Persistent edema suggests that the child is not responding to the treatment as expected.

(D) Failure to gain at least 5 g/kg body weight in day four – Failure to gain weight is a key

sign that the treatment is not effective.

Correct Answer: (D) Failure to gain at least 5 g/kg bodyweight in day four

Explanation: Weight gain is a key indicator of recovery in children with severe acute malnutrition, and failure to gain weight within the first few

days suggests that the treatment is not working.

16. A nurse is checking the health status of children prior to vaccinations. What is the most likely contraindication to be ruled out by the nurse before vaccination?

(A) Malnutrition – Malnutrition may affect immune responses but does not necessarily contraindicate vaccination.

(B) Chronic non-communicable diseases – These are generally not contraindications for most vaccines.

(C) Symptomatic HIV infection – Symptomatic HIV infection is

a contraindication for certain live vaccines.

(D) Family history of allergy to vaccines – Family history of allergies is generally not a contraindication unless there is a known allergy to a specific component of the vaccine.

**Correct Answer: (C)
Symptomatic HIV infection**

Explanation: Children with symptomatic HIV infection may not be able to receive live vaccines due to immunocompromise.

17. A nurse working in an immunization unit is responsible for maintaining the potency of vaccines. What

**is the best way to monitor
vaccine refrigerator
temperature?**

**(A) Measuring vaccine
refrigerator temperature daily
for seven days – This is less
frequent than required.**

**(B) Measuring vaccine
refrigerator temperature twice
a day for seven days – This**

ensures more consistent monitoring.

(C) Measuring vaccine refrigerator temperature once per day on working days – This is not frequent enough.

(D) Measuring vaccine refrigerator temperature twice per day on working days – This is the correct frequency for monitoring to ensure the

temperature is within the required range.

Correct Answer: (D) Measuring vaccine refrigerator temperature twice per day on working days

Explanation: Vaccines must be stored at specific temperatures to remain effective, and this frequency ensures the proper storage conditions.

18. A mother brought her seven-week-old child to an immunization clinic. The child was born at home and has not received any vaccinations. What vaccines should the nurse administer?

(A) BCG, rota 1, PCVI, and Penta 1, measles – Measles vaccine is given later.

(B) OPV 1, BCG, rota 1, PCVI, and Penta 1 – This includes the first dose of important vaccines but is missing the OPV0 (oral polio vaccine).

(C) OPV 1, IPV, BCG, rota 1, PCVI, and Penta 1 – IPV (inactivated polio vaccine) is

not needed in this case, as OPV0 would be given.

(D) OPV 0, OPV 1, BCG, rota 1, PCVI, and Penta 1 – This includes the appropriate vaccines for a neonate who missed initial vaccination.

Correct Answer: (D) OPV 0, OPV 1, BCG, rota 1, PCVI, and Penta 1

Explanation: For a child who was born at home and missed vaccinations, this combination of vaccines is appropriate to ensure early protection.

19. A nine-year-old child diagnosed with hearing impairment due to medication side effects is brought to a

clinic. Despite proper medication treatment, the child's hearing is limited, and the child cannot identify the meaning of words. What is the most likely nursing diagnosis?

(A) Impaired psychological status – The child may face psychological challenges due to hearing impairment, but this is secondary.

(B) Impaired communication skill – The child's inability to understand words is a clear communication difficulty.

(C) Impaired activities of daily living – This may be affected, but communication is more central.

(D) Impaired daily activities – This is too broad and less

specific to the child's main issue.

Correct Answer: (B) Impaired communication skill

Explanation: The primary issue for this child is impaired communication due to hearing loss.

20. A six-year-old child is admitted to a hospital for physical rehabilitation due to a physical disability. The child has bowed legs, delayed motor development, and a history of gross motor developmental delay. What is the most likely cause of this disability?

(A) Iron deficiency – Iron deficiency does not typically

cause the skeletal deformities seen here.

(B) Calcium deficiency – Calcium deficiency can contribute to bone problems but is not as likely as vitamin D deficiency.

(C) Vitamin D deficiency – This is a common cause of rickets, which results in bowed legs and delayed development.

**(D) Phosphorus deficiency –
Phosphorus deficiency can
contribute to bone deformities
but is less common than
vitamin D deficiency.**

**Correct Answer: (C) Vitamin D
deficiency**

**Explanation: Vitamin D
deficiency leads to rickets,**

causing bowed legs and other skeletal deformities.

4. A woman using a combined oral contraceptive complains of nausea, mood changes, decreased sex drive, and weight gain. What is the most likely sign indicating a side effect of the contraceptive?

(A) Nausea – Nausea is a common side effect of oral contraceptives but may not be

the most significant or primary complaint.

(B) Weight gain – Weight gain is a common side effect of hormonal contraceptives and is often a primary complaint when considering side effects.

(C) Mood change – Hormonal changes can affect mood, but it may not be the first sign noticed.

(D) Decreased sex-drive – A decrease in libido can occur with hormonal contraceptives, but it may not be as obvious to the user as other side effects.

Correct Answer: (B) Weight gain

Explanation: Weight gain is a well-known and frequent side

**effect of combined oral
contraceptives.**

**5. A 35-year-old woman is
admitted with vesicovaginal
fistula. She has been isolated
from society due to urine
incontinence. Her HCT is 30%,
and urinalysis is normal. What**

is the priority nursing diagnosis?

(A) Acute confusion – There is no evidence suggesting confusion.

(B) Sexual dysfunction – This is a possible concern, but not the priority issue at this stage.

(C) Post-trauma syndrome – This could be a concern but

does not fit the current scenario perfectly.

**(D) Disturbed personal identity
– The isolation and humiliation due to incontinence suggest an issue with her sense of self-worth.**

Correct Answer: (D) Disturbed personal identity

Explanation: The woman's feelings of humiliation and isolation are central to her psychological and emotional state, making this diagnosis the most fitting.

6. A 45-year-old woman with second-degree uterine prolapse presents with bearing

down sensation, dragging discomfort, and low back pain. What is the most appropriate nursing care for this patient?

(A) Teach about patient self-care – Important but not the priority at this moment.

(B) Advise the mother about exercise – Exercise may be helpful but should be done after surgical intervention.

(C) Physical and psychological preparation – This includes both physical and emotional readiness for surgery, which is the most relevant care at this stage.

(D) Prescribe and administer sedative drugs – Sedatives are not the priority in this case.

Correct Answer: (C) Physical and psychological preparation

Explanation: Preparing the patient physically and psychologically for surgery is crucial at this stage.

7. A 43-year-old woman complains of vaginal bleeding for two months and profuse

discharge. Pathology shows stromal invasion of the cervix with a depth of 3 mm and horizontal extension of 5 mm. What is the most likely stage of cervical cancer?

(A) Stage-IB – This could be the correct stage based on the description of the cervical cancer's spread.

(B) Stage-IB – This appears to be a typographical error.

(C) Stage-IA – This would indicate a more localized stage, which does not match the pathology described.

(D) Stage-IA – Similar to (C), this is incorrect.

Correct Answer: (A) Stage-IB

Explanation: Based on the stromal invasion and extent of the cancer, stage IB fits the clinical presentation.

8. A woman with breast cancer is admitted for chemotherapy. What is the priority nursing intervention?

(A) Giving analgesics – Pain management is important, but it's not the priority before starting chemotherapy.

(B) Giving cytotoxic agent – This is the goal of the treatment, but administering it is typically done under careful supervision and after adequate preparation.

**(C) Advice to take caffeine –
This is not relevant to
chemotherapy preparation.**

**(D) Aspiration of cysts – This
would be done only if
necessary, but chemotherapy
is the next step.**

**Correct Answer: (B) Giving
cytotoxic agent**

Explanation: The main intervention in chemotherapy is administering the cytotoxic agent, although pre-treatment assessments and preparations should be done.

9. A toddler is brought to the emergency department after a near-drowning incident. The

child is shivering, grunting, and has severe intercostal muscle retraction with cyanosis. What is the most likely nursing diagnosis?

(A) Risk for aspiration – Aspiration is a concern, but the primary issue is respiratory distress.

(B) Risk for hypothermia – This could be a concern, but the

child's respiratory distress is the primary issue.

**(C) Ineffective airway clearance
– Given the respiratory distress, this is a likely diagnosis due to obstruction or difficulty in breathing.**

(D) Ineffective breathing pattern – This diagnosis fits the presentation of respiratory

**distress and difficulty
breathing.**

**Correct Answer: (C) Ineffective
airway clearance**

**Explanation: The child is
displaying signs of airway
obstruction and respiratory
distress, which makes airway
clearance the priority.**

10. A child is admitted after salicylate poisoning with convulsions, dehydration, and hyperventilation. The child is managed with lavage, charcoal, isotonic fluid, sodium bicarbonate, and potassium supplementation. What laboratory value indicates progress?

(A) Low serum K⁺ values – This would indicate worsening of the child's condition, not improvement.

(B) Urine pH of greater than 7.0 – A urine pH greater than 7.0 indicates that the alkalization treatment is working to reverse the effects of salicylate poisoning.

(C) Urine flow of 1 mL/kg/h established – This is important but not the primary indicator of progress.

(D) Laboratory value indicating metabolic acidosis – This would indicate worsening, not improvement.

Correct Answer: (B) Urine pH of greater than 7.0

Explanation: Alkalinization of the urine is a key treatment goal in salicylate poisoning to enhance elimination of the drug.

11. A 12-year-old child presents with swelling of the knees, ankles, and elbows, with hot, red, tender, and

migratory swelling. What additional manifestation is needed to confirm a diagnosis of acute rheumatic fever?

(A) Fever – Fever is a common symptom of acute rheumatic fever, so it is likely a part of the diagnostic criteria.

(B) Carditis – Carditis is a major diagnostic criterion for acute rheumatic fever.

(C) Arthralgia – Arthralgia is part of the presentation but not sufficient for confirmation.

(D) Positive throat culture – A positive throat culture for Group A Streptococcus is needed for diagnosis.

Correct Answer: (B) Carditis

Explanation: The presence of carditis, in addition to migratory arthritis, is a major diagnostic feature of acute rheumatic fever.

12. An adolescent complains of weakness and pain in the legs after exercise. On examination, the adolescent has a bounding

pulse in the arms, weak femoral pulse, and lower blood pressure in the lower extremities compared to the upper extremities. What is the most likely diagnosis?

(A) Atrial septal defect – This is not the best match for the physical findings.

(B) Pulmonary stenosis – This would not explain the specific physical findings described.

(C) Coarctation of the aorta – This is the most likely diagnosis, as it explains the difference in blood pressure between the upper and lower extremities.

Correct Answer: (C)

Coarctation of the aorta

Explanation: Coarctation of the aorta causes high blood pressure in the upper body and low blood pressure in the lower body, explaining the symptoms described.

13. An adolescent presents with fever, erythema, and focal tenderness over the tibial bone. The culture reveals methicillin-resistant *Staphylococcus aureus*

(MRSA). What is the most appropriate drug of choice for this patient?

(A) Nafcillin and cefotaxime – Nafcillin is not effective against MRSA, so this option is incorrect.

(B) Oxacillin and cefotaxime – Oxacillin is ineffective against MRSA, making this combination inappropriate.

(C) Vancomycin and oxacillin – Vancomycin is effective against MRSA, but oxacillin is not.

(D) Vancomycin and cefotaxime – Vancomycin is appropriate for treating MRSA, and cefotaxime can be used for broader coverage if necessary.

Correct Answer: (D)

Vancomycin and cefotaxime

Explanation: Vancomycin is effective for MRSA, and cefotaxime provides additional coverage against potential other infections.

14. A 7-year-old child presents with vesicular lesions that

initially were tiny and rapidly changed to honey-colored crusted plaques around the mouth. What is the most likely diagnosis?

(A) Impetigo – This is the most likely diagnosis, as the honey-colored crust is characteristic of impetigo, often caused by *Staphylococcus aureus*.

(B) Cellulitis – Cellulitis is a deeper skin infection and would typically present with more extensive redness and swelling.

(C) Tinea corporis – This fungal infection presents as ring-shaped lesions, not vesicular or crusted plaques.

(D) Varicella-zoster – This presents with fluid-filled

blisters but often in a different pattern, typically appearing in a band or around the torso.

Correct Answer: (A) Impetigo

Explanation: The rapid transition of vesicles to honey-colored crusts is a hallmark of impetigo.

15. A 2-year-old child presents with a barking cough that worsens at night and when crying. The child also has low-grade fever and slightly increased respiratory rate. What is the most likely diagnosis?

(A) Whooping cough – This typically presents with a characteristic "whoop" after

coughing but is not known for a barking cough.

(B) Epiglottitis – This presents with sudden onset of high fever, drooling, and difficulty swallowing, not a barking cough.

(C) Croup – Croup is characterized by a barking cough, particularly worsened at night and with crying.

(D) Pertussis – Pertussis presents with a characteristic paroxysmal cough, but a barking cough is more suggestive of croup.

Correct Answer: (C) Croup

Explanation: The barking cough and worsening symptoms at night point to

croup, a viral respiratory infection.

16. A 5-year-old child presents with pain on swallowing and shortness of breath for 3 days. Examination reveals edematous and reddish pharyngeal and tonsillar areas. The child's z-score for weight

for height is -3. What is the priority nursing diagnosis for this child?

(A) Imbalanced nutritional pattern – This is a concern due to the child's poor weight-for-height ratio but not the priority issue.

(B) Ineffective breathing pattern – The child has

shortness of breath, so this is a relevant concern.

**(C) Risk for airway obstruction
– Given the edematous tonsils and difficulty swallowing, this is the most pressing concern.**

**(D) Body fluid disturbance –
This could be a concern but is not the most immediate issue.**

Correct Answer: (C) Risk for airway obstruction

Explanation: The swelling of the tonsils and difficulty breathing makes airway obstruction the priority.

17. A 3-year-old child with diabetes presents with cough and fever. After starting

antibiotics, the child's blood sugar increases to 350 mg/dL and they are drinking more water. What is the most likely outcome criterion to discharge this child?

(A) Random plasma sugar of 70 mg/dL – This is too low and not appropriate for discharge.

(B) Random plasma sugar of 195 mg/dL – This may still be

too high for a child with diabetes.

(C) Random plasma sugar of 200 mg/dL – This is still higher than ideal for a child with diabetes but closer to normal levels.

(D) Random plasma sugar of 110 mg/dL – This is a safe and controlled level for discharge.

Correct Answer: (D) Random plasma sugar of 110 mg/dL

Explanation: A random blood sugar of 110 mg/dL is within the desired range for a child with diabetes and would indicate stability for discharge.

18. A patient with HIV presents with blisters on the shoulder, which do not pass midline of the body. The patient reports tingling sensations and severe pain, and experiences discomfort when clothing touches the affected area. What is the most appropriate management for this patient?

**(A) Administer Tramadol –
Tramadol could help with pain**

but does not address the underlying cause.

**(B) Administer antibiotics –
This is not appropriate unless there is a secondary infection, but the symptoms suggest a viral etiology.**

**(C) Administer Acyclovir –
Acyclovir is appropriate for treating herpes zoster, which**

**can cause blisters and pain
along dermatomes.**

**(D) Administer Calamine lotion
– Calamine lotion would not be
effective for herpes zoster and
does not address the viral
cause.**

**Correct Answer: (C) Administer
Acyclovir**

Explanation: Herpes zoster (shingles) is likely, and acyclovir is the antiviral treatment of choice.

19. A 59-year-old woman with a history of long-term NSAID use presents with epigastric fullness, nausea, and belching. What is the most important

additional question the nurse should ask?

(A) "Have you ever noticed a change in the odor of your breath?" – This is not directly related to NSAID use or the current symptoms.

(B) "Have you noticed any problems with your breathing?" – Breathing issues

are not the primary concern here.

(C) "Have you ever vomited blood or had very dark stools?" – This is crucial to rule out a potential GI bleed, a known complication of long-term NSAID use.

(D) "Did you experience nausea always whenever you take the drug?" – Nausea is

common with NSAID use but doesn't address the potential for a more serious complication.

Correct Answer: (C) "Have you ever vomited blood or had very dark stools?"

Explanation: Long-term NSAID use can lead to gastrointestinal bleeding, and this question directly addresses that risk.

20. A 22-year-old male presents with severe generalized abdominal pain that worsens in the right lower quadrant after 24 hours. The abdomen is rigid and there are normal bowel sounds. The temperature is 37.9°C. What is the most likely complication of

**this patient's suspected
appendicitis?**

**(A) Perforation – This is a
common and serious
complication of appendicitis.**

**(B) Abscess – An abscess may
form after a perforated
appendix, but it is not the
initial complication.**

(C) Peritonitis – This can occur after perforation but is a later complication.

(D) Infection – Infection is a concern but is part of the overall process after perforation or peritonitis.

**Correct Answer: (A)
Perforation**

Explanation: The clinical picture suggests appendicitis, and perforation is a common and serious complication.

21. A 58-year-old hypertensive woman with a stroke is comatose, and a nurse plans to insert an NG tube for feeding. What is the indicator used to

confirm proper placement of the NG tube?

(A) Bubbling – Bubbling is not used to confirm NG tube placement.

(B) Clear fluid aspiration – Aspiration of clear fluid could indicate proper placement in the stomach.

(C) Aspiration with pH value of 4 – Gastric pH is typically acidic and would confirm NG tube placement in the stomach.

(D) Litmus paper color change from red to blue – This tests for acidity but is not specific for confirming NG tube placement.

**Correct Answer: (C) Aspiration
with pH value of 4**

**Explanation: Gastric aspirate
with a pH of around 4 confirms
the NG tube is properly placed
in the stomach.**

**Here are the answers and
explanations for your
questions:**

1. A 15-year-old female presents with difficulty breathing for 2 hours. Physical examination shows resonant chest sounds on percussion. Chest X-ray reveals narrowing of the airway. What is the most likely diagnosis?

(A) Bronchial asthma – Asthma is characterized by airway narrowing, which aligns with the patient's symptoms.

(B) Allergic reaction – While allergic reactions can cause respiratory distress, the narrowing of airways is more suggestive of asthma.

(C) Plugged mucus – This could cause difficulty breathing but is less likely to show the narrowing seen in asthma.

(D) Pneumonia – Pneumonia would typically present with

abnormal lung sounds and consolidation rather than just airway narrowing.

Correct Answer: (A) Bronchial asthma

Explanation: The narrowing of airways and difficulty breathing align with a diagnosis of asthma.

2. A 45-year-old male working in a factory has been diagnosed with chronic bronchitis. After assessment, the nurse develops a diagnosis of "impaired gas exchange due to chronic inhalation of irritants." What is the most likely predicted goal for this nursing diagnosis?

(A) Improvement in blood oxygenation – This directly addresses the impaired gas exchange.

(B) Independency in self-care activities – This is important but not the most direct goal for impaired gas exchange.

(C) Improvement in activity tolerance – This is a secondary goal, but improving blood

oxygenation is more directly tied to gas exchange.

(D) Achievement of airway clearance – This could be a relevant goal but does not directly address the impairment in gas exchange itself.

Correct Answer: (A)
**Improvement in blood
oxygenation**

**Explanation: The goal for
impaired gas exchange is to
improve blood oxygenation,
which is the core issue.**

**3. A 45-year-old female
underwent extensive**

abdominal surgery and has been unconscious for several days. The nurse suspects pulmonary embolism. What is the most likely assessment finding that led the nurse to suspect this diagnosis?

(A) Increased blood pressure, chills, jugular vein distension, and painful cough – These findings are more typical of other conditions, not pulmonary embolism.

(B) Lethargy, nausea, vomiting, and diminished heart sounds – These symptoms could suggest other complications, not pulmonary embolism.

(C) Fever, right upper quadrant abdominal pain, and dyspnea – This could indicate a complication like a liver issue or infection, not necessarily a pulmonary embolism.

(D) Chest pain, shortness of breath, and tachypnea – These are classic symptoms of pulmonary embolism.

Correct Answer: (D) Chest pain, shortness of breath, and tachypnea

Explanation: Chest pain, shortness of breath, and

tachypnea are hallmark signs of a pulmonary embolism.

4. A patient experiences flushing, itching, and anxiety after receiving the first dose of procaine penicillin. What is the most appropriate immediate management for this hypersensitivity reaction?

**(A) Administer appropriate dose of epinephrine –
Epinephrine is commonly used in anaphylaxis or severe allergic reactions.**

**(B) Take history about hypersensitivity reaction –
While this is useful, it's not an immediate intervention.**

(C) Administer antihistamines
– Antihistamines are helpful
but not the first-line treatment
for severe allergic reactions.

(D) Immediately refer to
hospital for better care –
Referral may be needed, but
immediate treatment should
be the priority.

Correct Answer: (A) Administer appropriate dose of epinephrine

Explanation: Immediate administration of epinephrine is crucial for managing severe allergic reactions, especially anaphylaxis.

5. A 57-year-old woman presents to the surgical emergency unit with multiple rib fractures. On assessment, there is paradoxical chest-wall movement during inspiration and expiration. What is the priority nursing intervention next to immobilization?

(A) Administering pain relief – Pain relief is important, but it's not the immediate priority in this situation.

(B) Monitoring signs of respiratory infection – This is important but is secondary to managing breathing difficulties in the acute phase.

(C) Instructing the patient to breathe on the affected side – This could worsen the condition by causing further respiratory distress.

(D) Instructing the patient to take deep breaths – This is essential for preventing atelectasis and improving lung expansion.

Correct Answer: (D) Instructing the patient to take deep breaths

Explanation: Deep breathing is crucial to ensure adequate lung expansion and prevent

complications like pneumonia or atelectasis.

6. A 68-year-old man with a history of bronchiectasis has been diagnosed with heart failure. He breathes easily only in a sitting position. What is the priority nursing diagnosis for this patient?

(A) Anxiety – Anxiety might be present, but it is not the priority diagnosis in this case.

(B) Activity intolerance – This could be a concern, but the priority is managing his airway and breathing.

(C) Ineffective airway clearance – This is the priority, as bronchiectasis and heart

failure affect the ability to clear the airway.

(D) Impaired physical comfort
– While the patient may experience discomfort, the priority is airway clearance.

Correct Answer: (C) Ineffective airway clearance

Explanation: Ineffective airway clearance is a priority in this

patient with bronchiectasis and heart failure, as he may have difficulty clearing mucus or secretions.

7. A 25-year-old female patient with a dislocated left lower extremity was treated with immobilization and reduction. What is the most likely

**instruction the nurse should
notify the patient?**

**(A) Do not flex the hip – This
may not be relevant to the
patient's injury.**

**(B) Reduce range of motion –
Limiting range of motion might
be important in the early
healing phase, but further
clarification is needed.**

(C) Report abnormal sensation
– This is a key instruction, as it would alert the nurse to potential complications like nerve damage.

(D) Keep the knees apart – This might be more applicable to hip replacement or other conditions.

Correct Answer: (C) Report abnormal sensation

Explanation: Reporting abnormal sensations is important to monitor for complications such as nerve or vascular damage.

Here are the answers and explanations for your questions:

1. A 44-year-old diabetic man with an infected foot and a recommendation for amputation. What should be the priority patient outcome?

(A) Identifies signs and symptoms of hypoglycemia or hyperglycemia – This is essential for managing diabetes, but it does not directly address the immediate issue of foot care.

(B) Demonstrates proper technique for administering insulin injection – While this is important for overall diabetes management, it does not directly help with the foot infection.

(C) Demonstrates appropriate knowledge of disease selection and proper foot care – This is crucial because foot care is directly related to preventing

**further complications,
especially in diabetic patients.**

**(D) Maintains skin integrity
and proper foot care – Skin
integrity is directly related to
the infection and long-term
care.**

**Correct Answer: (C)
Demonstrates appropriate**

**knowledge of disease selection
and proper foot care**

**Explanation: This is the most
relevant outcome as proper
foot care is key in managing
diabetic foot complications
and preventing further
damage.**

2. A 26-year-old male with burning eye pain, purulent discharge, and lymphadenopathy. What is the most likely type of conjunctivitis?

(A) Viral – Viral conjunctivitis often presents with watery discharge and might not have significant purulent discharge or lymphadenopathy.

(B) Toxic – Toxic conjunctivitis can occur due to irritation, but it does not usually cause purulent discharge.

(C) Allergic – Allergic conjunctivitis typically presents with itchy, watery eyes and is not associated with purulent discharge.

(D) Bacterial – Bacterial conjunctivitis typically

presents with purulent discharge, and lymphadenopathy is common.

Correct Answer: (D) Bacterial

Explanation: The symptoms of purulent discharge and lymphadenopathy are indicative of bacterial conjunctivitis.

3. A 24-year-old male patient complains of itching, burning sensation, and purulent discharge in both eyes, making it hard to open them. What is the priority nursing diagnosis?

(A) Anxiety – While anxiety might be present, the priority diagnosis should address the physical symptoms.

(B) Acute pain – Acute pain is a concern but doesn't encompass the full impact of the condition.

(C) Impaired mucosal lining – This could be relevant, but it's more focused on the underlying physical issue.

(D) Disturbed sensory perception – Given the

symptoms, such as difficulty opening the eyes, disturbed sensory perception could be the most appropriate diagnosis.

Correct Answer: (D) Disturbed sensory perception

Explanation: The burning sensation and difficulty in opening the eyes suggest disturbed sensory perception

as the priority nursing diagnosis.

4. A 47-year-old patient presents with difficulty reading and blurred vision.

Ophthalmoscopic examination and tonometry reveal opacity and increased intraocular

pressure. What is the most appropriate management?

(A) Surgery – Surgery may be necessary, particularly for glaucoma or cataracts, where vision is impaired.

(B) Antibiotics – Antibiotics are not appropriate for vision issues related to opacity or increased pressure.

(C) Eye irrigation – Eye irrigation is used for foreign bodies or irritation, not for vision changes or pressure issues.

(D) Corrective eyeglasses – Corrective glasses would not address the underlying issue of increased intraocular pressure or opacity.

Correct Answer: (A) Surgery

Explanation: Increased intraocular pressure suggests glaucoma, and opacity suggests cataracts, both of which may require surgical intervention.

5. A 12-year-old boy from a malaria-endemic area presents

with fever, headache, and body aches. What clinical finding is most helpful for diagnosing complications of malaria?

(A) Fever – Fever is a common symptom of malaria, but it does not indicate complications.

(B) Myalgia – Muscle pain is common in malaria but not

specifically indicative of complications.

(C) Jaundice – Jaundice could be a sign of severe malaria, as it indicates liver involvement.

(D) Headache – While common in malaria, it is not a key indicator of complications.

Correct Answer: (C) Jaundice

Explanation: Jaundice is a significant sign of severe malaria and liver involvement, helping to diagnose complications.

6. A 36-year-old female with a painless, non-itching, hard symmetrical chancre and a positive VDRL test. What is the

**best clinical feature to
diagnose this case?**

**(A) Non-itching chancre – This
is typical of syphilis but is not
the sole distinguishing factor.**

**(B) Chancre on the genital area
– While syphilis typically
presents with genital lesions,
other conditions may also
cause similar lesions.**

(C) Positive serological test – A positive VDRL test confirms syphilis, making this a key diagnostic feature.

(D) Hard symmetrical chancre – This is a classic presentation for syphilis.

Correct Answer: (C) Positive serological test

Explanation: The VDRL test is a reliable serological test for diagnosing syphilis, confirming the diagnosis.

7. A 27-year-old patient presents with acute onset of profuse, purulent urethral discharge, urgency, and a

painless genital ulcer. What is the most likely diagnosis?

(A) Syphilis – Syphilis can cause painless genital ulcers, but purulent discharge and urgency suggest a different condition.

(B) Chancroid – Chancroid is caused by *Haemophilus ducreyi*, and it often presents

with painful ulcers and purulent discharge.

(C) Gonorrhea – Gonorrhea commonly presents with purulent urethral discharge and may be associated with genital ulcers.

(D) Herpes genitalis – Herpes causes painful ulcers, not painless ones.

Correct Answer: (C) Gonorrhea

Explanation: Gonorrhea can present with purulent discharge and painless genital ulcers, making it the most likely diagnosis.

8. An elderly patient diagnosed with rabies after a dog bite

presents with an open wound and tingling at the bite site. What is the initial management?

**(A) Suture the wound –
Suturing should be avoided in cases of rabies exposure to ensure the wound is properly treated.**

**(B) Administer antibiotics –
Antibiotics are not the first-**

line treatment for rabies prevention.

(C) Administer post-exposure prophylaxis – Rabies post-exposure prophylaxis (PEP) is essential in preventing the disease after exposure.

(D) Wash the wound with soap and water – Washing the wound is important but should

be followed by administering PEP.

Correct Answer: (C) Administer post-exposure prophylaxis

Explanation: Post-exposure prophylaxis with rabies vaccine and rabies immunoglobulin is the most critical treatment after potential rabies exposure.

Here are the answers and explanations for your questions:

36. Nursing intervention to facilitate venous blood return from distal areas to the center in a patient with reduced fracture and cast application.

(A) Elevate the leg above the heart – This helps facilitate venous return, as gravity will assist in returning blood from the distal part of the leg.

(B) Have client exercise toes every 15 minutes – Exercise may help with circulation but does not specifically address venous return.

(C) Monitor circulatory status every 30 minutes – Monitoring is important but does not actively assist venous return.

(D) Apply an ice bag on the cast every 20 minutes – Ice helps with swelling, but does not directly improve venous return.

Correct Answer: (A) Elevate the leg above the heart

Explanation: Elevating the leg above the heart is the most effective intervention to facilitate venous return and prevent complications like swelling.

37. Most appropriate site for the nurse to support the wet cast during application.

(A) Firm surface – A firm surface is ideal to prevent deforming the wet cast.

(B) Soft mattress – A soft surface may not provide adequate support and could distort the shape of the cast.

(C) Tips of the fingers – Supporting with fingertips could leave marks on the wet plaster and potentially deform it.

(D) Palms of the hands – Using the palms is the best option to support the weight of the cast evenly and avoid indentations.

Correct Answer: (A) Firm surface

Explanation: A firm surface ensures that the wet cast does not get deformed and can dry properly.

38. What is the most appropriate instruction for a

**patient taking aspirin for
osteoarthritis?**

**(A) Take the medication with
food – Aspirin can irritate the
stomach lining, so taking it
with food can minimize
irritation.**

**(B) Take the medication with
plenty of water – While water
helps with absorption, it is not
the most effective way to**

**avoid gastrointestinal
irritation.**

**(C) Take the medication on an
empty stomach – This
increases the risk of stomach
irritation and ulcers.**

**(D) Take the medication when
symptoms worsen – Aspirin
should be taken regularly for
osteoarthritis, not just when
symptoms worsen.**

Correct Answer: (A) Take the medication with food

Explanation: Taking aspirin with food helps minimize gastrointestinal side effects like irritation or ulcers.

39. Interpretation of Glasgow Coma Scale score for a patient with eye opening to pain, incoherent response, and arm flexion to touch.

(A) Coma – A coma would typically involve a score of 3 or lower on the Glasgow Coma Scale (GCS).

(B) Conscious – Consciousness involves being aware and

**responding appropriately,
which this patient does not
exhibit.**

**(C) Lethargy – Lethargy refers
to a state of drowsiness or
reduced awareness, but the
GCS score suggests more
severe impairment.**

**(D) Stupor – Stupor involves a
state of near-unconsciousness,
where the patient only**

**responds to painful stimuli,
which fits the GCS findings.**

Correct Answer: (D) Stupor

**Explanation: A patient with
eye opening to pain,
incoherent response, and
flexion to touch indicates a
stuporous state, as per the
Glasgow Coma Scale.**

40. Priority nursing diagnosis for an 18-year-old with headache, stiff neck, fever, petechial rash, and a GCS score of 12.

(A) Disturbed thought processes – While this might be relevant to the patient's cognitive state, the priority is addressing the acute issue.

**(B) Impaired physical mobility
– Mobility is not the primary
concern here.**

**(C) Hypothermia – The patient
presents with fever, not
hypothermia.**

**(D) Pain – The most important
diagnosis to address in a
patient with fever, headache,
stiff neck, and a petechial rash**

would be the potential for meningitis, which could cause pain as well.

Correct Answer: (D) Pain

Explanation: The symptoms suggest a possible infection such as meningitis, where pain management becomes a priority.

57. A trauma patient with vital signs indicating cold temperature, lethargy, and an open wound with active bleeding. What reason best explains the cold body temperature?

(A) Decreased peripheral blood circulation – Cold body temperature is often a result

of blood being redirected to vital organs in response to shock, reducing blood flow to the extremities.

(B) Activation of anterior hypothalamus – This would typically cause fever, not cold temperature.

(C) Increased peripheral vasodilatation – This would lead to warmth, not coldness.

(D) Exposing patient unnecessarily – Exposure might contribute to cooling, but it is less likely than shock and circulatory issues.

Correct Answer: (A) Decreased peripheral blood circulation

Explanation: In shock, blood flow is prioritized to vital organs, which leads to cold

extremities and a low body temperature.

58. Interpretation of vital signs for a menopausal woman with hyperthyroidism presenting with agitation and diaphoresis.

(A) Euthermia; tachycardia; tachypnea; hypotension –

Euthermia is not indicated, as the patient is febrile.

(B) Hypothermia; tachycardia; eupnea; hypertension – The patient's condition is not consistent with hypothermia.

(C) Hyperthermia; tachycardia; eupnea; normo-tension – Hyperthyroidism can cause elevated body temperature,

heart rate, and normal blood pressure.

(D) Hyperthermia; tachycardia; tachypnea; normo-tension – This best reflects the symptoms of hyperthyroidism, as the patient exhibits increased temperature, heart rate, and breathing.

Correct Answer: (D)

**Hyperthermia; tachycardia;
tachypnea; normo-tension**

**Explanation: Hyperthyroidism
often presents with
hyperthermia, tachycardia,
and tachypnea, with normal
blood pressure.**

59. Factors influencing blood pressure in a patient with polycythemia and increased hematocrit.

(A) Medication – Medications can influence blood pressure, but they are not the most significant factor here.

(B) Blood viscosity – Increased blood viscosity, as seen in

polycythemia, can contribute to increased blood pressure.

(C) Peripheral resistance – Increased blood viscosity typically leads to higher peripheral resistance, which raises blood pressure.

(D) Circulating blood volume – Increased blood volume could raise blood pressure, but in

polycythemia, viscosity plays a bigger role.

Correct Answer: (B) Blood viscosity

Explanation: The increased viscosity of the blood in polycythemia leads to higher peripheral resistance, which can raise blood pressure.

60. Priority nursing measure for a swollen, cold IV site with pain.

(A) Irrigate the line with distilled water – Irrigating might worsen the situation and should only be done if the line is not infiltrated.

**(B) Maintain IV on extremity –
If there's swelling and pain,
the IV should be removed, not
maintained.**

**(C) Maintain IV on extremity –
Repetition; the IV should be
assessed and potentially
removed if it is causing
complications.**

****Correct Answer: ** Would clarify a specific nursing measure**

Here are the answers and explanations for the questions:

91. A 26-year-old man with multiple sexual partners complains of burning sensation

during urination, acute onset of profuse and purulent urethral discharge, and swollen testicles. What is the possible diagnosis?

(A) Candidiasis – Candidiasis typically causes itching and thick, white discharge but is not associated with purulent urethral discharge or testicular swelling.

(B) Gonorrhea – Gonorrhea can cause purulent urethral discharge and testicular tenderness, making it a likely diagnosis.

(C) Herpes genitalia – Herpes causes painful ulcers but not typically purulent discharge.

(D) Trichomoniasis – Trichomoniasis typically presents with itching and a

frothy discharge, not purulent discharge or testicular swelling.

Correct Answer: (B) Gonorrhea

Explanation: Gonorrhea is a common sexually transmitted infection that can cause purulent urethral discharge and testicular swelling (epididymitis).

92. A woman came to the hospital with white, thick, and curd-like discharge from the vagina. She also has burning sensation during urination. What is the most likely diagnosis?

(A) Candidiasis – This is the most likely diagnosis, as thick,

white, curd-like discharge and a burning sensation during urination are classic symptoms of vaginal candidiasis.

(B) Gonorrhea – Gonorrhea typically causes a different type of discharge, often purulent and greenish, not curd-like.

(C) Trichomoniasis – This causes a frothy, yellow-green

discharge, not thick and curd-like.

(D) Urethritis – Urethritis affects the urethra and is not associated with thick vaginal discharge.

**Correct Answer: (A)
Candidiasis**

Explanation: The white, thick, curd-like discharge and

burning during urination are characteristic symptoms of vaginal candidiasis.

93. What is the vital strategy to prevent the spread of anthrax after multiple cows died?

(A) Mass vaccination of animals in the area – This is an important measure to prevent future outbreaks of anthrax in livestock.

(B) Bury animals believed to have died of anthrax – Burying the animals is crucial to prevent environmental contamination and the spread of anthrax spores.

(C) Use occupational health attire before any contact – While protective attire is important for workers, it does not prevent the initial spread from infected animals.

(D) Isolate and control exposed zone and individuals – Isolating the zone and controlling exposure is key, but this alone is not as effective as preventing new infections

through vaccination and proper disposal.

Correct Answer: (B) Bury animals believed to have died of anthrax

Explanation: Proper disposal of dead animals is a crucial step in preventing the spread of anthrax.

94. A woman bitten by a rabid fox develops headache, fever, nausea, and tingling at the bite site after three weeks. What clinical phase of rabies is most likely present?

(A) Excitatory phase – This phase involves extreme agitation, hallucinations, and

hydrophobia, not the early symptoms described.

(B) Neurologic phase – The neurologic phase involves paralysis and other severe neurologic symptoms but usually comes later in the disease.

(C) Paralytic phase – This phase includes paralysis and is

usually seen after the neurologic phase.

(D) Prodromal phase – The prodromal phase is characterized by nonspecific symptoms like fever, headache, nausea, and tingling at the bite site, which fits the patient's presentation.

Correct Answer: (D) Prodromal phase

Explanation: The symptoms described are typical of the prodromal phase of rabies, which occurs before more severe symptoms like paralysis and agitation develop.

66. A newly diagnosed type II diabetes patient has a nursing diagnosis of 'knowledge deficit related to medication administration.' Which outcome criteria need improvement?

(A) Verbalize understanding of desired and adverse effects of medications – This is important for the patient to understand the medications, so it's a reasonable outcome.

(B) Be able to monitor blood glucose levels to determine medication – Monitoring blood glucose is critical in managing diabetes, so this is an appropriate outcome.

(C) State signs, symptoms, and treatment of hypoglycemia – This is a key outcome in diabetes management to ensure safety.

(D) Request insulin 30 minutes after taking meal on a daily basis – Insulin timing should be more individualized, and it is not a correct or safe practice for everyone.

Correct Answer: (D) Request insulin 30 minutes after taking meal on a daily basis

Explanation: The timing of insulin administration should be individualized based on the type of insulin and meal timing. This instruction may not be universally applicable.

67. The wound drainage from a patient with peritonitis is pale yellow, watery, and fluid from

a blister. Which wound drainage is described?

(A) Serous – Serous drainage is pale yellow and watery, which matches the description of the wound drainage.

(B) Purulent – Purulent drainage is thick, green or yellow, and typically indicative of infection.

(C) Sanguineous – Sanguineous drainage is blood-tinged, not watery.

(D) Serosanguineous – Serosanguineous is a mix of blood and serum, typically pink or light red.

Correct Answer: (A) Serous

Explanation: Serous drainage is pale yellow and watery, fitting

the description of the wound drainage.

68. A patient with an above-knee cast shows minimal swelling, discomfort, and color change, but with prompt capillary refill response and decreased sensation. Which

sign best indicates a poor prognosis?

(A) Minimal swelling – This is generally a good sign, not an indicator of poor prognosis.

(B) Minimal sensations – Decreased sensation could indicate nerve damage or poor circulation, which is a concern.

**(C) Minimal discomfort –
Minimal discomfort is
generally a positive sign.**

**(D) Minimal color change –
This is a good sign and does
not indicate a poor prognosis.**

**Correct Answer: (B) Minimal
sensations**

**Explanation: Decreased
sensation could be an early**

indicator of poor circulation or nerve injury, which is a poor prognostic sign.

69. What psychological component should be assessed first in a 29-year-old male scheduled for emergency surgery for appendicitis?

(A) Level of understanding for pre & postoperative instructions – This is important but may be influenced by the patient's emotional state.

(B) Coping strategies for common health problems – Coping strategies are useful but not the first priority.

(C) Emotional state related to current diagnosis – Assessing

emotional state related to the diagnosis (anxiety, fear) is important for preparing the patient.

(D) Roles and responsibilities at home – This is relevant, but addressing the emotional state first is crucial in an emergency.

Correct Answer: (C) Emotional state related to current diagnosis

Explanation: Understanding the patient's emotional response to the diagnosis is critical in addressing anxiety and preparing them for surgery.

70. Who is responsible for solving the issue of a 70-year-old male patient who doesn't fully understand the care outcome before signing the informed consent?

(A) The nurse – The nurse ensures that the patient understands the procedure and can explain it, but may need to involve the surgeon for clarification.

(B) The surgeon – The surgeon is responsible for explaining the procedure, risks, and expected outcomes, so they are responsible for resolving this issue.

(C) The assistant surgeon – The assistant surgeon may not be the primary person to address this issue.

(D) The anesthetist – The anesthetist is responsible for explaining anesthesia-related risks, not the entire surgical procedure.

Correct Answer: (B) The surgeon

Explanation: The surgeon is responsible for explaining the procedure and ensuring the

**patient understands before
signing the informed consent.**

**Here are the answers and
explanations for the questions:**

**61. What is the most likely
purpose of surgical forceps for
cleaning a septic wound?**

(A) To remove inner cover of wound – Surgical forceps may be used to handle wound coverings, but this is not the primary use in cleaning a wound.

(B) To clean the wound before dressing – Surgical forceps are often used to handle and remove foreign material from a wound, making this a possible answer.

(C) To dry cleaned wound before dressing – While drying a wound is important, forceps are not typically used for drying purposes.

(D) To apply new dressing for final dressing – Forceps may be used to handle dressings, but this option is not specific to cleaning a septic wound.

Correct Answer: (B) To clean the wound before dressing

Explanation: Surgical forceps are commonly used to handle dressings or foreign bodies when cleaning a wound to ensure the area is cleared of debris before applying a new dressing.

62. What is the immediate risk factor for pressure ulcer in the 75-year-old woman who is urine incontinent, dehydrated, and neglected with a bed sore over her buttock?

(A) Age – Age can contribute to pressure ulcer formation but is not the immediate risk factor in this case.

(B) Dehydration – Dehydration can make skin more fragile and more prone to breakdown, making it a significant risk factor.

(C) Urine Incontinence – Urine incontinence is a significant risk factor for pressure ulcers because moisture from urine can cause skin breakdown.

(D) Serous drainage from wound – Serous drainage is a sign of a wound that is healing or not infected but is not a direct risk factor for further ulceration.

Correct Answer: (C) Urine Incontinence

Explanation: Urine incontinence creates moisture on the skin, increasing the risk

**of pressure ulcer formation,
especially in a neglected and
dehydrated patient.**

**63. What is the mechanism of
action of warm tap water
enema pre-operatively for
bowel surgery?**

(A) Isotonic with simple expansion of colon – Isotonic enemas are not typically used for bowel evacuation before surgery.

(B) Irritation to the colon to cause sense of defecation – This is the mechanism of action for irritant enemas, not for warm tap water enemas.

(C) Hypotonic with simple expansion of colon –

Hypotonic solutions like warm tap water cause the colon to expand and facilitate bowel evacuation.

(D) Hypertonic with absorption of fluid from colon –

Hypertonic solutions are used to draw fluid into the colon but are not typically used for warm tap water enemas.

**Correct Answer: (C) Hypotonic
with simple expansion of colon**

**Explanation: Warm tap water
is hypotonic and causes the
colon to expand, facilitating
the evacuation of stool before
surgery.**

64. What is the most likely type of colostomy in a patient with fluid discharge from the colostomy opening?

(A) Sigmoid colostomy – This type of colostomy typically produces more formed stool, not fluid.

(B) Ascending colostomy – Ascending colostomy produces

more liquid stool due to the lack of absorption in the colon.

**(C) Transverse colostomy –
Transverse colostomy may produce semi-formed to liquid stool depending on the location.**

**(D) Descending colostomy –
This type of colostomy usually produces more formed stool.**

Correct Answer: (B) Ascending colostomy

Explanation: An ascending colostomy is located in the right side of the abdomen and produces liquid stool, which is consistent with the patient's symptoms.

65. What is the most priority nursing diagnosis for a 75-year-old male patient with type II diabetes, decreased vision, decreased peripheral sensation, and dribbling urine?

(A) Risk for pressure ulcer related to decreased peripheral sensation – Decreased sensation could increase the risk of pressure ulcers, but the main concern here is vision loss.

(B) Risk for constipation related to active physical exercise – Exercise usually helps prevent constipation, so this is not the most relevant diagnosis.

(C) Risk for infection related to dribbling of urine – Dribbling urine could increase the risk of infection, but vision and sensation loss are greater concerns.

(D) Risk for injury related to decreased vision – Decreased vision presents a clear risk for falls or other injuries, making it a priority diagnosis.

Correct Answer: (D) Risk for injury related to decreased vision

Explanation: Decreased vision puts the patient at high risk for

falls and injuries, making it the priority nursing diagnosis.

80. What is the most appropriate initial action the nurse should take to prevent a medication error?

(A) Checking the right medication label – This is

**important to ensure the
correct medication is given.**

**(B) Checking the right dose –
This is crucial but comes after
confirming the medication.**

**(C) Checking the right client –
Confirming the patient's
identity is essential before
administering medication.**

**(D) Checking the right route –
The correct route is important
but is the final step after
confirming the medication,
client, and dose.**

**Correct Answer: (C) Checking
the right client**

**Explanation: The initial action
should always be confirming
the right patient to ensure**

medication is given to the correct individual.

81. What is the possible diagnosis for a male prisoner patient with fever, malaise, anorexia, abdominal discomfort, visible jaundice, and abnormally increased IgM?

(A) Hepatitis A – Hepatitis A typically presents with acute symptoms like jaundice and elevated IgM antibodies, and is often transmitted via contaminated food or water, making this the most likely diagnosis.

(B) Hepatitis B – Hepatitis B is associated with more chronic infection and would typically

show IgG antibodies rather than IgM in an acute phase.

(C) Hepatitis C – Hepatitis C typically presents with chronic symptoms and is less likely to present with acute jaundice and elevated IgM.

(D) Hepatitis D – Hepatitis D only occurs in patients infected with Hepatitis B, so it is unlikely in this case.

Correct Answer: (A) Hepatitis A

Explanation: Hepatitis A is the most likely diagnosis based on the patient's symptoms and elevated IgM antibodies, indicating an acute infection.

82. What is the priority control measure the nurse should take to prevent cholera transmission while a man washes clothes from a cholera patient in a stream without protective equipment?

(A) Disinfect the contaminated clothes – While disinfecting clothes is important, the immediate concern is preventing contamination of the water source.

(B) Avoid contamination of drinking water – Preventing contamination of drinking water is critical to stop the spread of cholera.

(C) Boil drinking water from the stream source – Boiling drinking water is important but is a reactive measure rather than preventive.

(D) Isolate the victim and wash hands frequently – While hand washing and isolation are important, controlling water contamination is the top priority.

Correct Answer: (B) Avoid contamination of drinking water

Explanation: Avoiding contamination of the drinking

water is the most effective and immediate control measure to prevent the spread of cholera.

83. What is the best treatment option for a four-year-old boy with long roundworms coming out of his anus with stool?

**(A) Albendazole 400 mg PO
stat – This dosage is
appropriate for older children
and adults, not young children.**

**(B) Albendazole 800 mg PO
stat – This dosage is too high
for a four-year-old child.**

**(C) Mebendazole oral
suspension 500 mg/5ml – This
dose is too high for a four-
year-old.**

(D) Mebendazole oral suspension 250 mg/2.5 ml – This is the correct dosage for a four-year-old child to treat roundworm infection.

**Correct Answer: (D)
Mebendazole oral suspension
250 mg/2.5 ml**

Explanation: Mebendazole is commonly used to treat

roundworm infections, and the appropriate dose for a four-year-old child is 250 mg.

84. What should be the first action of the nurse to prevent TB transmission in a HIV-positive woman living with her husband who has smear-positive pulmonary TB?

(A) Give her isoniazid preventive therapy – Preventive therapy should be considered, but the first action is to assess her clinical status.

(B) Conduct appropriate clinical evaluation – The first action is to evaluate the woman for any symptoms of TB and her risk of transmission.

(C) Start first line anti-TB treatment regimen – The woman does not have symptoms yet, so anti-TB treatment is not indicated at this stage.

(D) Go home and screen other family members – Screening other family members is important but comes after assessing the patient.

Correct Answer: (B) Conduct appropriate clinical evaluation

Explanation: The first step is to assess the woman's health status to determine if she has been infected with TB, before proceeding with preventive therapy or treatment.

Here are the answers and explanations for the given questions:

71. What is the primary responsibility of the nurse when tachycardia is recognized as an early indicator of hemorrhage and shock?

(A) Administer anti-pain and continue as usual – Administering pain medication without addressing the potential cause of tachycardia (hemorrhage) is not appropriate.

(B) Re-assess the patient and identify the cause, Check vital sign, and report to the physician – The priority action is to reassess the patient and identify the cause of

tachycardia, as this could indicate shock or hemorrhage.

(C) Document findings and wait for 30 minutes – Delaying action could result in worsening of the patient's condition.

(D) Document findings and wait for 30 minutes – Waiting is inappropriate in this urgent situation.

Correct Answer: (B) Re-assess the patient and identify the cause, Check vital sign, and report to the physician

Explanation: Tachycardia is an early sign of shock and hemorrhage; immediate reassessment and intervention are necessary.

72. What is the initial activity to be done by the nurse when receiving surgical equipment for preparation for the next case?

**(A) Decontamination –
Decontamination is the first step in cleaning surgical instruments to remove any**

biological material before further cleaning.

(B) Cleaning – Cleaning comes after decontamination to remove residual debris.

(C) High level disinfection – High-level disinfection is performed after cleaning and decontamination.

(D) Sterilization – Sterilization is the final step after decontamination and cleaning.

**Correct Answer: (A)
Decontamination**

Explanation: The first step in preparing surgical instruments for the next case is decontamination to remove any contamination.

73. Which lab result best predicts the presence of induced pleural effusion in this case (sharp chest pain, purulent drainage, glucose equal to serum levels, increased WBC, increased RBC)?

(A) Drainage appearance – The appearance of the pleural fluid (purulent) suggests infection, but it does not specifically predict pleural effusion.

(B) Glucose level – A normal glucose level in pleural fluid can suggest that the effusion is not caused by TB or another infection like empyema.

(C) RBC level – Elevated RBC levels in pleural fluid could indicate trauma or hemorrhage, but it's not the most indicative of pleural effusion.

(D) WBC level – Increased WBC levels suggest infection or inflammation, which can indicate pleural effusion from infections such as tuberculosis.

Correct Answer: (D) WBC level

Explanation: Increased WBC levels in the pleural fluid suggest an inflammatory or infectious cause, which could be consistent with pleural effusion, especially from TB.

74. What additional data should be assessed as a priority in the 19-year-old female after a motorcycle crash with multiple injuries?

(A) Airway and breathing – Airway and breathing are critical to assess first to ensure the patient is receiving adequate oxygenation and ventilation.

(B) Consciousness level – This is important but secondary to assessing airway and breathing.

(C) Body fluid status – Body fluid status is important but is not the first priority.

(D) Vital sign – Vital signs are important, but airway and breathing are the first priority.

Correct Answer: (A) Airway and breathing

Explanation: Ensuring the airway is clear and the patient is breathing adequately is the priority in trauma situations.

75. What appropriate nursing intervention should be done

first for a 39-year-old male patient with a chest injury (fractured ribs, hemothorax)?

(A) Stop hemorrhage – Stopping hemorrhage is crucial, but the priority in this case is stabilizing the patient's respiratory status.

(B) Ventilation support – The patient's tachypnea and absent breath sounds suggest

the need for respiratory support.

(C) Initiate IV resuscitation – IV fluids may be necessary for shock, but respiratory support should come first.

(D) Provide morphine sulfate – While pain management is important, it is not the priority over airway management.

Correct Answer: (B) Ventilation support

Explanation: Given the patient's respiratory distress and absent breath sounds, ventilation support is the immediate priority.

25. What is the most likely diagnosis for a patient with a soft nasal bump that spread to her lip and vesicles covered with honey-colored crusts?

(A) Eczema – Eczema typically does not present with vesicles or honey-colored crusts.

(B) Impetigo – Impetigo presents with vesicles and honey-colored crusts, often

**around the nose and mouth,
making it the likely diagnosis.**

(C) Carbuncle – A carbuncle is a deeper skin infection, usually more localized and not associated with vesicles.

(D) Pemphigus – Pemphigus causes blistering but is not typically characterized by honey-colored crusts.

Correct Answer: (B) Impetigo

Explanation: Impetigo is a highly contagious bacterial infection characterized by vesicles and honey-colored crusts, commonly around the nose and mouth.

26. What effect is most likely expected after 24-hour resuscitation of a burn patient with 50% total body surface area burn injury?

(A) Increment in PaO₂ – After resuscitation, the PaO₂ may improve if oxygenation improves, but this is not a specific outcome.

**(B) Reduction in BUN level –
After fluid resuscitation, BUN
levels may decrease as kidney
perfusion improves.**

**(C) Increment in urine output –
After 24 hours of resuscitation,
urine output should increase
as fluid balance is restored.**

**(D) Reduction in serum
creatinine level – Serum
creatinine may improve as**

kidney function improves with adequate fluid resuscitation.

Correct Answer: (C) Increment in urine output

Explanation: An increase in urine output is expected after proper resuscitation, indicating improved renal perfusion.

27. Where will the patient feel the pain in acute severe periumbilical pain with a positive Rovsing's sign?

(A) Left lower quadrant – Rovsing's sign, where palpation of the left lower quadrant causes pain in the right lower quadrant, suggests appendicitis.

(B) Left upper quadrant – This is not typical for Rovsing's sign.

(C) Right lower quadrant – Pain is often localized in the right lower quadrant in appendicitis, but Rovsing's sign involves referred pain.

(D) Right upper quadrant – This is not associated with appendicitis.

Correct Answer: (A) Left lower quadrant

Explanation: Rovsing's sign is positive when palpation of the left lower quadrant causes pain in the right lower quadrant, often indicating appendicitis.

28. What is the most appropriate dietary advice to enhance functional health status for a patient with fatigue, nausea, vomiting, constipation, weight loss, abdominal distension, and low serum albumin?

(A) High fat diet – A high-fat diet is not appropriate for a patient with gastrointestinal symptoms.

(B) High protein diet – A high-protein diet is beneficial for patients with low serum albumin and muscle wasting.

(C) High-calorie diet – A high-calorie diet may be necessary, but protein intake should be prioritized in this case.

(D) Nutritional supplements – Nutritional supplements could

be helpful in supporting nutritional intake.

Correct Answer: (B) High protein diet

Explanation: A high-protein diet is essential for patients with low serum albumin and signs of malnutrition or muscle wasting.

29. What is the priority nursing diagnosis for a patient with peptic ulcer disease who has been treated with triple antibiotics?

(A) Pain – Pain is a common symptom of peptic ulcer disease but is not the priority diagnosis.

(B) Anxiety – Anxiety may be a concern but is secondary to the need for education and understanding.

(C) Knowledge Deficit – This is a priority diagnosis, as the patient may need education regarding medication adherence and lifestyle changes.

**(D) Imbalanced nutrition –
Nutrition may be affected, but
knowledge deficit is more
critical for effective treatment.**

**Correct Answer: (C) Knowledge
Deficit**

**Explanation: The patient may
need more information about
managing peptic ulcer disease,
including the importance of
completing the antibiotic**

regimen and dietary adjustments.

Here are the answers and explanations for the questions:

46. What is the most suggestive feature of acute nephritic syndrome in a 30-year-old patient with a history

of "group A" beta hemolytic streptococcal throat infection?

(A) Azotemia – Azotemia (elevated nitrogenous waste products like BUN and creatinine) is a hallmark of kidney dysfunction and can be seen in acute nephritic syndrome.

(B) Confusion – Confusion is not typically a direct feature of

acute nephritic syndrome but may result from severe electrolyte imbalances or uremia.

(C) Flank pain – Flank pain could be associated with kidney issues but is not specific to acute nephritic syndrome.

(D) Urgency – Urgency is more typical of urinary tract

infections, not acute nephritic syndrome.

Correct Answer: (A) Azotemia

Explanation: Azotemia is a key feature of acute nephritic syndrome, indicating impaired renal function often after an infection such as a streptococcal throat infection.

47. What is the best outcome the nurse expects after an intervention for "Excess Fluid Volume related to sodium and water retention"?

**(A) Fluid volume will be stable
– This is a reasonable and realistic outcome that aligns with the goal of managing fluid retention.**

(B) Patient will be able to perform activities – While this is a positive outcome, it is secondary to stabilizing the fluid balance.

(C) Patient will not experience bleeding – This outcome is not directly related to the fluid retention issue.

(D) Patient will not develop infection – Infection prevention is important but not directly related to managing fluid volume excess.

Correct Answer: (A) Fluid volume will be stable

Explanation: The immediate goal is to stabilize the fluid balance, which is the most

direct response to fluid volume excess.

48. What is the most likely diagnosis for a 40-year-old patient with heat intolerance, rapid heart rate, and bulging eyes?

(A) Hypothyroidism –
Hypothyroidism typically presents with weight gain, cold intolerance, and lethargy, not heat intolerance and rapid heart rate.

(B) Hyperthyroidism –
Hyperthyroidism is characterized by heat intolerance, tachycardia, and bulging eyes (exophthalmos), which are classic symptoms.

(C) Hyperparathyroidism –
Hyperparathyroidism generally leads to hypercalcemia, which may cause fatigue, bone pain, and kidney stones, but not the symptoms described here.

(D) Hypoparathyroidism –
Hypoparathyroidism would typically cause symptoms related to hypocalcemia like muscle cramps, not the symptoms described here.

Correct Answer: (B)

Hyperthyroidism

Explanation: Heat intolerance, rapid heart rate, and bulging eyes are characteristic symptoms of hyperthyroidism, often seen in conditions like Graves' disease.

49. What is the next appropriate nursing action after teaching a patient to stabilize the skin during self-injection of insulin?

(A) Train on how to push the plunger all the way in – This is the next step after the skin is stabilized, which involves injecting the insulin.

(B) Train on disposing of used syringe – Disposal of the syringe is important but not immediately after the first step.

(C) Train on how to pick up syringe – This is an earlier step in the process before stabilizing the skin.

(D) Train on removing the needle – The needle should be

removed after the injection is given, not during the process of stabilizing the skin.

Correct Answer: (A) Train on how to push the plunger all the way in

Explanation: After stabilizing the skin, the next step is to inject the insulin by pushing the plunger all the way in.

50. What is the most likely diagnosis for a 40-year-old diabetic patient with vomiting, increased fluid intake, headache, fruity odor, and increased breathing?

**(A) Hypoglycemia –
Hypoglycemia presents with symptoms like shakiness,**

sweating, and confusion, not the fruity odor or increased breathing.

(B) Diabetic ketoacidosis – Diabetic ketoacidosis (DKA) is characterized by vomiting, fruity odor (due to ketones), and rapid breathing (Kussmaul respiration), making it the most likely diagnosis.

**(C) Diabetic neuropathies –
Diabetic neuropathies do not
cause these acute symptoms.**

**(D) Hyperglycemic
hyperosmolar nonketotic
syndrome – While this can
present with hyperglycemia, it
does not typically cause the
fruity odor seen in DKA.**

Correct Answer: (B) Diabetic ketoacidosis

Explanation: DKA presents with the symptoms described: vomiting, fruity odor, rapid breathing, and significantly elevated blood glucose.

30. What is the most likely final action after inserting a

**nasogastric tube for a patient
in the intensive care unit?**

**(A) Confirm tube size – This is
important during the insertion
process, not the final action.**

**(B) Ensure tube patency –
Checking for tube patency
ensures that it is working and
unobstructed.**

**(C) Checking bowel sound –
Bowel sounds are important
but are not directly related to
the immediate post-insertion
care of the tube.**

**(D) Secure the tube in place –
Securing the tube is the final
step to prevent it from being
dislodged.**

Correct Answer: (D) Secure the tube in place

Explanation: The final step after inserting the nasogastric tube is to secure it to ensure it stays in place.

31. What is the priority nursing intervention for a 50-year-old

male patient admitted after surgery for bowel obstruction?

**(A) Check the surgical dressing
– Checking the surgical dressing is important but is secondary to monitoring vital signs and ensuring patient stability.**

**(B) Monitor vital signs –
Monitoring vital signs is the first priority to assess for any**

signs of complications like shock or infection.

(C) Monitor input and output – Monitoring fluid balance is important but secondary to vital signs.

(D) Start colostomy irrigation – This should be done as needed but is not an immediate priority.

Correct Answer: (B) Monitor vital signs

Explanation: Monitoring vital signs is the priority after surgery to ensure the patient is stable and there are no signs of complications.

32. What is the most likely outcome criteria the nurse should set for a patient with chronic liver disease and fatigue?

(A) Decide her nutritional requirements – This is important but not directly related to the fatigue outcome.

(B) Report strategies to preserve power – This aligns with managing fatigue and improving energy.

(C) Perform activities of daily living – This is a good goal, as it directly addresses the patient's ability to manage daily tasks.

(D) Report increased level of comfort – Comfort is important but may not be the

**most specific outcome for
fatigue management.**

**Correct Answer: (B) Report
strategies to preserve power**

**Explanation: Teaching the
patient strategies to conserve
energy and manage fatigue is
the most realistic and
actionable goal.**

33. What is the expected outcome after placing a pillow between the legs of a patient with a hip fracture?

(A) Actively participates in exercise regimen – This is not directly related to the pillow placement.

(B) Uses ambulatory aids correctly and safely – The pillow placement is more related to positioning, not mobility aids.

(C) Participates in progressive ambulation program – The pillow helps with positioning, not ambulation.

(D) Cooperates in position changes and increased

independence – The pillow placement helps with positioning and comfort, facilitating easier position changes.

Correct Answer: (D)

Cooperates in position changes and increased independence

Explanation: Placing a pillow between the legs can help with positioning and may assist the

patient in cooperating with repositioning.

34. What is the most likely diagnosis for a patient with severe pain, tenderness, edema, and abnormal joint movement?

(A) Fourth degree sprain – A fourth-degree sprain is not a standard classification for sprains.

(B) Third degree sprain – A third-degree sprain involves complete ligament rupture, which fits the description of severe pain and abnormal joint movement.

(C) Second degree sprain – A second-degree sprain involves partial ligament tearing but not complete rupture.

(D) First degree sprain – A first-degree sprain involves mild overstretching, not the severe symptoms described.

Correct Answer: (B) Third degree sprain

Explanation: A third-degree sprain involves a complete rupture of the ligament, leading to severe pain, tenderness, edema, and joint instability.

35. What is the initial nursing intervention for a patient with

a soft tissue injury of the right upper extremity?

(A) Promote rest – Rest is important but should follow initial management.

(B) Apply cold packs – Cold packs are the first intervention to reduce swelling and pain in soft tissue injuries.

**(C) Encourage elevation –
Elevation should follow after
cold therapy to reduce
swelling.**

**(D) Apply elastic bandage –
This may be used for
compression but is secondary
to cold therapy initially.**

**Correct Answer: (B) Apply cold
packs**

Explanation: The first step in managing a soft tissue injury is applying cold packs to reduce swelling and pain.

Here are the answers and explanations for the questions:

20. What is the most probable impending complication for a

42-year-old male patient with a blunt abdominal injury, back pain, and epigastric bruit secondary to renal artery occlusion?

(A) Acute renal failure – Renal artery occlusion can lead to acute renal failure due to decreased blood flow to the kidneys.

**(B) Chronic renal failure –
Chronic renal failure develops
over a longer period, not
acutely due to an injury.**

**(C) Primary hypertension –
Hypertension is not typically
caused by acute renal artery
occlusion.**

**(D) Secondary hypertension –
While renal artery occlusion
can cause secondary**

hypertension, acute renal failure is a more immediate concern in this case.

Correct Answer: (A) Acute renal failure

Explanation: Renal artery occlusion can lead to a sudden decrease in renal function, resulting in acute renal failure.

21. What is the most likely medical diagnosis for a 25-year-old female with declining blood pressure, pulse, and hot flushed skin after spinal injury and IV fluid administration?

(A) Septic shock – Septic shock typically involves infection and systemic inflammatory

response, but the symptoms don't align with this scenario.

**(B) Neurogenic shock –
Neurogenic shock can occur after spinal injury and is characterized by hypotension, bradycardia, and warm, flushed skin.**

**(C) Anaphylactic shock –
Anaphylactic shock usually presents with signs of allergic**

reaction, including swelling and difficulty breathing, which are not mentioned here.

(D) Hypovolemic shock – Hypovolemic shock usually results from significant blood loss or fluid loss, but the administration of IV fluids does not fit the profile of this shock type.

Correct Answer: (B)

Neurogenic shock

Explanation: Neurogenic shock results from damage to the spinal cord and is marked by hypotension, bradycardia, and warm, flushed skin due to vasodilation.

22. What content should be primarily included in the teaching plan for a patient with myocardial infarction about activities and exercises during the first week post-discharge?

(A) Perform aerobic exercises – Aerobic exercise is important long-term but may be too strenuous during the first week.

(B) Walk daily, increasing distance and time – Walking is a safe and gradual exercise to begin after a myocardial infarction.

(C) Avoid any activity that produces chest pain – Avoiding activities that produce chest pain is crucial, but walking can generally be done at a pace that avoids triggering pain.

(D) Pursue activities that relieve and reduce stress – Stress reduction is important, but physical activity should be emphasized initially.

Correct Answer: (B) Walk daily, increasing distance and time

Explanation: Gradual walking increases cardiovascular health while being gentle enough to

begin during recovery from a myocardial infarction.

23. Which type of anemia could be managed by including beef, chicken, and egg yolk in the diet for a 23-year-old female with light-headedness, menstrual bleeding, and low hemoglobin?

(A) Iron deficiency – Iron-rich foods such as beef, chicken, and egg yolk can help manage iron deficiency anemia.

(B) Folate deficiency – Folate deficiency is typically treated with foods like leafy greens, not animal products.

**(C) Cobalamin deficiency –
Cobalamin (Vitamin B12)**

deficiency is usually treated with B12-rich foods, but this patient's symptoms don't suggest this type of anemia.

(D) Vitamin B12 deficiency – Vitamin B12 deficiency would require supplementation with B12-rich foods, not primarily iron.

Correct Answer: (A) Iron deficiency

Explanation: Beef, chicken, and egg yolk are rich in heme iron, which is beneficial for treating iron deficiency anemia.

24. Which stage of pressure ulcer does the nurse diagnose for a bedridden patient with

**open but shallow ulcerations
around the occipital and
scapular areas?**

**(A) Stage I – Stage I involves
non-blanchable erythema, not
open ulcerations.**

**(B) Stage II – Stage II includes
partial-thickness skin loss,
often with an open ulceration.**

(C) Stage III – Stage III involves full-thickness skin loss, possibly involving underlying tissue, which is more severe than what is described.

(D) Stage IV – Stage IV involves full-thickness tissue loss, including muscle or bone involvement.

Correct Answer: (B) Stage II

Explanation: Stage II pressure ulcers involve open, shallow ulcerations without full-thickness skin loss, which aligns with the patient's description.

72. What is the priority nursing diagnosis for a 78-year-old patient admitted with chest

**pain, pale and diaphoretic skin,
and hypotension?**

**(A) Acute pain – Acute pain is
important but not the priority
in this emergency situation.**

**(B) Activity intolerance –
Activity intolerance is a
concern long-term, but not the
immediate priority.**

(C) Fluid volume deficit – The patient's hypotension is likely due to inadequate tissue perfusion rather than fluid volume deficit.

(D) Ineffective tissue perfusion – The most immediate concern is ineffective tissue perfusion due to hypotension, which could be a sign of cardiac ischemia or shock.

Correct Answer: (D) Ineffective tissue perfusion

Explanation: The symptoms of chest pain, hypotension, and altered skin perfusion point to ineffective tissue perfusion as the priority nursing diagnosis.

73. What is the definitive laboratory test for diagnosing an infection caused by *Salmonella typhi*?

(A) Widal test – The Widal test is commonly used but has limitations in terms of sensitivity and specificity for typhoid fever.

(B) Blood culture – Blood culture is the most definitive

**method for diagnosing
Salmonella typhi infection.**

**(C) Urine analysis – Urine
analysis is not typically used to
diagnose Salmonella typhi.**

**(D) Stool examination – Stool
examination can help identify
Salmonella but is less
definitive than blood culture.**

Correct Answer: (B) Blood culture

Explanation: Blood culture is the gold standard for diagnosing Salmonella typhi infection, especially in the early stages.

74. What is the most effective method to control an epidemic

of bacillary dysentery in the community?

(A) Case detection and treatment – Detecting and treating cases is important, but prevention is crucial for controlling an epidemic.

(B) Teaching about proper excreta disposal – Proper sanitation is a key preventive

measure to control the spread of bacillary dysentery.

(C) Teaching about cleaning and cooking of foods – Proper food handling can help reduce transmission but is secondary to sanitation.

(D) Teaching about effective handwashing practice – Handwashing is an essential part of preventing

transmission, but proper excreta disposal is more directly effective in controlling the epidemic.

Correct Answer: (B) Teaching about proper excreta disposal

Explanation: Proper excreta disposal is one of the most effective methods to control the spread of bacillary dysentery.

75. What is the most appropriate investigation for confirming a suspected diagnosis of leprosy in a 63-year-old female with a small, painless lump and decreased muscle strength?

(A) ESR – ESR is a nonspecific test and not directly useful for diagnosing leprosy.

(B) Sputum AFB – Sputum AFB testing is used to diagnose tuberculosis, not leprosy.

(C) Gene X-pert – Gene X-pert is used for detecting tuberculosis, not leprosy.

(D) Slit skin smear – The slit skin smear is the most specific test for diagnosing leprosy by detecting acid-fast bacilli in the skin.

Correct Answer: (D) Slit skin smear

Explanation: The slit skin smear is the most appropriate diagnostic test for leprosy, as it

**directly detects
Mycobacterium leprae.**

76. What is the most effective management for a 36-year-old female patient diagnosed with acute viral disease secondary to influenza?

(A) Bed rest – Bed rest is effective in supporting the body's immune response and is a primary management strategy for influenza.

(B) Hot compression – Hot compression is not effective for viral infections like influenza.

(C) Diet rich in proteins – A protein-rich diet can support

recovery but is not the primary intervention.

(D) Administer hydrochloride – Hydrochloride is not a treatment for influenza.

Correct Answer: (A) Bed rest

Explanation: Bed rest is the most effective management for acute viral infections like

influenza to help the body recover.

Here are the answers and explanations for the questions:

25. What is the most likely diagnosis for a female client with a soft nasal bump that spread to her lip, with discrete,

**thin-walled vesicles covered
with honey-colored crusts?**

(A) Eczema – Eczema involves inflammation and itchiness but doesn't typically present with honey-colored crusts.

(B) Impetigo – Impetigo is characterized by honey-colored crusts that form after thin-walled vesicles break,

often starting around the nose and spreading to the lips.

(C) Carbuncle – A carbuncle is a deep skin infection, often with multiple abscesses, not associated with honey-colored crusts.

(D) Pemphigus – Pemphigus involves deep, fragile blisters, not the honey-colored crusts seen in impetigo.

Correct Answer: (B) Impetigo

Explanation: Impetigo typically presents with honey-colored crusts after the vesicles rupture, commonly seen around the nose and mouth.

26. What effect is most likely expected after 24 hours of resuscitation in a burn patient with 50% total body surface area involvement?

(A) Increment in PaO_2 – PaO_2 may improve after resuscitation, but this is not the most likely immediate effect.

**(B) Reduction in BUN level –
BUN may decrease as renal
perfusion improves, but this
isn't as immediate as urine
output.**

**(C) Increment in urine output –
A common response to
adequate resuscitation in burn
patients is increased urine
output as kidney function
improves.**

(D) Reduction in serum creatinine level – Serum creatinine may decrease later but not immediately after resuscitation.

Correct Answer: (C) Increment in urine output

Explanation: After adequate fluid resuscitation, burn patients typically show an

increase in urine output as renal perfusion improves.

27. What is the most appropriate site where a patient feels intense pain on palpation with acute periumbilical pain and positive Rovsing's sign?

(A) Left lower quadrant –
Rovsing's sign refers to pain in
the right lower quadrant when
the left lower quadrant is
palpated, so the pain is felt in
the right lower quadrant.

(B) Left upper quadrant – Pain
is not typically felt in the left
upper quadrant in this case.

(C) Right lower quadrant – This
is the correct location for pain,

**as Rovsing's sign suggests
appendicitis.**

**(D) Right upper quadrant –
This is not the location of pain
in this case.**

**Correct Answer: (C) Right
lower quadrant**

**Explanation: Rovsing's sign is
positive when palpation of the
left lower quadrant causes**

pain in the right lower quadrant, often indicating appendicitis.

28. What is the most appropriate dietary advice to enhance functional health status for a 60-year-old male patient with fatigue, nausea, vomiting, constipation,

abdominal distension, and lab results showing decreased platelet count and liver enzyme levels?

(A) High fat diet – A high-fat diet may not be beneficial in this case, especially with liver issues.

(B) High Protein diet – A high-protein diet is usually helpful

in liver diseases and improving nutritional status.

(C) High-calorie diet – A high-calorie diet may be needed but may not address the specific liver dysfunction or protein needs.

(D) Nutritional supplements – Nutritional supplements can help, but a balanced diet with

adequate protein is more essential in this case.

Correct Answer: (B) High Protein diet

Explanation: A high-protein diet is important for patients with liver dysfunction to support healing and repair of tissues and muscles, and to address nutritional deficits.

29. What is the priority nursing diagnosis for a 28-year-old male patient diagnosed with peptic ulcer disease, currently receiving triple antibiotics, and referred for further management?

(A) Pain – Pain management is important, but it is not the priority in this case.

(B) Anxiety – Anxiety is a potential concern, but it is secondary to the immediate medical issues.

(C) Knowledge Deficit – Educating the patient about his condition and treatment plan is a priority for understanding

and managing his peptic ulcer disease.

(D) Imbalanced nutrition – Nutritional needs should be addressed, but the priority is patient education.

Correct Answer: (C) Knowledge Deficit

Explanation: The priority is to educate the patient about his

**condition, treatment options,
and lifestyle changes needed
to manage peptic ulcer
disease.**

**1. What is the most likely
diagnosis for a 57-year-old
diabetic male patient with
chills, chest pain, dyspnea,
cough with mucopurulent**

sputum, headache, crackles on auscultation, and opacity on chest X-ray?

(A) Asthma – Asthma typically presents with wheezing and airway obstruction, not the symptoms seen here.

(B) Bronchitis – Bronchitis involves coughing and sputum production but doesn't typically cause fever and

pneumonia-like opacity on X-ray.

(C) Pleuritis – Pleuritis involves inflammation of the pleura and may cause chest pain, but the symptoms are not fully consistent.

(D) Pneumonia – Pneumonia fits this patient's symptoms, including fever, cough,

sputum, and the chest X-ray findings.

Correct Answer: (D)

Pneumonia

Explanation: Pneumonia is the most likely diagnosis given the patient's fever, chest pain, productive cough, and findings of opacity on chest X-ray.

2. What is the most likely priority nursing intervention for a 28-year-old woman who underwent a tracheostomy and developed dullness over the right chest, stridor, rhonchi, poor concentration, and tachypnea?

(A) Wound care – Wound care is important but not the priority in this situation.

(B) Airway suctioning – Suctioning may be needed if the patient is having respiratory distress due to secretions, but the priority is addressing the underlying issue.

**(C) Proper positioning –
Positioning can help, but
airway management should be
prioritized.**

**(D) Oxygen administration –
The patient is exhibiting signs
of respiratory distress, and
oxygen administration should
be prioritized to improve
oxygenation.**

Correct Answer: (D) Oxygen administration

Explanation: Oxygen administration is the priority intervention to improve the patient's oxygenation and address respiratory distress.

3. What is the initial pathophysiological process

that occurs in bronchial asthma after exposure to a known allergen?

(A) Neutrophils infiltration – Neutrophil infiltration is more common in bacterial infections, not asthma.

(B) Lymphocytes production – Lymphocytes may play a role in asthma but are not the

primary cells involved in the immediate response.

(C) Mast cells activation – Mast cells release histamine and other chemicals that cause bronchoconstriction, which is the initial event in asthma attacks.

(D) Eosinophils release – Eosinophils are involved in the later stages of asthma

inflammation but are not the first responders.

Correct Answer: (C) Mast cells activation

Explanation: Mast cells are activated by allergens in asthma, leading to the release of histamine and other inflammatory mediators that cause bronchoconstriction.

Here are the answers and explanations for the new set of questions:

1. What is the priority intervention for a woman who complains of excessive premenstrual syndrome and other symptoms after six cycles?

(A) Remove the IUCD and appoint her after seven days – The cause of the symptoms may be related to the IUCD (Intrauterine Contraceptive Device), so removal and follow-up would be a logical option.

(B) Change the contraceptive – If the IUCD is suspected to be the cause of her symptoms, changing the method of

contraception could be appropriate.

(C) Counsel and support – Providing counseling and emotional support might help manage the psychological aspects of her condition but should not be the first step.

(D) Assess for any medical condition – This is the most comprehensive first step. The

symptoms may indicate a medical condition that requires assessment.

Correct Answer: (D) Assess for any medical condition

Explanation: The priority is to assess for any underlying medical conditions causing the symptoms, including the possibility of IUCD-related

issues or hormonal imbalances.

2. What is the initial laboratory investigation to be requested for a woman with infertility complaints and regular menstrual periods, with no previous contraceptive use?

(A) DNA analysis – DNA analysis is not typically the first step for infertility assessment.

(B) Semen analysis – This test is essential for evaluating male fertility, but since this question is about the woman, it's not the first step.

(C) Antibody studies – Antibody studies may be

needed in some cases, but it is not typically the first investigation for unexplained infertility.

(D) Sperm penetration assay – This is a specific test used in certain infertility cases but not the initial investigation for unexplained infertility.

Correct Answer: (B) Semen analysis

Explanation: The first step in infertility investigations is often to assess male fertility, including semen analysis, before further testing on the woman.

3. What is the most likely diagnosis for a 17-week pregnant woman with vaginal bleeding, lower abdominal cramping, backache, and mild uterine contractions, with a closed cervix and intact membranes?

(A) Incomplete abortion – This involves partial loss of pregnancy tissue but would typically show cervical dilation and expulsion of tissue.

(B) Threatened abortion – This is a common condition where the cervix remains closed, and pregnancy may still be viable, despite symptoms like bleeding and cramping.

(C) Inevitable abortion – This would involve cervical dilation, and in this case, the cervix is closed.

(D) Complete abortion – This involves complete expulsion of pregnancy tissue, which is not indicated by the current symptoms.

**Correct Answer: (B)
Threatened abortion**

Explanation: Threatened abortion is characterized by bleeding and cramping, but the

cervix remains closed, and the pregnancy may still continue.

4. What is the priority nursing diagnosis for a 16-year-old postpartum mother with urinary incontinence, a temperature of 37.9°C, and suspected vesico-vaginal fistula?

(A) Hypotension – This is not the priority in this case.

(B) Hyperthermia – The temperature of 37.9°C may indicate an infection, but the primary concern is the condition itself.

(C) Knowledge deficient – While the mother may need education about the condition,

the priority is the emotional and psychological impact.

**(D) Disturbed body image –
The priority nursing diagnosis is often related to the psychological distress caused by the condition, including body image concerns.**

Correct Answer: (D) Disturbed body image

Explanation: The priority is to address the psychological distress and potential body image issues associated with vesico-vaginal fistula, as it can have a significant emotional impact.

5. What is the most likely health care policy strategy

applied when decision-making, organization, capacity building, planning, implementation, and monitoring are transferred to the regions with a clear definition of roles?

(A) Democratization – This typically refers to the process of making something more democratic or participatory, but it doesn't exactly fit the description here.

(B) Decentralization – This is the transfer of authority and responsibilities from a central government to local or regional levels, which fits the description provided.

(C) Community participation – This focuses more on involving the community in decision-making but doesn't align exactly with the transfer of roles described.

(D) Inter-sectorial collaboration – This focuses on cooperation between different sectors but is not the primary strategy described.

**Correct Answer: (B)
Decentralization**

Explanation: Decentralization refers to the transfer of decision-making powers to

regional or local levels, as described in the scenario.

6. What is the most likely health insurance scheme recommended for a public that requests a system that does not rely much on local or national solidarity and formal labor markets?

(A) Social Health Insurance –
This is based on social
solidarity, which is not the
focus in this scenario.

(B) Individual Health Insurance
– This is typically based on
individual contributions and
may not fit the public's needs
for low reliance on solidarity
or formal labor markets.

(C) Voluntary Health Insurance
– Voluntary schemes allow individuals to choose participation but may not align with the scenario's criteria.

(D) Community-Based Health Insurance – This system is often locally organized and may not depend heavily on national solidarity or formal labor markets.

Correct Answer: (D)
Community-Based Health Insurance

Explanation: Community-Based Health Insurance is often tailored to local populations and does not rely heavily on formal labor markets, making it a suitable recommendation for this scenario.

7. What is the most appropriate type of evaluation for assessing the immediate observable effects of a program after one year?

(A) Process evaluation – This focuses on how a program is being implemented and its operational aspects.

(B) Impact evaluation – Impact evaluation assesses the long-term effects and outcomes, which is most likely what this question asks about.

(C) Formative evaluation – This is conducted during the development phase to improve the program.

**(D) Summative evaluation –
Summative evaluation
assesses the overall success of
a program at its conclusion.**

**Correct Answer: (B) Impact
evaluation**

**Explanation: Impact evaluation
focuses on assessing the
observable effects of a
program after it has been
implemented.**

8. What ethical principle is applied when a nurse encourages a hepatitis B patient to share her information with her family?

(A) Advocacy – Advocacy involves supporting the patient's rights and interests,

but the primary focus here is on the patient's decision to share information.

(B) Privacy – Privacy involves keeping the patient's information confidential, but in this case, the patient has chosen to disclose.

(C) Confidentiality – This principle involves protecting

patient information, but the patient is choosing to share.

**(D) Informed Consent –
Informed consent is the process by which a patient is given all necessary information to make a decision, which fits the context of this question.**

Correct Answer: (D) Informed Consent

Explanation: Informed consent applies here because the nurse is supporting the patient's decision to share her health information after understanding the implications.

9. What principle is applied when the hospital

management decides to provide cardiac surgery services based on the order of patients' visits due to resource scarcity?

(A) Justice – Justice focuses on fairness and equitable distribution of resources, which fits this scenario.

(B) Autonomy – Autonomy refers to respecting the

patient's right to make decisions, which is not the primary concern in this case.

(C) Beneficence – Beneficence focuses on providing care that benefits patients, but resource limitations may hinder its application here.

(D) Nonmaleficence – Nonmaleficence involves doing no harm, but the priority in

this case is ensuring fairness in the distribution of scarce resources.

Correct Answer: (A) Justice

Explanation: The principle of justice is applied when the allocation of resources is done fairly, based on the available information and the patients' needs.

Here are the answers and explanations for your latest set of questions:

85. What is the prevalence rate of common cold among health workers over a month if 10 and 38 health workers were investigated on the first and second day, respectively, and

no change was observed over the month in a hospital with 1000 health workers?

(A) 1.0% – This would indicate 10 cases out of 1000, which is too low.

(B) 1.8% – This is a possible answer but needs calculation.

(C) 4.8% – This would suggest more cases than expected.

(D) 3.8% – Similar to 4.8%, a more moderate value.

To calculate the prevalence rate:

Total cases = 10 (Day 1) + 38 (Day 2) = 48

Prevalence rate = $(48/1000) * 100 = 4.8\%$

Correct Answer: (C) 4.8%

Explanation: The prevalence rate is calculated as the total number of cases divided by the total population, then multiplied by 100.

86. What is the most likely interpretation of the association in a study where 40 out of 160 patients with acute appendicitis and 12 out of 320 healthy individuals have a history of urinary tract infection?

(A) Urinary tract infection is not a risk factor for acute appendicitis – This could be

the interpretation if there is no strong association.

(B) Acute appendicitis is the risk factor for urinary tract infection – This would suggest that appendicitis causes UTIs, which seems unlikely.

(C) Acute appendicitis is not a risk factor for urinary tract infection – The data doesn't

support this, as it is about the UTIs and not appendicitis.

(D) Urinary tract infection is a risk factor for acute appendicitis – This interpretation makes the most sense, as a higher proportion of appendicitis patients have had a UTI compared to the healthy individuals.

Correct Answer: (D) Urinary tract infection is the risk factor for acute appendicitis

Explanation: The data suggests a higher occurrence of urinary tract infection (UTI) in those with acute appendicitis, implying a possible association where UTI could be a risk factor for developing appendicitis.

87. What teaching method did the nurse likely use when each participant in a water treatment course was able to see and practice the water treatment process?

(A) Drama – This involves acting out scenarios, but it's not the primary method used here.

(B) Lecture – A lecture would typically involve verbal instruction but would not focus on hands-on practice.

(C) Brainstorming – Brainstorming involves group discussions to generate ideas, not hands-on practice.

(D) Demonstration – This method involves showing how

something is done, which fits the description of participants practicing the water treatment process.

Correct Answer: (D)

Demonstration

Explanation: Demonstration involves showing and practicing a process, which is exactly what the nurse did during the teaching session.

88. What is the most appropriate type of curriculum evaluation used when a curriculum development team involves the Ministry of Health (MOH) to ensure the curriculum meets national health professionals' quality standards?

(A) Internal evaluation – This type of evaluation is conducted within the organization and may not involve external review.

(B) External evaluation – This involves an outside body, such as the MOH, to review the curriculum for alignment with national standards.

(C) Formative evaluation – This is done during the development process to improve the curriculum but not necessarily with an external body like the MOH.

(D) Summative evaluation – This occurs at the end of a process, often to judge overall effectiveness.

Correct Answer: (B) External evaluation

Explanation: In this case, the involvement of an external body (MOH) to ensure national standards are met points to external evaluation.

22. What is the most likely nursing diagnosis for a one-

week-old neonate with poor feeding, cough, difficulty breathing during feeding, a pulse rate of 130/min, respirations of 58/min, central cyanosis, and systolic ejection murmur?

(A) Activity intolerance – This refers to difficulty with physical activity but is less relevant in a neonate with respiratory issues.

(B) Ineffective airway clearance – This could be a relevant diagnosis, but the cyanosis and murmur point to a different issue.

(C) Decreased cardiac output – The presence of cyanosis and a murmur suggests a possible cardiac issue, making this the most appropriate diagnosis.

(D) Ineffective breathing pattern – This could apply to respiratory issues, but the findings of cyanosis and murmur suggest a more serious underlying condition like a cardiac issue.

Correct Answer: (C) Decreased cardiac output

Explanation: The symptoms of cyanosis and murmur in this

neonate, along with difficulty feeding and breathing, suggest a possible congenital heart defect leading to decreased cardiac output.

23. What is the most likely next step in management for a preterm neonate in a neonatal ICU with a body temperature

of 35.5°C after receiving IV antibiotics and fluids, and kangaroo mother care?

(A) Changing the drug – The temperature is likely related to hypothermia, not an infection, so changing the drug is not the priority.

(B) Providing additional IV glucose – This could help with energy needs but would not

directly address the low temperature.

(C) Performing further investigations – Investigations are not immediately necessary in the case of hypothermia.

(D) Putting the neonate under a radiant warmer – This is the most appropriate action to help raise the body

temperature and stabilize the neonate.

Correct Answer: (D) Putting the neonate under radiant warmer

Explanation: The neonate is likely hypothermic, and placing them under a radiant warmer will help maintain their body temperature.

24. What is the most likely cause of a 4-hour-old very preterm neonate's body temperature of 35.6°C, born via caesarean section and placed nearby the unconscious mother after being covered with a blanket in a room at 26°C?

(A) Lack of feeding – Lack of feeding would not typically cause hypothermia in the first few hours.

(B) Low room temperature – While the room temperature is 26°C, this may not be sufficiently warm for a very preterm neonate.

(C) Lack of sufficient covering clothes – This may contribute

to heat loss but is unlikely to be the sole cause.

(D) Lack of skin-to-skin contact with the mother – Preterm neonates benefit from skin-to-skin contact to regulate body temperature, and the absence of this could be a key factor.

Correct Answer: (D) Lack of skin-to-skin contact with the mother

Explanation: Skin-to-skin contact helps maintain the neonate's body temperature, which is crucial for preterm babies.

25. What is the most likely finding to confirm the nurse's suspicion of severe pneumonia in a 5-year-old child with pneumonia who was previously treated according to IMNCI guidelines?

(A) Stridor – Stridor indicates upper airway obstruction, not typically associated with severe pneumonia.

(B) Wheezing – Wheezing is more commonly associated with asthma or bronchiolitis.

(C) Fast breathing – This is a common sign of pneumonia but may not indicate severe pneumonia.

(D) Chest indrawing – This is a key sign of severe pneumonia and indicates difficulty

breathing due to compromised lung function.

Correct Answer: (D) Chest indrawing

Explanation: Chest indrawing is a classic sign of severe pneumonia in children, indicating respiratory distress.

Here are the answers and explanations for your latest set of questions:

69. What is the most appropriate next step that the nurse should follow after administering a medication to a patient with gloves on?

(A) Remove gloves and administer the medication – It's important to remove gloves to avoid contamination when giving the medication.

(B) Leave the gloves and administer the medication – This would not be safe practice due to the potential for cross-contamination.

(C) Give the medication later to prevent removal of the gloves – This is not recommended as it delays care and doesn't address the need for proper hygiene.

(D) Remove gloves and wash hands before administering medication – This is the best practice to ensure hygiene and prevent contamination.

Correct Answer: (D) Remove gloves and wash hands before administering medication

Explanation: The nurse should always remove gloves and wash their hands before giving medication to maintain hygiene and reduce infection risk.

70. A 40-year-old male ingested a corrosive poison, and the nurse gave him milk as an emergency measure. For which condition is this patient at greatest risk?

(A) Gastritis – Corrosive ingestion could cause inflammation of the stomach lining (gastritis), but this isn't the most immediate risk after giving milk.

(B) Pneumonia – Milk could cause aspiration, especially if the patient vomits, leading to aspiration pneumonia.

(C) Lymphangitis – This is unlikely to be the main concern in this scenario.

(D) Tuberculosis – There's no direct correlation to tuberculosis in this situation.

Correct Answer: (B)

Pneumonia

Explanation: The patient is at greatest risk for aspiration pneumonia because milk may be aspirated into the lungs if the patient is not carefully managed.

71. A 65-year-old female developed severe headache, dysphagia, positive Chvostek's sign, and Trousseau's sign three days after being admitted for heart failure. What is the most likely diagnosis?

(A) Hypocalcaemia – Positive Chvostek's and Trousseau's signs are indicators of hypocalcemia, which is

consistent with the patient's symptoms.

(B) Hypokalemia – Hypokalemia could cause cardiac symptoms but does not cause Chvostek's or Trousseau's signs.

(C) Hypercalcemia – This would cause different symptoms, such as fatigue, nausea, and confusion.

(D) Hyperkalemia –
Hyperkalemia is associated
with cardiac issues but not the
symptoms described here.

Correct Answer: (A)
Hypocalcaemia

Explanation: The signs of
Chvostek's and Trousseau's are
specific indicators of
hypocalcemia, which is likely

the cause of the patient's symptoms.

72. A 22-year-old mentally unstable patient ingested an excessive dose of aspirin and is complaining of blurred vision, restlessness, and has a temperature of 38.7°C. What is the priority nursing diagnosis?

(A) Hyperthermia related to ingestion of poison – Hyperthermia is a potential symptom, but the focus here should be on the ingestion of poison.

(B) Risk for injury related to ingestion of poison – This is a valid concern, especially since the patient is mentally unstable.

(C) Impaired gas exchange related to central poisoning – Central poisoning could lead to impaired respiratory function, making this a relevant concern.

(D) Impaired sensory function related to central poisoning – The blurred vision could suggest impaired sensory function.

Correct Answer: (B) Risk for injury related to ingestion of poison

Explanation: The priority concern in this situation is ensuring the patient's safety due to the ingestion of poison, which could lead to further harm.

73. A two-year-old child presents with diarrhea and sunken eyes. The nurse classifies the child as having severe persistent diarrhea according to IMNCI. What is the most likely finding to confirm the classification?

(A) Drinks eagerly and thirsty with five days duration of

diarrhea – This finding suggests dehydration, but it's not severe persistent diarrhea.

(B) Skin pinch goes back very slowly with 12 days duration of diarrhea – This could indicate severe dehydration but not necessarily persistent diarrhea.

(C) Skin pinch goes back very slowly and 14 days duration of

diarrhea – This suggests severe dehydration and persistent diarrhea.

(D) Not able to drink or drink poorly with five days duration of diarrhea – This suggests severe dehydration but may not fully meet the criteria for persistent diarrhea.

Correct Answer: (C) Skin pinch goes back very slowly and 14 days duration of diarrhea

Explanation: A prolonged duration of diarrhea with severe dehydration, as indicated by the slow return of the skin pinch, confirms the classification of severe persistent diarrhea.

74. A 40-year-old mother on the PMTCT program comes with her 45-day-old baby for routine vaccination. What is the most appropriate counseling for infant feeding?

(A) Exclusive breast feeding for six months – This is the most appropriate recommendation for an HIV-exposed infant, as it reduces the risk of HIV transmission.

(B) Exclusive cow's milk feeding for six months – This is not recommended, as it does not provide the benefits of breast milk.

(C) Exclusive formula feeding for six months – Formula feeding could be used if breastfeeding is contraindicated, but exclusive breastfeeding is preferred in most cases.

(D) Expressed heat-treated breast milk for six months – This could be an alternative if direct breastfeeding is not an option.

Correct Answer: (A) Exclusive breast feeding for six months

Explanation: For a mother on the PMTCT program, exclusive breastfeeding is recommended

unless contraindicated, as it reduces the risk of HIV transmission.

75. A 12-day-old neonate is classified as jaundiced and managed according to IMNCI guidelines. In the current visit, the nurse suspects severe jaundice. What is the most

likely finding to confirm this suspicion?

**(A) Skin and eyes are yellow –
This is typical of jaundice, but
not necessarily severe
jaundice.**

**(B) Palm and sole are yellow –
This indicates more advanced
jaundice and could suggest
severe jaundice.**

**(C) High serum bilirubin level –
A high serum bilirubin level
would confirm severe
jaundice.**

**(D) High conjugated bilirubin
level – This could suggest liver
dysfunction, but it's not the
most immediate confirmation
for severe jaundice.**

Correct Answer: (C) High serum bilirubin level

Explanation: A high serum bilirubin level is the most reliable indicator for confirming severe jaundice.

76. A 16-year-old boy who was previously treated for pulmonary TB presents with a

cough and has a positive smear and Gene X-pert test. What is the most likely case definition for this patient?

(A) New – A "new" case refers to a patient who has never been treated for TB before, which doesn't apply here.

(B) Relapse – This patient had TB previously, and now

**symptoms have returned,
suggesting a relapse.**

(C) Treatment failure – This refers to a patient whose treatment did not succeed, but the patient was previously cured of TB.

(D) Treatment after loss to follow-up – This is not applicable as the patient has not been lost to follow-up.

Correct Answer: (B) Relapse

Explanation: Given the patient had previously been treated and is now showing symptoms again, this would be classified as a relapse.

77. What is the most appropriate care to give to a 37-week-old newborn from an HIV-positive mother immediately after birth?

(A) Give first-line ARTs – ARTs would be given as part of post-exposure prophylaxis, but this is not the first step.

(B) Initiate artificial feeding – This is not the primary

recommendation for HIV-exposed infants.

(C) Give Cotrimoxazole prophylaxis – Cotrimoxazole is usually given after the first few weeks of life for HIV-exposed infants.

(D) Give post-exposure prophylaxis – Post-exposure prophylaxis (PEP) with ART should be initiated

**immediately after birth to
prevent HIV transmission.**

**Correct Answer: (D) Give post-
exposure prophylaxis**

**Explanation: HIV-exposed
infants should receive post-
exposure prophylaxis (PEP)
with ART immediately after
birth to prevent transmission.**

Here are the answers and explanations for your latest set of questions:

45. A 16-year-old schizophrenic patient has been admitted with symptoms of delusions and paranoia. He believes that people are making gestures and

comments directed at him. He was treated for the same condition three years ago. What is the most important finding that supports the diagnosis of the patient?

(A) Presence of referential delusions – The belief that others' gestures and comments are specifically directed at oneself is a referential delusion, which is characteristic of schizophrenia.

(B) Symptoms in the last six weeks – Duration of symptoms is important, but the specific type of symptom (referential delusions) is more telling in this case.

(C) History of the same episode – A past history of a similar episode supports a diagnosis of schizophrenia, but the current delusions are more important for the diagnosis.

**(D) Risk of harming others –
While this is a concern, it does
not directly support the
diagnosis of schizophrenia.**

**Correct Answer: (A) Presence
of referential delusions**

**Explanation: The delusions of
being targeted by others'
gestures and comments**

(referential delusions) are a hallmark of schizophrenia.

46. A 17-year-old boy presents with elevated mood, inflated self-worth, distractibility, and flight of ideas every day. He is functioning well and does not need hospitalization. What is

the least required duration to diagnose his condition?

(A) One week – This would be sufficient to diagnose mania or hypomania in the context of a mood disorder.

(B) One month – Typically, a duration of at least one week is required for diagnosis, so one month is excessive.

(C) Six months – This is too long to wait for a diagnosis; mood disorders are diagnosed based on shorter timeframes.

(D) One day but four days – This is not a standard criterion for diagnosis.

Correct Answer: (A) One week

Explanation: The symptoms described (elevated mood,

flight of ideas) are consistent with hypomania, which requires at least one week of symptoms to meet diagnostic criteria.

47. A 40-year-old man on antidepressants was admitted after alcohol intoxication. He reports drinking excessively to

cope with life stressors over the past 10 years. What is the priority nursing diagnosis?

(A) Ineffective coping related to knowledge deficit – This could be a relevant concern, but it doesn't address the alcohol use problem directly.

(B) Risk of delirium tremens related to hospitalization – This is a valid concern for

someone with a history of alcohol abuse.

(C) Frequent hospitalization secondary to poor adherence – This is not as directly related to the current situation.

(D) Nonadherence to treatment regimen secondary to knowledge deficit – This is not the primary concern in this case.

Correct Answer: (B) Risk of delirium tremens related to hospitalization

Explanation: Given the patient's history of excessive alcohol use, the primary concern is the risk of withdrawal symptoms such as delirium tremens.

48. A 26-year-old business manager is unable to speak in front of people, which is affecting his ability to sustain his company. The problem has lasted for six years. What is the best psychological approach to manage his problem?

(A) Help him in uncovering and analyzing unconscious conflicts – This is a more psychoanalytic approach but might not be the best for addressing phobias.

(B) Expose him to the phobic situation for as long as they can tolerate – This approach could lead to overwhelming the patient and worsening anxiety.

(C) Help him actively confront the phobic situation with the help of a therapist – This is a good approach and is used in cognitive-behavioral therapy (CBT) to confront the phobia in a controlled manner.

(D) Expose him serially to a predetermined list of anxiety-provoking situations – This approach, known as systematic desensitization, is one of the most effective

treatments for specific phobias.

Correct Answer: (D) Expose him serially to a predetermined list of anxiety-provoking situations

Explanation: Systematic desensitization, which involves gradual exposure to anxiety-provoking situations, is one of the most effective treatments

for specific phobias like the one described.

75. During a seminar presentation, a graduating student explains the mode of transmission of leprosy. The students are confused and ask for clarification. What is the best answer to explain?

(A) Through sexual contact – This is not the correct mode of transmission for leprosy.

(B) Through respiratory secretion – Leprosy is primarily transmitted through respiratory droplets.

(C) Through broken skin and drainage – While leprosy can potentially be spread through

contact with skin lesions, this is not the main mode of transmission.

(D) Through mother to child transplacental – Leprosy is not transmitted from mother to child in utero.

Correct Answer: (B) Through respiratory secretion

Explanation: The primary mode of transmission for leprosy is through respiratory droplets from prolonged contact with an infected person.

Here are the answers and explanations for the new set of questions:

35. A 3-year-old HIV-positive male patient was admitted to a surgical ward after BPH surgery. One day after surgery, his condition deteriorated with disorientation, fluctuating consciousness, and perceptual distortions. What is the most likely diagnosis?

(A) Delirium – Delirium is characterized by an acute change in mental status,

fluctuating consciousness, and perceptual disturbances, often triggered by factors like surgery, medications, or infection.

(B) Medication-induced neurocognitive disorder – This is a possibility but less likely given the acute onset after surgery, making delirium a more probable cause.

(C) Neurocognitive disorder due to HIV infection – While HIV can lead to neurocognitive disorders, the rapid onset after surgery points more toward delirium.

(D) Neurocognitive disorder due to another medical condition – The symptoms could be related to an underlying medical condition, but delirium remains the most

likely cause due to the post-operative setting.

Correct Answer: (A) Delirium

Explanation: Delirium is most likely due to the acute onset of symptoms after surgery, which can be triggered by multiple factors, including medications and infection.

58. A six-year-old boy shows diminished sustained attention and high impulsivity, with academic difficulties and behavioral issues at home and school. What is the most likely diagnosis?

(A) Autistic disorder – Autism typically involves social

communication difficulties, not just attention and impulsivity.

(B) Conduct-related disorder – This involves more serious antisocial behaviors, which doesn't fit this case.

(C) Pervasive developmental disorders – This group of disorders includes autism, but the symptoms here are more aligned with attention issues

rather than developmental disorders.

**(D) Attention deficit hyperactivity disorder (ADHD)
– ADHD is characterized by attention problems, impulsivity, and hyperactivity, which fits the description of this child.**

Correct Answer: (D) Attention deficit hyperactivity disorder (ADHD)

Explanation: ADHD is the most likely diagnosis based on the symptoms of inattention, impulsivity, and academic difficulties.

57. The president of a public university in Ethiopia addresses overall activities of the university and signs a service provision agreement. What is the most likely managerial role played by the president?

(A) Liaison – A liaison role typically involves connecting people or groups.

(B) Figurehead – A figurehead represents the organization symbolically, which fits the scenario where the president addresses the public and signs agreements.

(C) Leader – While the president is a leader, this scenario focuses more on symbolic actions.

(D) Entrepreneur – An entrepreneur is someone who innovates or creates new ventures, which doesn't apply here.

Correct Answer: (B) Figurehead

Explanation: The president's actions, like making a speech and signing agreements, suggest a ceremonial role,

**fitting the figurehead
description.**

**58. The CEO of a hospital
organizes a SWOT analysis task
force that identifies issues like
drug shortages, poorly
motivated staff, and low
transportation availability.
What is the most likely**

**component of the SWOT
analysis observed here?**

(A) Strength – Strengths are positive aspects, not negative issues like shortages or low motivation.

(B) Weakness – These are internal issues that represent areas for improvement, such as the identified problems in the hospital.

(C) Opportunity – Opportunities refer to external factors that could be beneficial, which does not apply to internal issues like drug shortages.

(D) Threat – Threats are external factors, not internal weaknesses.

Correct Answer: (C) Weakness

Explanation: The identified issues—drug shortages, poor staff motivation, and lack of transportation—are internal weaknesses in the hospital.

59. A human resource officer wants to estimate the number of nurses needed next year.

What is the most appropriate base for estimating the human resources required by the hospital?

**(A) Health service targets –
These are useful for planning services, but not directly for estimating staff needs.**

**(B) Health needs approach –
This approach involves determining staff needs based**

on the health needs of the population but may not be the most specific method for estimating nurse numbers.

(C) The healthcare demand – This is an essential approach to estimating staffing requirements based on patient needs.

(D) Human resource-population ratios – This

method uses ratios of healthcare workers to population, which is a standard approach to estimate the required number of staff.

Correct Answer: (D) Human resource-population ratios

Explanation: This method is a well-established approach for estimating the required

**number of healthcare workers,
including nurses.**

31. A 3-day-old neonate with a diagnosis of neonatal sepsis is lethargic and has a pulse rate of 165/min, a respiratory rate of 60/min, and a body temperature of 36°C. What is

the most appropriate nursing management?

(A) Administer 100% oxygen – Neonatal sepsis can cause respiratory distress, but oxygen therapy should be based on the oxygen saturation levels.

(B) Give 5ml of 10% glucose – This could be appropriate if there is concern about

hypoglycemia, but no specific information about glucose levels is provided.

(C) Put the baby on kangaroo mother care – This is a good supportive care option, but it doesn't address the immediate medical needs of neonatal sepsis.

(D) Counsel the mother to continue breastfeeding –

Breastfeeding is important but may not address the immediate management of sepsis.

Correct Answer: (B) Give 5ml of 10% glucose

Explanation: In neonatal sepsis, ensuring that the baby has adequate glucose levels is essential for stabilizing the infant.

32. A 14-day-old neonate with neonatal jaundice and sepsis is having difficulty feeding and has a baseline temperature of 36.2°C. What is the most appropriate way to monitor the neonate's progress after hypothermia management?

(A) Offer feedings at 2-3 hours interval – This ensures adequate nutrition but does not directly monitor the neonate's temperature or glucose levels.

(B) Check if body temperature decreases – Monitoring temperature is critical in assessing recovery from hypothermia.

(C) Recheck blood glucose after 30 minutes – This is an important action, especially if the neonate was at risk for hypoglycemia.

(D) Look for signs of clinical hypoglycemia – It's important to monitor for clinical signs of hypoglycemia, but it's better to check glucose levels directly.

Correct Answer: (C) Recheck blood glucose after 30 minutes

Explanation: Rechecking glucose levels after initial management ensures that hypoglycemia is addressed promptly in a newborn with sepsis.

33. A 7-month-old infant with difficulty sucking breast milk and swallowing complementary foods is brought to a health center. What is the most important nursing diagnosis?

(A) Imbalanced nutrition – This is a relevant diagnosis given the feeding difficulties.

(B) Ineffective breastfeeding – This is important but not the most comprehensive diagnosis here.

(C) Risk for fluid volume deficit – This is a concern due to difficulty feeding, but it's secondary to the main issue.

(D) Ineffective infant feeding pattern – This diagnosis

accurately reflects the child's feeding difficulties.

Correct Answer: (D) Ineffective infant feeding pattern

Explanation: The difficulty with feeding and swallowing is the primary concern and the most accurate diagnosis.

34. A two-year-old patient has oral cavity ulcers with difficulty chewing but no obstruction in the gastrointestinal tract.

What is the most likely nursing diagnosis?

(A) Imbalanced nutrition – This could be relevant if the child is not eating properly due to the oral ulcers.

(B) Impaired swallowing – The issue described seems more related to chewing than swallowing.

(C) Risk for fluid volume deficit – This is a concern but secondary to the difficulty in chewing.

(D) Impaired oral mucosal membrane – This is the most appropriate diagnosis for oral

ulcers affecting the child's ability to chew.

Correct Answer: (D) Impaired oral mucosal membrane

Explanation: The oral ulcers directly affect the mucosal membrane and cause difficulty in chewing, making this the most appropriate diagnosis.

Here are the answers and explanations for the new set of questions:

45. A 21-year-old woman with no previous history of mental illness complains of hearing persistent voices commenting on her actions for the last three months. The mental

status examination reveals frequent derailment of speech and apathy. What is the most likely diagnosis?

(A) Schizophrenia – Schizophrenia is characterized by auditory hallucinations, disorganized speech, and negative symptoms such as apathy. This is the most likely diagnosis.

(B) Brief psychotic disorder –
This involves sudden onset of psychosis, but the duration of symptoms (3 months) suggests schizophrenia rather than a brief episode.

(C) Schizoaffective disorder –
Schizoaffective disorder involves mood disturbances along with psychotic symptoms, but there's no mention of a mood disorder here.

(D) Schizophreniform disorder
– Schizophreniform is diagnosed when symptoms last for more than a month but less than six months, and the patient's symptoms have lasted three months, making schizophrenia more likely.

Correct Answer: (A)
Schizophrenia

Explanation: The patient exhibits classic signs of schizophrenia, including auditory hallucinations, disorganized speech, and apathy.

46. A known adult patient with schizophrenia is on haloperidol 5 mg. After resisting

medication, he began showing returning symptoms. There is no evidence of drug side effects. What is the most appropriate drug of choice?

(A) Thioridazine – A first-generation antipsychotic, but there are newer options that are preferred due to side effects.

(B) Chlorpromazine – Another first-generation antipsychotic, but not the most appropriate choice.

(C) Fluphenazine decanoate – A long-acting injectable antipsychotic, which might be appropriate in cases where patients have trouble adhering to medication.

(D) Combination of haloperidol and carbamazepine – A combination of antipsychotics and mood stabilizers might be appropriate in some cases, but there's no indication here that carbamazepine is necessary.

Correct Answer: (C)

Fluphenazine decanoate

Explanation: Fluphenazine decanoate is a long-acting

**antipsychotic that can help
with medication adherence
and control symptoms.**

**47. A 19-year-old male student
reports sleep disturbances,
difficulty concentrating, and
periods of extreme muscle
tension. He worries constantly
about his parents, his**

university studies, and many other issues. He recognizes his worries are often unfounded, but he cannot stop worrying. What is the most likely diagnosis?

(A) Panic disorder – Panic disorder involves episodes of intense fear or discomfort, but it doesn't fit the chronic worry pattern described.

(B) Separation anxiety disorder
– This involves excessive anxiety about separation from attachment figures, not general worries.

(C) Social anxiety disorder –
This involves fear of social situations, but the student's issues are broader than social situations.

(D) Generalized anxiety disorder (GAD) – GAD is characterized by excessive and uncontrollable worry about multiple areas of life, which fits the patient's symptoms.

**Correct Answer: (D)
Generalized anxiety disorder
Explanation: GAD is characterized by chronic worry about a variety of life events,**

which matches this student's symptoms.

48. A 20-year-old university student with a history of anxiety disorder is found sitting in his dorm room, claiming to hear voices when no one is around. What is the most likely current

psychopathology of this student?

(A) Thought disturbance – This could refer to disorganized thinking but is less specific to auditory hallucinations.

(B) Emotional disturbance – This would relate more to mood issues rather than hallucinations.

**(C) Cognitive disturbance –
Cognitive disturbances
typically involve memory or
perception issues, but this case
centers on perception.**

**(D) Perceptual disturbance –
The student is experiencing
auditory hallucinations, which
are a form of perceptual
disturbance.**

Correct Answer: (D) Perceptual disturbance

Explanation: The student is experiencing auditory hallucinations, which are disturbances in perception.

49. A 30-year-old male patient presents with persistent low mood, insomnia, hopelessness,

and worthlessness for the last two weeks. There is no underlying medical condition. What is the most likely diagnostic symptom to be used to diagnose his problem?

(A) Low mood – This is a primary symptom of depression but doesn't alone confirm the diagnosis.

(B) Insomnia – Insomnia is a common symptom in depression but is not the key diagnostic feature.

(C) Hopelessness – Hopelessness is a core symptom of depression and often used in diagnosing major depressive disorder.

(D) Suicidal ideation – Suicidal ideation is serious and

common in depression but not the most diagnostic in this case.

Correct Answer: (C)

Hopelessness

Explanation: Hopelessness is a hallmark symptom of major depressive disorder, strongly supporting the diagnosis.

60. The Ministry of Health certified a hospital for excellent infection prevention performance. What is the most likely standard plan used by the hospital?

(A) Rules – Rules are specific guidelines for behavior but might not cover all aspects of infection prevention.

(B) Policies – Policies provide general guidelines for action and are likely used in infection prevention practices.

(C) Programs – Programs are usually broader initiatives but might not specifically focus on infection prevention.

(D) Procedures – Procedures are detailed steps to carry out

tasks, which are directly relevant to infection prevention.

Correct Answer: (B) Policies

Explanation: Infection prevention policies set broad guidelines that hospitals must follow to ensure safety and compliance.

61. The HR department of a primary hospital prepared a document with job descriptions, position specifications, and posted it for applicants. What is the most likely human resource activity?

(A) Staffing – Staffing involves hiring employees, but this step goes beyond just posting.

(B) Selection – Selection refers to choosing candidates for positions, which typically occurs after the document is posted.

(C) Orientation – Orientation is about introducing new employees to the organization.

(D) Recruitment – Recruitment involves the process of attracting and posting for job openings.

**Correct Answer: (D)
Recruitment**

Explanation: The HR department is engaging in recruitment by posting the job

descriptions and attracting candidates.

62. A health manager at a general hospital wants to decide the effectiveness of previous hiring procedures and plans to motivate staff. What is the most appropriate performance data to use?

**(A) Performance appraisal –
Performance appraisals are
useful for evaluating the
effectiveness of staff.**

**(B) Organizational analysis –
This involves analyzing the
overall health of the
organization, not specific
performance data.**

(C) Compensation record – This tracks pay but does not evaluate performance directly.

(D) Operational analysis – This would focus more on the efficiency of the hospital's operations rather than individual staff performance.

**Correct Answer: (A)
Performance appraisal**

Explanation: Performance appraisals directly evaluate staff effectiveness and can help assess the outcomes of previous hiring processes.

63. Nurses attend a morning session where one nurse shares their achievements of using time and medical

supplies properly. What management concept is being highlighted?

(A) Efficiency – Efficiency is about doing things with minimal waste and maximum output, which this nurse demonstrated.

(B) Information – This refers to data and facts, not the management concept here.

(C) Effectiveness –

Effectiveness is achieving desired results, but the focus here is more on the efficient use of resources.

(D) Work Relations – Work relations are about interpersonal dynamics, not the main focus in this scenario.

Correct Answer: (A) Efficiency

Explanation: The nurse is being recognized for efficiently using time and resources, making efficiency the correct concept.

64. A newly appointed matron influenced colleagues by organizing nurses to follow standardized procedures. As a

result, the hospital won an annual award. What leadership characteristic did the matron exhibit?

(A) Copying – Copying involves imitating others, not leading through innovation.

(B) Imitation – Similar to copying, this involves following someone else's methods.

(C) Innovation – Innovation involves creating new methods or approaches, which fits the matron's actions.

(D) Maintenance – Maintenance focuses on preserving existing systems, not changing them.

Correct Answer: (C) Innovation

Explanation: The matron's efforts to introduce standardized procedures and improve hospital performance demonstrate innovation.

Here are the answers and explanations for the new set of questions:

65. A matron has conducted supervision in the inpatient wards, communicated with staff, and made corrections on resource mobilization.

However, the matron receives complaints about her inability to bring change and use subordinates' expertise. What is the most likely supervisory role missed by the matron?

(A) Provide performance feedback to staff – This is

important for motivating and guiding staff, but the matron seems to be missing the ability to engage with staff expertise and bring change.

(B) Periodic assessment of staff's performance – Assessing staff performance periodically is crucial for understanding their progress, but the problem here seems to be a lack of engagement and using the expertise of staff.

(C) Work with the staff to deliver quality service – The matron is not working effectively with the staff, which is the key issue raised.

(D) Identify standards of good performance to staff – The matron has identified performance standards, but it's the lack of collaborative engagement and change

management that's being criticized.

Correct Answer: (C) Work with the staff to deliver quality service

Explanation: The matron is not effectively engaging with the staff to leverage their expertise, which affects overall service delivery and change management.

66. A zonal health administration team plans to perform controlling activities in the HIV free generation project and wants to know the number of nurses trained on ART and VCT. What is the most likely monitoring result level to be used by the team?

(A) Input – Input refers to resources, like the number of nurses trained.

(B) Process – Process involves monitoring the procedures or activities being carried out, not necessarily the number of trained personnel.

(C) Output – Output involves the results of the activities,

such as the number of trained nurses.

(D) Outcome – Outcome focuses on the long-term effects of the project, such as the reduction in HIV transmission.

Correct Answer: (A) Input

Explanation: The team is focused on the number of

trained nurses, which is an input measure to track resource allocation.

67. A researcher observes that worksite injuries are common in the metal manufacturing industry and wants to research the magnitude and associated

factors. What is the most likely study design for this research?

(A) Cohort – Cohort studies track groups over time, but this study is more focused on the magnitude of injuries at a point in time.

(B) Ecological – Ecological studies look at populations and groups, not individual risk factors.

(C) Case control – Case-control studies are used to compare individuals with a condition to those without it, often used to identify risk factors.

(D) Cross-sectional – Cross-sectional studies look at data at a single point in time and are ideal for determining the prevalence of injuries.

Correct Answer: (D) Cross-sectional

Explanation: A cross-sectional study is ideal for assessing the prevalence and factors associated with worksite injuries at a specific point in time.

68. A researcher selects only male secondary school students to assess alcohol drinking. What is the most likely type of error made by the researcher?

(A) Random error – Random errors are due to chance and don't involve systematic biases like this.

(B) Sampling error – Sampling error involves errors in how the sample is chosen, such as selecting only males, leading to biased results.

(C) Non-systematic error – Non-systematic error refers to unplanned errors, which is not the main issue here.

(D) Non-sampling error – Non-sampling errors arise from

other aspects of data collection, but the issue here is more about sampling method.

Correct Answer: (B) Sampling error

Explanation: By only selecting male students, the researcher has introduced a sampling error, which leads to bias in the study results.

69. A nurse researcher selects 500 households from 10,000 households using systematic sampling, with the 4th household selected first. What is the most likely sequence of selected households?

(A) 4, 6, 8, 12 – This sequence doesn't match the systematic sampling method.

(B) 4, 24, 44, 64 – This sequence correctly reflects systematic sampling where every 20th household is selected (calculated by dividing 10,000 by 500).

(C) 4, 30, 58, 82 – This sequence also reflects

systematic sampling but with different interval calculations.

(D) 4, 504, 1004, 1504 – This sequence is too large for the given sample size.

Correct Answer: (B) 4, 24, 44, 64

Explanation: The interval between selections is 20 ($10,000 \div 500$), so every 20th

**household after the 4th one
would be selected.**

70. A one-day-old neonate is brought to the hospital with complaints of difficulty breastfeeding and yellowish discoloration of palms and soles. The baby was born at 30 weeks gestation. What is the

most likely indicator to classify the child as a preterm baby?

(A) Thick – This does not relate to preterm classification.

(B) Tachycardia – Tachycardia could indicate distress but not necessarily preterm status.

(C) Tachypnea – Tachypnea is rapid breathing and could

**indicate respiratory distress
but not preterm status.**

**(D) Weak sucking reflex –
Weak sucking is often seen in
preterm babies, who may have
underdeveloped feeding
reflexes.**

**Correct Answer: (D) Weak
sucking reflex**

Explanation: Preterm infants often exhibit weak sucking reflexes due to underdeveloped oral motor coordination.

71. A child is brought to the hospital with fever, chills, and loss of appetite, with a temperature of 38°C and a

respiratory rate of 33/min. The child is diagnosed with pneumonia. What additional manifestation is needed to confirm the diagnosis?

(A) Chest in-drawing – This refers to signs of severe respiratory distress, which can confirm pneumonia.

(B) Convulsion – Convulsions are not typical in pneumonia

and would suggest another condition.

**(C) Inspiratory stridor –
Inspiratory stridor typically
indicates upper airway
obstruction, not pneumonia.**

**(D) Cyanosis – Cyanosis could
indicate a lack of oxygen and
may be seen in pneumonia but
is not a specific diagnostic sign.**

Correct Answer: (A) Chest in-drawing

Explanation: Chest in-drawing is a sign of respiratory distress, which can confirm the diagnosis of pneumonia.

72. A 7-year-old child presents with a cough lasting three

weeks, weight loss, and a history of contact with a tuberculosis patient. What is the most likely confirmatory screening method for tuberculosis?

(A) Radiography – Chest X-rays are often used to confirm tuberculosis in symptomatic patients.

(B) Clinical manifestations – Clinical signs alone are not sufficient to confirm tuberculosis.

(C) Tuberculin Skin Test – The tuberculin test is used for screening but not for confirmatory diagnosis.

(D) Acid-fast bacilli test of sputum – The acid-fast bacilli test is used to confirm the

**presence of tuberculosis
bacteria.**

**Correct Answer: (D) Acid-fast
bacilli test of sputum**

**Explanation: The acid-fast
bacilli test on sputum is the
gold standard for confirming
tuberculosis.**

73. A 2-year-old child is brought to the hospital with diarrhea for two days. The child is lethargic, unable to drink, and has sunken eyes. What is the most likely classification of dehydration based on IMNCI protocol?

(A) No dehydration – The child shows signs of severe dehydration.

(B) Dehydration – The child is more than mildly dehydrated.

(C) Some dehydration – The child has more severe dehydration signs.

(D) Severe dehydration – The child's symptoms indicate severe dehydration.

Correct Answer: (D) Severe dehydration

Explanation: The child is lethargic, unable to drink, and has sunken eyes, all of which are indicators of severe dehydration.

74. A neonate born at 36 weeks gestation with jaundice

and a serum bilirubin level of 12 mg/dl is brought to the hospital. What is the most appropriate nursing intervention?

(A) Provide exchange transfusion – Exchange transfusion is typically used for severe hyperbilirubinemia, but this case may not require it.

(B) Put the neonate on phototherapy – Phototherapy is often used for jaundice to reduce bilirubin levels.

(C) Warm the infant by skin-to-skin contact – This is helpful for temperature regulation but not directly for jaundice.

(D) Advise the mother to expose to natural light – This can help with jaundice, but

phototherapy is more effective.

Correct Answer: (B) Put the neonate on phototherapy

Explanation: Phototherapy is the standard treatment for jaundice in neonates with elevated bilirubin levels.

Here are the answers and explanations for the latest set of questions:

14. A patient was treated with antifungal drugs. What was the most likely diagnosis?

(A) Tinea pedis – This is a fungal infection of the feet,

commonly known as athlete's foot, which would be treated with antifungal medication.

(B) Tinea corporis – This is a fungal infection of the body (ringworm), also treatable with antifungals.

(C) Tinea capitis – This is a fungal infection of the scalp, which requires antifungal treatment.

(D) Tinea unguium – This is a fungal infection of the nails, which is also treated with antifungal drugs.

Correct Answer: (C) Tinea capitis

Explanation: The most common reason for treating with antifungals in children is a fungal infection of the scalp,

**which would likely be
diagnosed as tinea capitis.**

**15. A 2-year-old child is
irritable, drooling, and has a
persistent cough. What is the
priority nursing diagnosis?**

**(A) Ineffective breathing
pattern – This could be**

relevant due to difficulty breathing but doesn't seem to be the priority here.

(B) Impaired sleep pattern – This doesn't seem to be the most critical diagnosis in this case.

(C) Poor body temperature control – The child doesn't exhibit significant fever or temperature instability.

(D) High risk for hypoglycemia
– The child's main issue
appears related to airway or
respiratory distress, not
hypoglycemia.

Correct Answer: (A) Ineffective
breathing pattern

Explanation: The child is
exhibiting signs that could
indicate difficulty breathing,

such as drooling and coughing, which suggests a compromised airway and breathing pattern.

16. A 3-year-old child has severe cough with a high-pitched crowing sound and vomiting, and a family history of a similar case. What is the priority nursing intervention?

(A) Isolate the child – If this child has suspected croup, isolation may not be necessary unless there is suspicion of a contagious disease like respiratory viruses.

(B) Give cough depressants – Cough suppressants may not be recommended for children with respiratory issues like croup.

(C) Administer maintenance fluid – This could be helpful, but the priority would be addressing the respiratory distress.

(D) Maintain blood transmission precaution – This is not relevant to the child's condition.

Correct Answer: (A) Isolate the child

Explanation: The child's symptoms, including a high-pitched cough (stridor), are indicative of croup, and isolation may be needed if there's suspicion of a contagious infection.

17. An 11-year-old child with full-thickness burns is admitted. What is the appropriate amount of fluid to provide during the second eight hours using the Parkland formula?

(A) Half of fluid required in 24 hours – This is incorrect; the second half is given during the remaining 16 hours.

(B) Quarter of fluid required in 24 hours – The second 8 hours should account for more fluid.

(C) One-third of fluid required in 24 hours – This is the correct amount to be given during the second 8 hours.

(D) Two-third of fluid required during the first day – This is incorrect since it applies to the

total fluid volume, not the breakdown.

Correct Answer: (C) One-third of fluid required in 24 hours

Explanation: According to the Parkland formula, the fluid is given in three phases, and one-third of the total volume is provided in the second 8 hours.

18. A 5-year-old child has pain while swallowing, difficulty breathing, and inflamed pharynx with petechial rashes. What is the most likely cause?

(A) Escherichia coli – E. coli is a bacterium that causes gastroenteritis, not throat infections.

**(B) Group B streptococci –
Group B strep can cause severe
infections in neonates but is
less likely in this child.**

**(C) Haemophilus influenzae
virus – Haemophilus influenzae
can cause respiratory
infections, but the symptoms
seem more indicative of a
bacterial cause.**

(D) Group A B-hemolytic streptococcus – This bacterium is a common cause of pharyngitis and could explain the child's symptoms.

Correct Answer: (D) Group A B-hemolytic streptococcus

Explanation: This is a common cause of throat infections in children, and the physical

findings are consistent with a streptococcal infection.

19. A pregnant woman with a diagnosis of amniotic fluid disorder is seen at an ANC visit. What is the best characteristic that led to this diagnosis?

**(A) Fetal heart rate 110 bpm –
A fetal heart rate of 110 bpm is
normal and doesn't indicate
amniotic fluid problems.**

**(B) Increased fetal weight –
Increased fetal weight may
suggest conditions like
gestational diabetes but is not
directly related to amniotic
fluid disorders.**

(C) Blood pressure 40/60 mmHg – Extremely low blood pressure could be indicative of complications, but it's not typical of amniotic fluid disorders.

(D) Amniotic fluid index 24 – An elevated amniotic fluid index (AFI) of 24 suggests polyhydramnios, which is an amniotic fluid disorder.

Correct Answer: (D) Amniotic fluid index 24

Explanation: An AFI of 24 is considered high and indicates polyhydramnios, which is an amniotic fluid disorder.

20. A 7-week pregnant woman presents with pallor and

fatigue. What is the most appropriate lab investigation to confirm anemia?

(A) Urine analysis – This would not confirm anemia.

(B) Hematocrit – Hematocrit is a good test to confirm anemia by measuring the proportion of red blood cells in the blood.

(C) Blood sugar – Blood sugar tests are not used to diagnose anemia.

(D) VDRL test – The VDRL test screens for syphilis, not anemia.

**Correct Answer: (B)
Hematocrit**

Explanation: A hematocrit test is used to assess anemia by

evaluating the percentage of red blood cells in the blood.

21. A pregnant woman comes for a second ANC visit, and her fundal height is above the umbilicus by four fingers. What is the most likely gestational age?

(A) 32 weeks – The fundal height above the umbilicus suggests a gestational age closer to 32 weeks.

(B) 28 weeks – This is a possibility but seems a bit early for the described measurement.

(C) 24 weeks – This would be more typical for a smaller

fundal height above the umbilicus.

(D) 20 weeks – This is too early to have the described fundal height.

Correct Answer: (A) 32 weeks

Explanation: Fundal height typically corresponds closely to gestational age in weeks. A measurement above the

**umbilicus by four fingers
suggests a gestational age of
around 32 weeks.**

**22. A 32-week pregnant
woman presents with fever,
chills, and loss of appetite, and
her blood film shows
Plasmodium falciparum. What**

is the most likely maternal complication?

(A) IUGR (Intrauterine Growth Restriction) – Malaria can lead to IUGR due to placental insufficiency.

(B) Anemia – Malaria can cause anemia, especially in pregnant women.

(C) Post-term labor – This is unlikely to be a direct result of malaria.

(D) Spontaneous abortion – Malaria can increase the risk of spontaneous abortion, especially in the early stages of pregnancy, but this is less likely at 32 weeks.

Correct Answer: (A) IUGR

Explanation: Malaria can cause intrauterine growth restriction (IUGR) due to placental dysfunction, which impairs nutrient and oxygen supply to the fetus.

Here are the answers and explanations for the latest set of questions:

24. A four-year-old child is brought to a health center with physical examination showing 110 bpm pulse rate, 90 mm Hg BP, and body temperature of 38°C. What is the appropriate management?

(A) Refer urgently to hospital – This may not be necessary unless the condition worsens.

(B) Provide oral amoxicillin for 5 days – Antibiotics could be useful if there's a bacterial infection, but further examination is required to confirm the diagnosis.

(C) Treat with Ampicillin and Gentamycin – These are appropriate antibiotics for serious infections but would need more clinical context.

(D) Counsel the mother to give fluid remedies – This would be more relevant for managing a mild illness or dehydration.

Correct Answer: (A) Refer urgently to hospital

Explanation: Given the fever and signs of possible infection, this child should be urgently referred to a hospital for

proper evaluation and management.

25. A 6-month-old HIV-positive baby presents with tachypnea, scattered rales in the lungs, and signs of respiratory distress. What is the most likely infectious disease?

(A) Pneumocystis carinii pneumonia – This is a common pneumonia in HIV-infected infants and children.

(B) Pulmonary Tuberculosis – TB can also present with respiratory symptoms but is less common in this age group unless there's a known exposure.

(C) Miliary Tuberculosis – This is less likely without a history of TB exposure or clinical signs of systemic spread.

(D) Severe Pneumonia – While pneumonia is possible, the presence of tachypnea and rales makes Pneumocystis pneumonia more likely in an HIV-positive infant.

Correct Answer: (A)
Pneumocystis carinii
pneumonia

Explanation: Pneumocystis pneumonia is common in HIV-positive infants and presents with respiratory distress and abnormal lung sounds.

26. A nursing team prepares an antibody test for infants. When is the most appropriate time to do this test?

(A) At birth – This is typically done at birth for HIV exposure.

(B) At 6 weeks – This is a common time for HIV testing in infants to check for transmission.

(C) At 10 weeks – HIV testing is usually done earlier, at birth and 6 weeks.

(D) At 1 year – While testing is done at 1 year, it's not ideal for early diagnosis.

Correct Answer: (B) At 6 weeks

Explanation: HIV testing in exposed infants is commonly performed at 6 weeks to

**assess possible HIV
transmission.**

**27. A pediatric nurse gives
essential newborn care and
observes that the newborn
breathes spontaneously. What
is the next appropriate step?**

(A) Cord care – Cord care is important but should follow immediate respiratory assessments.

(B) Apply Tetracycline eye ointment – This is done later to prevent infection in the eyes.

(C) Initiate breastfeeding immediately – While breastfeeding is important, the priority after ensuring

**respiratory stability is
providing warmth and contact.**

(D) Place the baby to skin-to-skin contact – This is the correct immediate next step for promoting warmth and bonding.

Correct Answer: (D) Place the baby to skin-to-skin contact

Explanation: Skin-to-skin contact is essential immediately after birth to regulate temperature and promote bonding.

28. A five-day-old term neonate has neonatal sepsis. The mother had fever and offensive vaginal discharge

before delivery. What is the most likely cause of sepsis in the neonate?

(A) Staphylococci – Staph infections are common but less likely from vaginal flora.

(B) Escherichia coli – E. coli is a common cause of neonatal infections but is not usually related to vaginal discharge in this context.

**(C) Haemophilus influenzae –
This is less likely to cause
neonatal sepsis in this case.**

**(D) Group B Streptococcus –
Group B Streptococcus is a
common cause of neonatal
sepsis, especially in infants
born to mothers with risk
factors such as fever and
vaginal discharge.**

Correct Answer: (D) Group B Streptococcus

Explanation: Group B Streptococcus is the most common cause of neonatal sepsis, particularly in the context of maternal infection.

29. A preterm baby has mild fever, difficulty feeding, and yellow conjunctivitis. What is the most likely criterion for diagnosing neonatal jaundice?

(A) Mild fever – Mild fever isn't specific to jaundice.

(B) Difficulty of feeding – While feeding difficulties may occur, they are not specific to jaundice.

(C) Pale eyelids – Pale eyelids are not a common feature of jaundice.

(D) Yellow conjunctivitis – Jaundice causes yellowing of the skin and sclera (eye whites), not conjunctivitis.

Correct Answer: (D) Yellow conjunctivitis

Explanation: The yellowing of the conjunctiva (scleral icterus) is a key indicator of neonatal jaundice.

30. A child has red and swollen eyes with pus drainage. What is the most appropriate management for this neonate?

(A) Clean the eyelids with sterile normal saline and apply tetracycline ointment – This is appropriate management for conjunctivitis.

(B) Apply eye drops – Eye drops may be helpful, but tetracycline ointment is a common treatment.

(C) Apply systemic antibiotics – This is unnecessary unless there's systemic infection.

(D) Send specimen of pus for culture – Culturing is useful if there's concern about specific pathogens, but initial treatment with antibiotics is standard.

Correct Answer: (A) Clean the eyelids with sterile normal saline and apply tetracycline ointment

Explanation: This is the appropriate immediate management for conjunctivitis in a neonate.

31. A three-day-old neonate has sepsis, lethargy, and a pulse rate of 165/min, temperature of 36°C, and blood glucose of 50 mg/dL. What is the most appropriate nursing management?

(A) Administer 100% oxygen – Oxygen administration may be necessary, but the priority is glucose management.

(B) Give 5ml/kg 10% glucose – This is appropriate for addressing hypoglycemia in a neonate.

(C) Put the baby on kangaroo mother care – This is important for stabilizing the infant's temperature but not the immediate priority.

(D) Counsel the mother to continue breastfeeding – This

**is important for overall care
but not the immediate
response to hypoglycemia.**

**Correct Answer: (B) Give
5ml/kg 10% glucose**

**Explanation: Immediate
management of low blood
sugar in a neonate includes
giving glucose.**

32. A 24-week-old neonate with jaundice and sepsis presents with difficulty feeding, low temperature, and hypoglycemia. What is the most appropriate way to assess progress after initial management of hypothermia?

(A) Offer feedings at 2-3 hours interval – This is an important

step but doesn't directly assess progress.

(B) Check if the child's body temperature is decreasing – Monitoring temperature is crucial to assessing progress.

(C) Recheck blood glucose after 30 minutes – Rechecking blood glucose is crucial to assess if hypoglycemia is resolved.

(D) Look for clinical signs of hypoglycemia – Clinical signs are useful but rechecking blood glucose is more objective.

Correct Answer: (C) Recheck blood glucose after 30 minutes

Explanation: After initial management, checking blood glucose is the most direct way to monitor the neonate's

**progress in resolving
hypoglycemia.**

33. A seven-month-old infant is unable to suck breast milk and has difficulty swallowing complementary foods. What is the most important nursing diagnosis?

(A) Imbalanced nutrition – This may be a concern but is not the primary diagnosis.

(B) Ineffective breastfeeding – This seems relevant, but it focuses specifically on breastfeeding.

(C) Risk for fluid volume deficit – This is a concern if feeding problems are not addressed.

(D) Ineffective infant feeding pattern – This is the most appropriate diagnosis related to the child's difficulty feeding.

Correct Answer: (D) Ineffective infant feeding pattern

Explanation: The child's inability to suck breast milk and difficulty with complementary foods

indicates an ineffective feeding pattern.

34. A two-year-old child has oral cavity ulcers with difficulty chewing. What is the most likely nursing diagnosis?

(A) Imbalanced nutrition – This could be relevant but focuses

on nutrition rather than the underlying problem.

(B) Impaired swallowing – This diagnosis would apply if the difficulty with swallowing was more related to a physical or structural issue.

(C) Risk for fluid volume deficit – This is a valid concern but not the main focus here.

(D) Impaired oral mucosal membranes – This is the most appropriate diagnosis given the oral cavity ulcers.

Correct Answer: (D) Impaired oral mucosal membranes

Explanation: The presence of oral ulcers makes "Impaired oral mucosal membranes" the most appropriate nursing diagnosis.

Here are the answers and explanations for the given family planning and gynecological questions:

1. A 25-year-old lactating mother seeks family planning services. She gave birth less than six weeks ago and is exclusively breastfeeding, with

no menstruation, BP 120/78mmHg, pulse 85/min, and weight 60kg. What is the most appropriate family planning method for this mother?

(A) Calendar method – This method depends on regular menstruation and may not be ideal as she is not menstruating yet.

(B) Progesterone only pills –
These are suitable for
breastfeeding mothers, but the
lactational amenorrhea
method is more natural and
effective at this stage.

**(C) Lactational amenorrhea
method (LAM) – This is the
most appropriate as it is a
natural contraceptive method
for women who are exclusively
breastfeeding, up to six
months postpartum.**

(D) Combined oral contraceptive pills – These are not ideal for breastfeeding mothers, as they can affect milk supply.

Correct Answer: (C) Lactational amenorrhea method

Explanation: LAM is the most effective and safe method during the first six months

postpartum if the mother is exclusively breastfeeding.

2. An HIV-positive woman comes to a family planning clinic for contraceptive services. She is on treatment for gonorrhea and has been on HAART for three years. What is the most appropriate

contraceptive method for this woman?

(A) IUCD with condom – This is a safe option as it provides dual protection (contraceptive and HIV prevention).

(B) Implant with condom – Implants are safe for HIV-positive women, but using condoms is still necessary for dual protection.

(C) Diaphragm with condom – This is not commonly recommended as a primary method, as condoms alone are the most effective in HIV prevention.

(D) Spermicide with condom – Spermicide alone is not recommended for HIV-positive individuals due to potential irritation and increased risk of transmission.

Correct Answer: (A) IUCD with condom

Explanation: The IUCD is safe for HIV-positive women and can be used along with condoms for added protection.

3. A 25-year-old woman prefers to use the injectable depot medroxyprogesterone acetate (DMPA). What is the most likely return visit for the next injection?

(A) Every month – DMPA injections are typically given every three months, not monthly.

(B) Every six months – This is not the standard schedule for DMPA.

(C) Every nine months – DMPA requires injection every three months.

(D) Every three months – This is the correct interval for DMPA injections.

Correct Answer: (D) Every three months

Explanation: DMPA is administered every three months.

4. A 25-year-old woman experiences frequent menstrual bleeding lasting more than seven days after

taking combined oral contraceptives. What is the most likely menstrual pattern for this woman?

(A) Metorrhagia – This refers to irregular, frequent bleeding, but not specifically prolonged.

(B) Menorrhagia – Menorrhagia refers to abnormally heavy bleeding,

but the woman's bleeding duration is also significant.

(C) Polymenorrhea – This refers to frequent menstrual cycles, not prolonged bleeding.

(D) Menometrorrhagia – This is a combination of menorrhagia and metrorrhagia, involving both heavy and prolonged bleeding.

Correct Answer: (D)

Menometrorrhagia

Explanation:

Menometrorrhagia describes heavy and prolonged menstrual bleeding, which matches the woman's symptoms.

5. A married couple has been trying to conceive for two years without success. What is the most likely first laboratory investigation for this couple?

(A) Urine HCG test – This test checks for pregnancy, but the couple is trying to conceive, not confirming pregnancy.

(B) Semen analysis – This is the first step in infertility

investigations to evaluate male fertility.

(C) Hormonal assay – This may be useful but is typically done after semen analysis if needed.

(D) Serum FSH level – FSH levels may be tested later for female fertility assessment.

Correct Answer: (B) Semen analysis

Explanation: Semen analysis is the first step in infertility investigations for the male partner.

6. A 30-year-old mother with vesicovaginal fistula secondary to obstructed labor is

scheduled for repair. What is the most appropriate nursing intervention for this mother?

(A) Emotional support – Providing emotional support is critical in managing the distress and mental health of the mother.

(B) Maintain patient on clear liquids – Maintaining nutrition and hydration is important

before surgery but not the primary nursing intervention.

(C) Insert catheter in the bladder – This is likely needed for managing urinary function in the presence of a fistula.

(D) Encourage frequent sitz baths – Sitz baths may be helpful post-surgery but not immediately pre-surgery.

Correct Answer: (A) Emotional support

Explanation: Emotional support is a priority for the mother, as the condition can cause significant psychological distress.

7. An 18-year-old lady has abdominal swelling, dull abdominal pain, and an enlarged, firm, and nodular liver. Imaging shows a unilateral solid tumor over 10 cm in diameter, originating from the primitive yolk sac. What is the most likely diagnosis?

**(A) Dysgerminoma tumor –
Dysgerminomas are ovarian**

tumors but are typically not associated with yolk sac origin.

(B) Choriocarcinoma tumor – This is associated with trophoblastic tissue, not yolk sac tumors.

(C) Immature teratoma tumor – Teratomas are typically cystic, not solid, and are not associated with yolk sac origin.

**(D) Endodermal sinus tumor –
This is the correct diagnosis, as
it arises from the yolk sac and
often presents as a solid
tumor.**

**Correct Answer: (D)
Endodermal sinus tumor**

**Explanation: Endodermal sinus
tumors (yolk sac tumors) are
common in young women and**

originate from yolk sac elements.

8. A 20-year-old pregnant woman is diagnosed with stage II cervical cancer. What is the most likely manifestation that corresponds to this stage?

**(A) Dry and itchy vaginal ulcer
– This is more likely for earlier
or localized disease.**

**(B) Adhesions and fixed pelvic
organs – This is characteristic
of advanced disease.**

**(C) Tender and watery vaginal
discharge – Watery discharge
is a common symptom of
cervical cancer.**

**(D) Excessive tumor growth
accumulating in the cul-de-sac
– This describes a more
advanced stage of cervical
cancer.**

**Correct Answer: (C) Tender
and watery vaginal discharge**

**Explanation: Watery vaginal
discharge is common in
cervical cancer, especially in
the early to moderate stages.**

Here are the answers and explanations for the questions based on the given scenarios:

1. Post-surgical management after a major procedure requires changing the management options for fluids. What is the most

appropriate fluid to be used in this situation?

(A) Normal saline – Normal saline is commonly used for general fluid replacement and during the postoperative period.

(B) Ringer's lactate solution – This solution is often preferred after major surgeries as it helps maintain electrolyte

balance and mimics the body's natural fluid composition.

Correct Answer: (B) Ringer's lactate solution

Explanation: Ringer's lactate is ideal post-surgery as it helps maintain a good balance of fluids and electrolytes.

2. A nurse is teaching students about wound healing phases and precautions after surgery. What event will most likely occur at the phase described?

(A) Tissue remodeling – This occurs in the final phase of wound healing, after collagen deposition and scar formation.

(B) Reepithelialization – This is part of the proliferative phase, where the skin heals and regrows.

(C) Collagen deposition – This occurs during the proliferative phase.

Correct Answer: (C) Collagen deposition

Explanation: Collagen deposition is a key event during the proliferative phase of wound healing.

3. A 3-year-old female presents with vision problems, light avoidance, and eye rubbing. What is the priority

nursing diagnosis for this child?

(A) Pain – Although pain may be present, the symptoms suggest a sensory issue rather than just pain.

(B) Disturbed body image – This may be relevant, but it does not match the symptoms provided.

(C) Disturbed sensory perception – The child's avoidance of light, eye rubbing, and squinting point to a vision issue, making this diagnosis most appropriate.

(D) Situational low self-esteem – This does not align with the child's clinical presentation.

Correct Answer: (C) Disturbed sensory perception

Explanation: The symptoms point toward a disturbance in vision or sensory perception, making this the most appropriate diagnosis.

4. A head nurse explains the standard precaution type

applied after decontaminating medical equipment. What is the most appropriate standard precaution?

(A) Contact precaution – This applies when an infection is spread through direct contact.

(B) Airborne precaution – This applies to diseases that spread through airborne droplets.

(C) Droplet precaution – This applies when the infection spreads through droplets from coughing or sneezing.

(D) Isolation precaution – This is a general term that includes all types of precautions.

Correct Answer: (A) Contact precaution

Explanation: Decontaminating equipment is part of contact precautions to prevent the spread of infections.

5. A male patient with a closed arm fracture on traction for two weeks is being prepared for discharge. What problem

**should the nurse focus on
during patient education?**

**(A) Compartment syndrome –
This is a potential issue with
fractures, especially if there is
swelling or restricted blood
flow.**

**(B) Muscle atrophy – Muscle
atrophy is a risk with
prolonged immobilization but**

is not typically the most immediate concern.

(C) Fat embolism – Fat embolism is a risk with long bone fractures, especially early on, but it is less likely after two weeks of traction.

(D) Infection – Infection risk is generally a concern, especially in open fractures, but less

**relevant in closed fractures
after two weeks.**

Correct Answer: (A)

Compartment syndrome

**Explanation: Compartment
syndrome is a potential
concern with any fracture and
should be monitored during
the recovery period.**

6. A surgical procedure is planned for a patient with potential airway issues. What is the most likely indication for the procedure?

(A) Laryngomalacia – This condition may cause airway obstruction in infants but is not typically an immediate surgical concern.

(B) Vocal cord injury – This could cause airway obstruction and may need surgical intervention.

(C) Laryngeal cancer – This can cause airway issues and may require surgery, depending on the stage.

(D) Laryngeal obstruction – This is a primary concern that

might require surgical intervention.

Correct Answer: (D) Laryngeal obstruction

Explanation: Laryngeal obstruction is a critical condition that typically requires surgical intervention.

7. A 15-year-old female is undergoing a gynecological procedure. What category of equipment should be prepared for this procedure?

(A) Probing and dilating – This equipment is generally used for procedures requiring insertion or dilation.

**(B) Cutting and dissecting –
This is for procedures that
involve incision and removal.**

**(C) Grasping and clamping –
This equipment is used when
tissue needs to be grasped or
held during a procedure.**

**(D) Exposing and retracting –
This equipment is used to
expose tissues during surgery.**

Correct Answer: (D) Exposing and retracting

Explanation: Exposing and retracting instruments are needed in most gynecological procedures to hold back tissues and provide a clear view.

8. A 55-year-old woman is scheduled for surgery under general anesthesia. What type of anesthesia is most appropriate for this procedure?

(A) Spinal and epidural blocks
– These are regional anesthetics that are effective for specific areas of the body but are less appropriate for abdominal surgery.

(B) General anesthesia – This is appropriate for abdominal surgery where full-body anesthesia is needed.

(C) Local anesthesia – This is used for minor, localized procedures, not major surgery.

(D) Nerve blocks – Nerve blocks are used for regional anesthesia but not for general body procedures.

Correct Answer: (B) General anesthesia

Explanation: General anesthesia is appropriate for abdominal surgeries where the patient needs to be completely unconscious.

9. A nurse is assessing a patient undergoing laparotomy under general anesthesia. What is the most likely complication the patient is at risk for?

(A) Vomiting and aspiration – Vomiting and aspiration are a risk during general anesthesia.

(B) Atelectasis – Atelectasis is a common complication after

general anesthesia due to shallow breathing or lung collapse.

(C) Peripheral edema – This can occur post-surgery, but is not the most immediate concern with laparotomy.

(D) Anemia – Anemia is possible, but not as likely an immediate complication from laparotomy.

Correct Answer: (B) Atelectasis

Explanation: Atelectasis, or the collapse of lung tissue, is a common complication after surgery under general anesthesia.

Here are the correct answers along with explanations for each question:

1. Preferred Intramuscular Injection Site for Young Children

Answer: A. Vastus Lateralis

Explanation: The vastus lateralis muscle (located on the anterolateral thigh) is the preferred site for IM injections in infants and young children because it has a large muscle mass, fewer major nerves, and

less fat compared to other sites. The deltoid muscle is small and not well-developed in young children, while the gluteal muscles (ventrogluteal and dorsogluteal) pose risks of nerve injury.

2. Correct Intramuscular Injection Site in the Buttock

Answer: B. Upper Outer Quadrant in both Right (Rt) and Left (Lt) sides of the buttock

Explanation: The correct site for IM injections in the gluteal region is the upper outer quadrant to avoid damaging the sciatic nerve and major blood vessels. Injecting in the inner quadrants or lower parts of the buttock increases the

risk of hitting nerves or blood vessels, which can lead to complications.

3. Most Common and Easily Detectable Pulse Point

Answer: C. Carotid

Explanation: The carotid pulse is the most accessible and reliable site for assessing pulse in emergency situations, as it is close to the heart and has a strong pulse. The radial pulse is commonly used in stable patients, but it may be weak if the patient has low blood pressure. The brachial pulse is mainly used for infants, and the axillary pulse is not a standard site for pulse assessment.

4. Immediate Action After NG Tube Insertion

Answer: B. Aspirate for gastric secretions with a syringe

Explanation: After inserting a nasogastric (NG) tube, the first

priority is to confirm proper placement before starting feedings. Aspiration of gastric contents and checking pH levels help verify correct placement in the stomach. An X-ray is the most definitive confirmation, but it is not typically delayed for 24 hours. Starting feeding immediately without confirming placement can lead to complications such as aspiration pneumonia.

Here's a detailed explanation of each option for every question, including why the correct choice is the best and why the others are incorrect.

1. Preferred Intramuscular Injection Site for Young Children

Correct Answer: A. Vastus Lateralis

Explanation: The vastus lateralis muscle (outer thigh) is the safest and most recommended IM injection site for infants and young children. It has a large muscle mass, fewer nerves, and is easily accessible.

Other Options Explained:

B. Deltoid muscle: ✗ Not recommended for young children because it is small and underdeveloped, increasing the risk of nerve injury and poor absorption.

C. Ventrogluteal: ✗ Though a good choice for older children and adults, it is difficult to locate in young children due to undeveloped muscles.

D. Dorsogluteal: ✗ Not recommended because it is close to the sciatic nerve, increasing the risk of nerve damage and poor drug absorption.

2. Correct Intramuscular Injection Site in the Buttock

Correct Answer: B. Upper Outer Quadrant in both Right (Rt) and Left (Lt) sides of the buttock

Explanation: The upper outer quadrant of the gluteal region (also called the dorsogluteal site) is preferred because it reduces the risk of hitting the

sciatic nerve or major blood vessels.

Other Options Explained:

A. Upper Inner Quadrant: ✗
Risky because the sciatic nerve and major blood vessels are present in this area, increasing the chance of serious complications.

C. Lower Outer Quadrant: ✖

Not recommended due to the presence of major blood vessels, leading to a higher risk of vascular injury or hematoma formation.

D. Lower Inner Quadrant: ✖

The worst option, as it has the highest risk of nerve damage, blood vessel injury, and improper drug absorption.

3. Most Common and Easily Detectable Pulse Point

Correct Answer: C. Carotid

Explanation: The carotid artery is the best site for checking a pulse in emergency situations

because it is close to the heart and provides a strong, easily detectable pulse.

Other Options Explained:

A. Radial: ✗ Often used in routine assessments but can be weak or undetectable in emergency situations, especially if the patient is in shock or has low blood pressure.

B. Brachial: ✗ Primarily used for infants under 1 year of age. In adults, it is not as reliable or easily accessible as the carotid.

D. Axillary: ✗ Not commonly used for pulse assessment, as it is harder to palpate and less reliable in emergency situations.

4. Immediate Action After NG Tube Insertion

Correct Answer: B. Aspirate for gastric secretions with a syringe

Explanation: The first step after inserting an NG tube is to check for proper placement by

aspirating gastric contents and confirming the pH (should be acidic, around 1-5). This ensures the tube is in the stomach and not in the lungs.

Other Options Explained:

A. Begin feeding slowly to prevent cramping: ✗

Dangerous, as feeding should never begin until proper tube placement is confirmed. If the

tube is in the lungs, this could cause aspiration pneumonia.

C. Get an X-ray of the tip of the tube within 24 hours: ✗ X-ray is the gold standard for confirming placement, but waiting 24 hours is too long. It should be done immediately if needed.

D. Clamp off the tube until the feedings begin: ✗ This does

**nothing to confirm placement
and could delay appropriate
care.**

**Here's a breakdown of each
question with an explanation
for all answer choices:**

1. Bowel Sounds in Paralytic Ileus

Correct Answer: B. Hypoactive

Explanation: Paralytic ileus is a condition where the intestines lose normal motility due to nerve or muscle dysfunction, resulting in decreased or absent bowel sounds.

Other Options Explained:

A. Hyperactive: ✗ Seen in early bowel obstruction or diarrhea, but not in paralytic ileus.

C. High-pitched: ✗ Occurs in mechanical bowel obstruction, where the intestines are trying to push contents past an obstruction.

**D. Blowing: ✗ Not a
recognized term in bowel
sound assessment.**

**2. Most Appropriate Drug for
Asthma & Hypertension**

Correct Answer: D. Albuterol and Hydralazine

Explanation:

Albuterol (a β_2 agonist) is the preferred bronchodilator for asthma, as it does not significantly raise blood pressure.

Hydralazine is a vasodilator that lowers blood pressure

**without causing
bronchoconstriction.**

Other Options Explained:

A. Epinephrine and Atenolol:

✗ Epinephrine is a non-selective adrenergic agonist that raises BP and heart rate, worsening hypertension.

Atenolol (a β -blocker) can

worsen asthma by causing bronchoconstriction.

B. Diphenhydramine and

Hydralazine: ✗

Diphenhydramine

(antihistamine) is not a

bronchodilator and does not treat asthma.

C. Adrenaline and

Nitroprusside: ✗ Adrenaline

(Epinephrine) increases BP and

is not the first choice for asthma. Nitroprusside can dangerously lower BP.

3. Most Likely Valvular Lesion in Rheumatic Heart Disease

Correct Answer: C. Mitral Stenosis

Explanation: Mitral stenosis is the most common valvular lesion in rheumatic heart disease and is associated with a mid-diastolic murmur at the apex.

Other Options Explained:

A. Aortic regurgitation: ✗

Causes a diastolic murmur, but is not the most common in rheumatic heart disease.

B. Mitral regurgitation: ✗

Associated with a holosystolic murmur, not a mid-diastolic murmur.

D. Tricuspid regurgitation: ✗

Rare in rheumatic heart

**disease and associated with
systolic murmur.**

4. Correct I.V. Insertion Site Selection

**Correct Answer: B. Choose a
distal site**

Explanation: I.V. catheter insertion should begin at a distal site (hand or forearm) to allow for additional access options if needed.

Other Options Explained:

A. Choose a proximal site: ✖
Proximal veins should be reserved if the distal ones fail.

C. Have the client hold his arm over his head: ✗ Reduces venous filling, making insertion difficult.

D. Leave the tourniquet on for at least 5 minutes: ✗ Can cause vascular damage or hemoconcentration.

5. Most Likely Diagnosis in a Woman with Thirst & Polyuria

Correct Answer: D. Diabetes Insipidus

Explanation: Diabetes insipidus (DI) is characterized by polyuria and excessive thirst with normal blood glucose levels (RBS = 90 mg/dL), ruling out diabetes mellitus.

Other Options Explained:

A. Diabetes Mellitus: ✖

Would present with glucosuria and hyperglycemia.

B. Diabetic Ketoacidosis (DKA):

✖ Would show high glucose, ketones, and metabolic acidosis.

C. Gestational Diabetes: ✖
Would also cause high glucose levels, which are absent here.

6. Drug That Delays Disease Progression in Rheumatic Heart Disease

Correct Answer: B. Penicillin

Explanation: Penicillin prophylaxis prevents recurrent streptococcal infections, slowing the progression of rheumatic heart disease.

Other Options Explained:

A. Furosemide: ✗ Provides symptom relief but does not slow disease progression.

C. Enalapril: ✗ Helps with heart failure but does not prevent disease progression.

D. Digoxin: ✗ Used for heart failure and atrial fibrillation, but does not prevent rheumatic disease progression.

7. Best Treatment for Food Allergy Symptoms

Correct Answer: A. Loratadine

Explanation: Loratadine (a second-generation antihistamine) treats mild food allergy symptoms like rash, itching, and nausea.

Other Options Explained:

B. Azithromycin: ✗ An antibiotic, which is not useful for allergies.

C. Chloroquine: ✗ An antimalarial, irrelevant for food allergies.

D. Vitamin K: ✗ Used for clotting disorders, not allergic reactions.

**Here's a detailed explanation
for each question, with
reasons for the correct and
incorrect choices:**

1. Next Step in Management of Pleural Effusion in Pneumonia

**Correct Answer: C.
Thoracentesis**

Explanation: The patient has parapneumonic pleural effusion with a low pH (7.2) and high neutrophil count, indicating empyema.

Thoracentesis helps in draining infected fluid and relieving symptoms.

Other Options Explained:

A. Dexamethasone: ✗ A corticosteroid used for inflammatory conditions, but not for treating infected pleural effusion.

B. Chest CT scan: ✗ May help in complex cases, but thoracentesis is the immediate step.

D. Lung function test: ✗ Not helpful in acute infectious conditions.

2. Most Likely Diagnosis in a Patient with Ear Discharge and Neurologic Deficits

Correct Answer: A. Brain Abscess

Explanation: The patient has fever, headache, and focal neurologic deficits, along with a history of chronic ear discharge, suggesting an otogenic brain abscess.

Other Options Explained:

B. Brain Tumor: ✗ Tumors cause gradual progression of neurological symptoms, not acute fever and weakness.

C. Ischemic Stroke: ✗ No sudden onset, and fever is uncommon.

D. Subdural Hematoma: ✗ More common with head trauma and does not present with fever.

3. Expected Electrolyte Abnormality in DKA After Treatment

**Correct Answer: B.
Hypokalemia**

Explanation: Insulin therapy shifts potassium into cells, leading to hypokalemia, which can cause cardiac arrhythmias.

Other Options Explained:

A. Hypocalcemia: ✗ Not a typical feature of DKA.

C. Hyponatremia: ✗ Can occur, but hypokalemia is more concerning.

D. Hypophosphatemia: ✗ Can occur but is less significant than potassium loss.

4. Most Likely Diagnosis for Sinus Symptoms

Correct Answer: A. Sinusitis

Explanation: Symptoms of nasal congestion, headache, purulent discharge, and facial pressure indicate sinusitis.

Other Options Explained:

B. Rhinitis: ✗ Does not cause purulent discharge or facial pressure.

C. Common Cold: ✗ Usually resolves within a few days and has clear nasal discharge.

D. COVID-19: ✗ Would present with loss of smell, cough, and systemic symptoms.

5. First Treatment for Hypoglycemia in a Conscious Patient

Correct Answer: C. 15 to 20 g of a fast-acting carbohydrate such as orange juice

Explanation: Conscious hypoglycemic patients should receive oral glucose to quickly raise blood sugar.

Other Options Explained:

A. I.M. or subcutaneous glucagon: ✗ Used for severe hypoglycemia or unconscious patients.

**B. I.V. bolus of dextrose 50%:
✗ Only if the patient is unconscious or unable to take oral glucose.**

D. 10 U of fast-acting insulin:

✗ Insulin lowers blood sugar, worsening hypoglycemia.

**6. Most Likely Diagnosis in a
Woman with Excessive Menses
& Bleeding**

Correct Answer: B. Idiopathic Thrombocytopenic Purpura (ITP)

Explanation: Symptoms of mucosal bleeding, petechiae, and ecchymosis suggest low platelet count, characteristic of ITP.

Other Options Explained:

A. Hemophilia: ✖ Rare in women; causes deep joint bleeding, not mucosal bleeding.

C. Scurvy: ✖ Causes gum bleeding, but not heavy periods.

D. Vitamin K deficiency: ✖ Affects clotting factors, not platelets.

7. Most Likely Cause of Urethral Discharge

Correct Answer: D. Neisseria gonorrhoeae

Explanation: Purulent urethral discharge and dysuria suggest gonorrheal urethritis.

Other Options Explained:

A. T. Vaginalis: ✗ Causes mild urethritis, but not purulent discharge.

B. Candida Albicans: ✗ Causes itching and white curd-

like discharge, not purulent discharge.

C. Chlamydia trachomatis: ✖
Causes watery discharge, not abundant pus.

8. Positive Sign in Meningitis Exam

Correct Answer: C. Kernig's Sign

Explanation: Pain when extending the knee with the hip flexed suggests meningeal irritation (Kernig's sign).

Other Options Explained:

A. Brudzinski's Sign: ✖

Involves neck flexion causing knee flexion.

**B. Psoas Sign: ✖ Indicates
psoas abscess or appendicitis.**

**D. Rebound Tenderness: ✖
Suggests peritonitis, not
meningitis.**

9. Most Likely Chest Deformity

Correct Answer: B. Funnel Chest (Pectus Excavatum)

Explanation: Depression at the lower sternum is characteristic of pectus excavatum.

Other Options Explained:

A. Pigeon Chest: ✗ Opposite of funnel chest—chest protrudes outward.

C. Barrel Chest: ✗ Seen in COPD, with rounded chest.

D. Gibbon Chest: ✗ Not a recognized medical term.

10. Most Likely Diagnosis in a Child with Proteinuria & Edema

Correct Answer: B. Nephrotic Syndrome

Explanation: Proteinuria (+4), low albumin, and edema indicate nephrotic syndrome.

Other Options Explained:

A. Pyelonephritis: ✗ Presents with fever and flank pain, not massive proteinuria.

C. Glomerulonephritis: ✗ Associated with hematuria and hypertension, not severe proteinuria.

D. Cystitis: ✗ Causes dysuria and frequency, not nephrotic-range proteinuria.

11. Highest Priority Nursing Diagnosis in AIDS with PCP

Correct Answer: A. Impaired Gas Exchange

Explanation: Pneumocystis pneumonia (PCP) causes hypoxia and respiratory distress, making gas exchange the priority.

Other Options Explained:

B. Impaired Oral Mucous Membrane: ✗ Common in AIDS but not life-threatening.

C. Activity Intolerance: ✗ A concern but not the primary life-threatening issue.

D. Imbalanced Nutrition: ✗ Important, but oxygenation is the priority.

12. Definitive Diagnostic Test for Typhoid Fever

Correct Answer: B. Blood Culture

Explanation: Blood culture confirms Salmonella typhi infection.

Other Options Explained:

A. Widal Test: ✗ Low sensitivity and specificity.

C. Urine Analysis: ✗ Not used for typhoid diagnosis.

D. Stool Examination: ✗ Helpful but not definitive.

13. Best Parenteral Fluid for Hemorrhagic Shock

Correct Answer: D. Isotonic Ringer's Lactate

Explanation: Ringer's lactate restores circulating volume without causing fluid shifts.

Other Options Explained:

A. Colloid Total Parenteral Solution: ✗ Not used for acute resuscitation.

B. Hypotonic D5W + 1/2 NaCl: ✗ Can cause cerebral edema.

C. Hypertonic 3% Normal Saline: ✗ Used for severe hyponatremia, not hypovolemia.

Here's a detailed explanation for each question, with reasons for the correct and incorrect choices:

1. Best Investigation for Epistaxis, Fatigue, Fever, and Hepatosplenomegaly

Correct Answer: B. Bone Marrow Aspiration

Explanation: The symptoms suggest a hematologic malignancy (e.g., leukemia or aplastic anemia), which requires bone marrow aspiration for confirmation.

Other Options Explained:

A. Blood Culture: ✗ Useful for infectious causes, but not for hematologic disorders.

C. Stool Examination: ✗ Not relevant unless suspecting parasitic infection.

**D. Ultrasound of Abdomen:
✗ Can detect**

hepatosplenomegaly, but not the underlying cause.

2. Priority Nursing Diagnosis in Traumatic Brain Injury

Correct Answer: A. Ineffective Tissue Perfusion

Explanation: Brain injury can lead to cerebral hypoxia and poor perfusion, making tissue perfusion the priority.

Other Options Explained:

B. Ineffective Breathing Pattern:  Important but secondary to cerebral perfusion.

C. Disturbed Thought

Processes: ✖ A concern but not life-threatening.

D. Compromised Family

Coping: ✖ Not a priority in an acute setting.

3. Likely Physical Finding in Anemia

Correct Answer: B. Ejection Systolic Murmur

Explanation: Anemia causes high-output cardiac states, leading to a functional ejection systolic murmur due to increased blood flow.

Other Options Explained:

A. Apical Diastolic Murmur:

✗ Seen in mitral stenosis, not anemia.

**C. Narrow Pulse Pressure: ✗
Seen in cardiac tamponade or aortic stenosis.**

D. Rumbling Mid-Diastolic Murmur: ✗ Suggests rheumatic heart disease, not anemia.

4. Most Likely Diagnosis in Sudden-Onset Weakness

Correct Answer: B. Ischemic Stroke

Explanation: Sudden-onset weakness with maximal symptoms at onset suggests an ischemic stroke due to an arterial blockage.

Other Options Explained:

A. Hemorrhagic Stroke: ✗
Presents with severe headache and rapid deterioration.

C. Subarachnoid Hemorrhage:

✗ Causes sudden, severe headache (“thunderclap headache”).

D. Transient Ischemic Attack (TIA): ✗ Symptoms resolve within 24 hours.

5. Most Likely Diagnosis in Rectal Bleeding with Weight Loss

Correct Answer: B. Colorectal Cancer

Explanation: Rectal bleeding, weight loss, and tenesmus are classic signs of colorectal cancer.

Other Options Explained:

A. Amebic Dysentery: ✖

Causes bloody diarrhea with fever, not progressive symptoms.

C. Sigmoid Volvulus: ✖

Causes obstruction symptoms (abdominal distension, constipation), not rectal bleeding.

D. Ulcerative Colitis: ✗

Causes diarrhea with blood and mucus, but without weight loss in early stages.

6. Most Likely Diagnosis in an HIV-Infected Child with Progressive Dyspnea

**Correct Answer: B.
Pneumocystis Pneumonia
(PCP)**

**Explanation: Gradual
worsening of dyspnea,
bilateral lung disease on X-ray,
and severe hypoxia without
fever suggest PCP, a common
opportunistic infection in HIV.**

Other Options Explained:

A. Heart Failure: ✗ Rare in children, no history of cardiac disease.

C. Pulmonary Tuberculosis: ✗ Typically presents with fever, night sweats, weight loss.

D. Severe Bacterial Pneumonia: ✗ Would present acutely with fever and purulent sputum.

7. Best Investigation for Rectal Bleeding Without a Mass

**Correct Answer: C.
Colonoscopy**

Explanation: Colonoscopy is the best test for diagnosing

**colorectal cancer or polyps,
especially when DRE (digital
rectal exam) is non-revealing.**

Other Options Explained:

**A. Abdominal CT-Scan: ✗ Can
show tumors but lacks
diagnostic accuracy for
colorectal cancer.**

**B. Barium Enema: ✗ Less
accurate than colonoscopy.**

D. Upper GI Endoscopy: ✖
Used for upper GI bleeding,
not rectal bleeding.

**8. Most Difficult Disease to
Control Based on Transmission
Mode**

Correct Answer: A. Pulmonary Tuberculosis

Explanation: TB spreads via airborne transmission, making it harder to control than diseases with single transmission routes.

Other Options Explained:

B. Relapsing Fever: ✗ Spread via lice or ticks, easier to control.

C. Trachoma: ✗ Spread through direct contact, can be managed with hygiene and antibiotics.

D. Syphilis: ✗ Spread via sexual contact, easier to control with safe sex practices.

9. Best Diagnostic Test for Suspected Coronary Artery Disease (CAD) in CHF

Correct Answer: C. Cardiac Catheterization

Explanation: Cardiac catheterization (coronary angiography) is the gold standard for diagnosing coronary artery disease in heart failure patients.

Other Options Explained:

**A. Echocardiogram: ✗
Evaluates heart function but does not confirm CAD.**

B. Blood Urea Nitrogen (BUN):

✗ Measures kidney function, not cardiac disease.

D. Brain Natriuretic Peptide

(BNP): ✗ Helps diagnose heart failure severity, but not CAD.

---Here's a detailed explanation of each question,

**analyzing all options and
identifying the correct
answers:**

1. Levothyroxine (Synthroid) Teaching Plan

**Correct Answer: A. Should be
taken in the morning**

Explanation: Levothyroxine should be taken in the morning on an empty stomach to ensure optimal absorption and prevent interference with other medications or food.

Other Options Explained:

B. May decrease the client's energy level: ✗

Levothyroxine increases

energy levels by correcting hypothyroidism.

C. Must be stored in a dark container: ✗ It does not require special storage beyond normal medication guidelines.

D. Will decrease the client's heart rate: ✗ Levothyroxine increases heart rate by boosting metabolism.

2. Neurological Assessment Post-Right CVA (Stroke) – Urgent Finding

**Correct Answer: A. Decrease in
level of consciousness**

Explanation: A declining level of consciousness suggests worsening brain function or increased intracranial pressure (ICP), requiring immediate intervention.

Other Options Explained:

B. Loss of bladder control:  Common in stroke but not immediately life-threatening.

C. Altered sensation to stimuli:

✗ Expected in stroke patients.

D. Emotional lability: ✗ Not urgent, though common in stroke due to frontal lobe involvement.

3. Priority Nursing Goal for Renal Calculi (Kidney Stones)

Correct Answer: C. Manage pain

Explanation: Pain management is the priority because kidney stones cause severe pain (renal colic).

Other Options Explained:

A. Maintain fluid and electrolyte balance: ✗
Important but not the first priority.

B. Control nausea: ✗ Nausea is secondary to pain.

D. Prevent urinary tract infection: ✗ Important but not the primary concern in an acute episode.

4. Post-Cardiac Catheterization Complication (First 24 Hours)

**Correct Answer: B. Thrombus
Formation**

**Explanation: Blood clot
formation (thrombosis) at the**

catheter site is a major risk after cardiac catheterization.

Other Options Explained:

A. Angina at rest: ✗ Not common after the procedure unless there's an underlying issue.

C. Dizziness: ✗ May occur but is not a major complication.

D. Falling blood pressure: ✖
Can occur, but thrombosis is the bigger concern.

5. Malfunctioning Morphine Drip (Basal + PRN Dosing)

Correct Answer: B. The client states, "I just can't get relief from my pain."

Explanation: If the patient still has pain despite the drip, it indicates that the pump is not delivering the medication properly.

Other Options Explained:

A. Discomfort at IV site: ✖

Suggests local irritation but not pump failure.

C. Drug level drop from 100 ml to 80 ml in 4 hours: ✖

Expected if dosing is correct.

D. Drug level drop from 100 ml to 50 ml in 4 hours: ✖ Also expected based on dosage.

6. Emergency Room Triage – Most Urgent Case

Correct Answer: A. 2-month-old infant with bulging fontanel and crying

Explanation: Bulging fontanel in an infant suggest increased intracranial pressure (e.g.,

**meningitis, hydrocephalus),
requiring immediate attention.**

Other Options Explained:

B. Teenager with singed beard:

**✗ Indicates airway burn risk
but not the most urgent.**

C. Elderly with liquid stools:

**✗ Likely gastroenteritis, not
immediately life-threatening.**

D. Middle-aged with scapular pain: ✗ Likely gallbladder disease, which is not the top priority.

7. Infection Most Commonly Leading to Pelvic Inflammatory Disease (PID)

Correct Answer: B. Chlamydia

Explanation: Chlamydia trachomatis is the most common cause of PID, which can lead to infertility.

Other Options Explained:

A. Trichomoniasis: ✗ Causes vaginal infection but not a major PID cause.

C. Staphylococcus: ✗ Rarely causes PID.

D. Streptococcus: ✗ More commonly associated with postpartum infections, not PID.

8. Nutritional Advice for Coronary Artery Disease (CAD)

Correct Answer: C. Avoiding very heavy meals

Explanation: Large meals increase heart workload and trigger angina in CAD patients.

Other Options Explained:

A. Eating 3 balanced meals: ✖
Important but not specific for CAD.

B. Adding complex carbohydrates: ✖ **Helps with general health but is not the key recommendation.**

D. Limiting sodium to 7g/day: ✖ **Too high; the recommended limit is less than 2.3g/day.**

9. Physical Finding in Anemia

**Correct Answer: B. Ejection
Systolic Murmur**

Explanation: Anemia increases blood flow, causing a functional systolic murmur.

Other Options Explained:

A. Apical Diastolic Murmur:

✗ Suggests mitral stenosis, not anemia.

C. Narrow Pulse Pressure: ✗

Seen in cardiac tamponade, not anemia.

**D. Rumbling Mid-Diastolic
Murmur: ✗ Associated with
mitral stenosis.**

**10. Cause of Sudden
Deterioration in Stroke Patient
with Respiratory Distress**

Correct Answer: D. Pulmonary Embolism

Explanation: Stroke patients are at high risk for deep vein thrombosis (DVT), which can cause a pulmonary embolism.

Other Options Explained:

A. Acute Myocardial Infarction:

✗ Less likely without cardiac history.

B. Aspiration Pneumonia: ✗
Would present gradually with fever and cough.

C. Fat Embolism: ✗ More common in long bone fractures, not stroke.

11. Most Common Cause of Hemorrhagic Stroke in Ethiopia

**Correct Answer: D.
Hypertension**

Explanation: Uncontrolled hypertension is the leading cause of hemorrhagic strokes

worldwide, including in Ethiopia.

Other Options Explained:

A. Aneurysm: ✗ Can cause stroke, but hypertension is more common.

B. Anticoagulation: ✗ Less common cause.

C. Brain Tumor: ✖ Rarely causes hemorrhagic stroke.

12. Least Likely Comorbidity in a Woman with Anemia (Hb = 8 g/dL, WBC = 4200, Platelets = 170,000)

Correct Answer: B. Chronic Obstructive Pulmonary Disease (COPD)

Explanation: COPD more commonly causes polycythemia (high RBCs), not anemia.

Other Options Explained:

A. Chronic Kidney Disease: ✖
Can cause anemia due to decreased erythropoietin.

C. Colorectal Cancer: ✖ Can
cause iron deficiency anemia
due to chronic bleeding.

D. Disseminated Tuberculosis:
✖ Can cause anemia due to
chronic inflammation and
malnutrition.

Here's a breakdown of each question, with explanations for each option:

Heart Failure Functional Classification

Question: A 45-year-old male with heart failure develops dyspnea and fatigue while walking to a nearby shop and must rest for 5 minutes before improving. What is his classification?

A. Stage A, Class II – Incorrect. Stage A includes patients at risk for heart failure but without symptoms.

B. Stage B, Class II – Incorrect.
Stage B refers to structural heart disease but without symptoms.

C. Stage C, Class IV – Incorrect.
Class IV means symptoms occur even at rest.

D. Stage C, Class III – Correct.
Stage C includes structural heart disease with symptoms.

Class III means symptoms occur with ordinary activity.

Empiric Therapy for Meningitis

Question: A 29-year-old alcoholic with fever, headache, and neck stiffness has

**suspected bacterial meningitis,
but lumbar puncture is
deferred due to papilledema.
What is the best empiric
therapy with ampicillin?**

**A. Vancomycin and
Chloramphenicol – Incorrect.
Chloramphenicol is less
effective for resistant
Streptococcus pneumoniae.**

**B. Ceftriaxone and Vancomycin
– Correct. Covers common
bacteria like *S. pneumoniae*
and *N. meningitidis*.**

**C. Ceftriaxone and
Chloramphenicol – Incorrect.
Lacks full MRSA and resistant
pneumococcal coverage.**

**D. Cloxacillin and Ciprofloxacin
– Incorrect. Not standard for
bacterial meningitis.**

Complication of Peptic Ulcer Disease

Question: What is the most common complication of peptic ulcer disease?

A. Hemorrhage – Correct.

Upper GI bleeding is the most common complication.

B. Gastric Outlet Obstruction – Possible but less common.

C. Malignancy – Rare. Most ulcers do not become cancerous.

D. Perforation – Possible, but hemorrhage occurs more frequently.

HIV and Dermatome-Based Rash

Question: An HIV-positive woman has vesicular rash along dermatomes (T4-5), confusion, and meningeal signs. What is the most likely diagnosis?

A. Fungal meningitis – Unlikely without chronic symptoms.

B. Pyogenic meningitis – Not typically associated with dermatomal rash.

**C. Tuberculosis meningitis –
Incorrect. Usually chronic
onset.**

**D. Viral meningoencephalitis –
Correct. Likely due to herpes
zoster (shingles), which can
cause encephalitis.**

Hepatic Injury Causes

Question: A patient with jaundice and AST/ALT >2000. What is the least likely cause?

A. Acetaminophen poisoning – Common cause of massive liver enzyme elevation.

**B. Acute viral hepatitis –
Correct. ALT/AST in viral
hepatitis rarely exceed 1000.**

**C. Alcoholic liver disease –
Possible, but AST/ALT ratio
would be 2:1.**

**D. Ischemic liver injury –
Severe elevations possible
with shock.**

Acute Viral Respiratory Infection (Influenza)

**Question: A patient with sore
throat and cough due to
influenza. Best management?**

**A. Bed rest – Correct.
Supportive care is first-line.**

B. Hot compression – No proven benefit.

C. Diet rich in proteins – Not the primary intervention.

D. Administer hydrochloride – Non-specific.

Cirrhosis Management

Question: A cirrhotic patient with ascites and leg swelling. Best initial management?

A. Furosemide – Not alone, as it can cause electrolyte imbalance.

**B. Furosemide and Spironolactone – Correct.
Spironolactone is first-line for ascites.**

C. Hydrochlorothiazide – Not used for cirrhosis.

D. Paracentesis and Furosemide – Paracentesis is reserved for refractory ascites.

Leprosy Diagnosis

Question: A woman with a painless facial lump and muscle weakness. Best test?

A. ESR – Non-specific.

B. Sputum AFB – Used for TB, not leprosy.

C. Gene Xpert – For TB, not leprosy.

D. Slit Skin Smear – Correct. Confirms Mycobacterium leprae.

Acute Kidney Injury from Obstruction

Question: A patient with renal calculi has no urine output. What should the nurse do first?

A. Check catheter patency – Correct. A blocked catheter can mimic anuria.

B. Initiate diuretics – May worsen an obstructed kidney.

C. Change patient position – Not helpful.

D. Consult physician – Needed if obstruction persists after catheter check.

Thyroid Mass Diagnosis

Question: A patient has a firm, fixed thyroid mass with hoarseness. Most likely diagnosis?

A. Thyroid adenoma – Usually mobile and non-fixed.

**B. Thyroid cancer – Correct.
Fixed mass and hoarseness
suggest malignancy.**

**C. Thyroiditis – More
commonly painful and tender.**

**D. Toxic nodule – Does not
usually present as a fixed
mass.**

Hepatitis B Confirmation

Question: A patient with jaundice and suspected hepatitis B. Best confirming lab test?

A. SGPT (ALT) = 9 U/L – Too low for acute hepatitis.

B. Albumin 5.6 g/dL – Not specific.

C. SGOT (AST) = 205 U/L – Elevated but non-specific.

D. Total bilirubin = 1.1 mg/dL – Mild elevation, not confirmatory.

Best diagnostic test: Hepatitis B surface antigen (HBsAg).

Asymptomatic Gallstones

Question: A patient has gallstones on ultrasound but no symptoms. Best management?

A. Cholecystectomy – Only for symptomatic patients.

**B. Reassurance – Correct.
Asymptomatic gallstones do not require treatment.**

C. ESWL – Used for kidney stones, not gallstones.

D. Ursodeoxycholic acid – Not recommended for asymptomatic stones.

Mastitis Diagnosis

Question: A lactating woman has breast pain, fever, and tenderness but no fluctuation. Most likely diagnosis?

A. Breast abscess – Would have fluctuation.

B. Breast cancer – Usually painless and chronic.

C. Fibroadenoma – Non-inflammatory and painless.

D. Mastitis – Correct. Common in lactating women, presents with pain, erythema, and fever.

**Here's a detailed explanation
of each question and answer:**

1. Alcohol Withdrawal in a Hospitalized Patient

Question: A 53-year-old male admitted with pneumonia starts developing insomnia, aggression, and tremors within 36 hours. What is the primary nursing action?

Answer: A. Re-assess the patient including history of alcohol use

A. Re-assess the patient including history of alcohol use



The patient's symptoms suggest alcohol withdrawal syndrome (AWS), which commonly manifests 6–48 hours after cessation of alcohol in dependent individuals. A focused reassessment, including alcohol history, is crucial.

B. Report to the respective physician as soon as possible



While notifying the physician is important, reassessment should come first to confirm AWS.

C. Physically restrain the patient to protect any harm



Restraints should be avoided unless absolutely necessary, as they can worsen agitation.

D. Record all findings and follow further progress ✖

Documentation is important but not the primary nursing action.

2. Diabetic Ketoacidosis (DKA) Management

Question: Patient with slurred speech, vomiting, dry mucosa, glucose 330 mg/dL, and ABG pH 7.02. What is the priority intervention?

Answer: C. Infuse 0.9% normal saline

A. Infuse 5% dextrose in water (D5%W) ✗

Dextrose is used later when glucose drops below 250 mg/dL, not initially.

B. Infuse half-strength (0.45%) normal saline ✗

Hypotonic fluids may worsen cerebral edema in DKA.

C. Infuse 0.9% normal saline



First-line treatment in DKA to correct dehydration.

D. Infuse 40% Glucose ✖

Would worsen hyperglycemia.

**3. Elderly Patient with
Confusion & Falls**

Question: An 87-year-old female with confusion and mild left hemiparesis. What test should be performed first?

Answer: D. CT scan

A. Full Blood Count (FBC) ✗

Helpful for anemia/infection but not the priority.

B. ECG (Electrocardiogram) ✖

Might be relevant but not first-line.

C. Thyroid function tests ✖

Hypothyroidism can cause confusion but should be checked later.

D. CT scan 

**Rule out stroke/subdural
hematoma, which is the most
urgent concern.**

4. Ketoacidosis Symptoms

Question: A patient with a history of ketoacidosis presents acutely. Which symptom is NOT expected?

Answer: C. Weight gain

A. Vomiting 

Common in DKA due to metabolic acidosis.

B. Extreme Thirst

Polydipsia occurs due to dehydration.

C. Weight Gain

DKA causes weight loss due to dehydration & catabolism.

D. Acetone breath smell 

**Caused by ketone production
(fruity odor).**

5. Pre-Operative Medication Administration

Question: What should the nurse do first before administering pre-op medications (Demerol, Atropine, Phenergan)?

Answer: D. Have the client empty bladder

A. Raise side rails 

Important but not the first step.

B. Place call bell within reach



Ensures safety but not the first priority.

C. Instruct the client to remain in bed ✖

After administration, the patient must remain in bed, but bladder emptying comes first.

D. Have the client empty bladder ✔

**Prevents urinary retention
post-sedation.**

6. Hypercalcemia in Multiple Myeloma

Question: A 24-year-old female with myeloma, confusion, constipation, polyuria. Likely cause?

Answer: B. Hypercalcemia

A. Diverticulosis 

**Unrelated to
confusion/polyuria.**

B. Hypercalcemia

Multiple Myeloma can cause hypercalcemia leading to confusion, constipation, and kidney issues.

C. Hypocalcemia

Would cause tetany, not polyuria/confusion.

D. Irritable Bowel Syndrome



**Does not explain
polyuria/confusion.**

7. Meningitis in College Student

Question: A 20-year-old female with fever and rash, found unconscious. First test?

Answer: C. Blood Cultures

A. Blood sugar check ❌

Not the priority unless diabetes is suspected.

B. CT Scan 

Only needed before lumbar puncture if raised ICP is suspected.

C. Blood Cultures 

**Meningococcal meningitis
must be diagnosed ASAP.**

D. Arterial blood gases ❌

Not diagnostic for meningitis.

8. Hyperparathyroidism Lab Findings

Question: A patient with primary hyperparathyroidism, expected lab finding?

Answer: A. Elevated serum calcium

A. Elevated serum calcium 

Due to excessive PTH secretion.

B. Low serum PTH ✖

Would be high, not low.

C. Elevated serum vitamin D ✖

Vitamin D levels are normal or low in primary hyperparathyroidism.

D. Low urine calcium ✖

Urine calcium is high due to increased excretion.

9. Exercise in Multiple Sclerosis

Question: Which type of exercise should MS patients avoid?

Answer: B. Lead to dehydration

A. Increase heart rate ❌

Cardiovascular exercise is beneficial in moderation.

B. Lead to dehydration ✅

Dehydration worsens MS symptoms.

C. Are considered aerobic ✖

Aerobic exercise is good in moderation.

D. May be competitive ✖

Competitive sports are okay if manageable.

10. Gravida and Para Notation

Question: A pregnant woman with twins and a prior miscarriage. How to document?

Answer: C. Gravida 3, Para 1

**Gravida = total pregnancies
(including current) = 3**

**Para = pregnancies carried to
viability (twins count as one) =
1**

11. Postmature Fetus Risks

Question: What is the biggest risk for a postmature fetus?

Answer: D. Progressive placental insufficiency

A. Excessive fetal weight 

Macrosomia is possible but not the biggest risk.

B. Low blood sugar levels ✖

Hypoglycemia occurs after birth, not before.

C. Depletion of subcutaneous fat ✖

Seen in growth-restricted postmature infants.

D. Progressive placental insufficiency

**Aging placenta →
oxygen/nutrient deficiency →
fetal distress.**

12. Ovarian Cancer Staging

Question: A woman with bilateral pelvic masses, lymphadenopathy, liver nodules. Cancer stage?

Answer: B. Stage III

Stage IIA ✖

Limited to pelvis.

Stage IIIB 

Spread beyond pelvis but not distant metastasis.

Stage IV 

Would have distant metastases (e.g., lungs).

**Here's a detailed breakdown
of each question, with
explanations for each option:**

1. Developmental Delay & Macrocephaly in a 2-Year-Old

**Question: A 2-year-old with
macrocephaly, stunting, coarse
features, protruded tongue,**

and umbilical hernia. What is the best initial test?

Answer: C. Thyroid Stimulating Hormone (TSH)

A. Cortisol ✗

Cortisol disorders (e.g., congenital adrenal hyperplasia) do not cause macrocephaly, stunting, or coarse features.

B. Growth Hormone ❌

GH deficiency causes proportional short stature, not coarse features or umbilical hernia.

C. Thyroid Stimulating Hormone (TSH) ✅

Congenital hypothyroidism (cretinism) is suspected. TSH and T4 levels confirm the diagnosis.

D. Vitamin D ✗

Vitamin D deficiency (rickets) causes bowed legs, not coarse facial features or macrocephaly.

2. Confirmatory Finding in Endometriosis

Question: A 28-year-old with dysmenorrhea, dyspareunia, constipation, rectal bleeding, and adnexal tenderness. What confirms endometriosis?

Answer: D. Adnexal masses

A. Pain ❌

Pain is common but non-specific.

B. Dysmenorrhea ❌

Dysmenorrhea is a symptom, not a diagnostic finding.

C. Constipation ❌

Occurs if bowel endometriosis is present but is not diagnostic.

D. Adnexal masses ✅

**Tender adnexal masses
(ovarian endometriomas)**

suggest endometriosis.

**Definitive diagnosis requires
laparoscopy.**

**3. Symptoms NOT Expected in
Acute Meningitis**

Question: A patient with a history of meningitis. Which symptom is not expected if it were acute?

Answer: A. Increased appetite

A. Increased appetite 

Meningitis causes nausea, anorexia, and vomiting. Increased appetite is not expected.

B. Vomiting ✖

Common due to increased intracranial pressure (ICP).

C. Fever ✖

Hallmark of infection.

**D. Poor tolerance of light
(Photophobia) ✖**

Classic meningeal sign.

**4. Antidote for Malathion
Poisoning**

Question: Which drug is an antidote for Malathion poisoning (Organophosphate toxicity)?

Answer: A. Atropine

A. Atropine 

**Organophosphates
(Malathion) inhibit
acetylcholinesterase → excess**

acetylcholine → cholinergic crisis.

**Atropine (anticholinergic)
blocks muscarinic effects
(salivation, bradycardia,
bronchorrhea).**

B. Flumazenil ✗

Used for benzodiazepine overdose, not organophosphate poisoning.

C. N-acetylcysteine ✖

Used for acetaminophen (paracetamol) toxicity.

D. Naloxone ✖

Opioid antagonist (used for morphine/heroin overdose).

5. Cold Chain Storage When Refrigerator Fails

Question: Nurse needs to store vaccines for 4-5 days due to a broken refrigerator. What is the best cold chain method?

Answer: C. Cold box with frozen water packs

A. Cold box with cool water pack ❌

Cool water packs do not maintain required low

temperatures for multiple days.

B. Vaccine carrier with cool water pack ❌

Only suitable for short-term storage (~24 hours).

C. Cold box with frozen water pack 

Maintains temperature for 4-5 days, best for extended outages.

D. Vaccine carrier with frozen water pack 

Used for short transport (not long-term storage).

6. Vitamin K Injection Site in an Infant

Question: A 30-day-old infant needs Vitamin K injection. Which is the best site?

Answer: C. Vastus lateralis

A. Gluteus maximus ✖

**Not recommended in infants
→ risk of sciatic nerve injury.**

B. Gluteus minimus ✖

**Also unsafe due to nerve injury
risk.**

C. Vastus lateralis 

Best site for IM injections in infants (largest muscle, no major nerves).

D. Vastus medialis 

Not commonly used due to poor muscle mass.

Would you like explanations for more cases?

**Would you like explanations
for any other questions in
more detail?**

Final Thoughts

These answers are based on clinical reasoning and guidelines. Let me know if you need further clarification or discussion!

Final Thoughts:

These answers are based on clinical guidelines and

**pathophysiological reasoning.
Let me know if you need
further clarification!**

**That's a complete breakdown.
Let me know if you need
further clarification!**

Final Thoughts

Each correct answer was chosen based on clinical guidelines and physiological mechanisms. Let me know if you need any clarifications!

**Would you like more
clarification on any of these
points?**

**Let me know if you need
further clarification!**

**Feel free to reach out if you
have more questions or need
further clarifications!**

**Let me know if you need more
explanations or further
questions!**

**Let me know if you need
further clarification or
assistance!**

Let me know if you need more clarifications!

Let me know if you'd like further details or clarification!

**Let me know if you'd like
further clarification!**

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further clarification!**

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clarification!**

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information or further
clarifications!**

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further clarifications!**

**Let me know if you need
further clarification or have
more questions!**

**I hope these explanations
help! Let me know if you need
more details.**

**Let me know if you need
further clarifications or have
more questions!**

**Let me know if you need
further assistance!**

**Let me know if you need
further clarifications!**

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further clarification!**

**Let me know if you'd like any
further clarification!**

Let me know if you'd like more details or further assistance!

Let me know if you need further clarification or additional details!

These answers are based on the patient's symptoms and clinical presentations. Let me know if you need further clarifications or details!

**Feel free to ask if you need
more explanations or have any
further questions!**

**If you need further clarification
or have additional questions,
feel free to ask!**

**Feel free to ask if you need
more details or further
clarification!**

**Let me know if you'd like
further explanations or if you
have additional questions!**

**Let me know if you'd like any
further explanations!**

**Let me know if you'd like any
further clarification!**

**Let me know if you need any
more clarifications!**

Let me know if you need any further clarifications!

These explanations should help clarify the reasoning behind each answer. Let me know if you'd like further details on any of the points!

**Would you like me to continue
with the remaining questions?**

**This covers all questions with
explanations! Let me know if
you need more clarification.**

**Would you like further
clarification on any of these?**

**Let me know if you need
explanations for the remaining
questions!**